GOLDEN YEARS

HOME HEALTH AGENCY

Policy and Procedure Manual

Chapter 246-335 WAC — In-Home Services Agencies (Home Health)

Golden Years Home Health Supported Living LLC

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Washington

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Table of Contents

REVIEW OF POLICY MANUALS 6

APPLICABILITY, SCOPE and PURPOSE of HOME HEALTH AGENCIES 7

WAC 246-335-515 PLAN OF OPERATION 9

- ORGANIZATIONAL STRUCTURE 9
- PERSONNEL JOB DESCRIPTIONS 10
- TITLE OF POSITION: CHIEF EXECUTIVE OFFICER 11
- TITLE OF POSITION: CHIEF FINANCIAL OFFICER 13
- TITLE OF POSITION: DIRECTOR OF HUMAN RESOURCES 14
- TITLE OF POSITION: QUALITY IMPROVEMENT MANAGER 16
- TITLE OF POSITION: ADMINISTRATOR 18
- TITLE OF POSITION: ALTERNATE ADMINISTRATOR 20
- TITLE OF POSITION: DIRECTOR OF CLINICAL SERVICES 22
- TITLE OF POSITION: ALTERNATE DIRECTOR OF CLINICAL SERVICES 25
- TITLE OF POSITION: REGISTERED NURSE 28
- TITLE OF POSITION: PEDIATRIC REGISTERED NURSE 32
- TITLE OF POSITION: LICENSED PRACTICAL NURSE 34
- TITLE OF POSITION: MEDICAL SOCIAL WORKER 36
- TITLE OF POSITION: REGISTERED PHYSICAL THERAPIST 38
- TITLE OF POSITION: PHYSICAL THERAPY ASSISTANT 40
- TITLE OF POSITION: OCCUPATIONAL THERAPIST 42
- TITLE OF POSITION: OCCUPATIONAL THERAPY ASSISTANT 44
- TITLE OF POSITION: SPEECH THERAPIST 46
- TITLE OF POSITION: NUTRITIONIST/DIETITIAN 48
- TITLE OF POSITION: RESPIRATORY THERAPIST 50
- TITLE OF POSITION: HOME HEALTH AIDE 52
- TITLE OF POSITION: STAFFING COORDINATOR 54
- TITLE OF POSITION: MEDICAL RECORDS MANAGER 56
- TITLE OF POSITION: OFFICE MANAGER 58
- TITLE OF POSITION: PAYROLL/BILLING/RECEIVABLES MANAGER 60
- TITLE OF POSITION: PAYROLL/BILLING/RECEIVABLES CLERK 62
- TITLE OF POSITION: BUSINESS CLERK 63
- TITLE OF POSITION: COORDINATOR 64
- TITLE OF POSITION: UNLICENSED INTAKE COORDINATOR 66
- TITLE OF POSITION: COMMUNITY LIAISON 68
- TITLE OF POSITION: RECEPTIONIST 69
- TITLE OF POSITION: DATA ENTRY OPERATOR 70
- TITLE OF POSITION: COURIER/CLERK 71
- RESPONSIBILITIES OF CONTRACTORS AND VOLUNTEERS 72

A. General Requirements 72
B. Background Checks and Suitability Determinations (WAC 246-335-425(6) & (7)) 73
SERVICES UNDER CONTRACT 74
SERVICES TO BE PROVIDED 75
Principal Purpose of the Agency 75
Definition of Term Care 75
Availability of Care 75
Services Provided 75
Remuneration Policy 76
MANAGEMENT AND SUPERVISION 77
SUPERVISION OF PERSONNEL, VOLUNTEERS, AND CONTRACTORS 79
PERFORMANCE EVALUATION FOR PERSONNEL, VOLUNTEERS, AND CONTRACTORS 81
QUALITY IMPROVEMENT PROGRAM 83
TRANSFER OF CARE 85
PRESERVING RECORDS 85
HOME HEALTH AGENCY PATIENT RECORD DOCUMENTATION 87
EMERGENCY PREPAREDNESS DISASTER PLAN 88
Staff Emergency Preparedness Plan 91
ACCIDENT PREVENTION 92
EMERGENCY AND DISASTER PROCEDURES 94
SNOW STORMS/ BLIZZARDS/SNOW EVENTS 95
HURRICANE INFORMATION 97
EARTHQUAKES/TORNADOES/OTHER DISASTERS 99
FIRE 99
COMMUNICATION PLAN 101
PATIENT INSTRUCTIONS IN THE EVENT OF AN EMERGENCY 103
COORDINATION OF SERVICES DELIVERY 104
STAFF TRAINING 106
Hazard Vulnerability Assessment (HVA) for a Home Health Agency 112
ADMINISTRATOR RESPONSIBILITIES 116
POLICY DURING THE ABSENCE OF THE ADMINISTRATOR 118

WAC 246-335-520 DELIVERY OF SERVICES 119
ADMISSION, TRANSFER & DISCHARGE POLICY 119
Notice Delivery to Representatives 119
Patients Admission 119
Admission of Patients for Home Health Services 120
Discharge / Transfer Policy 121
Discharge/Transfer Procedure 123
Transfer Summary 125
Interruption of Services 126
Completion of Discharged Records 126
DISCHARGE OF PATIENTS IN UNSAFE SITUATIONS 126

Care of the Dying Patient 127
CONSENT PRACTICES 128
REFUSAL OF CARE 129
SPECIFIC HOME HEALTH SERVICE INCLUDING ANY NON MEDICAL SERVICES 129
TIMELY INITIATION OF HOME HEALTH SERVICES 131
ROLES & RESPONSIBILITIES WITH MEDICATION ASSISTANCE 132
Management of Patient Medications and Treatments 132
COORDINATION OF CARE 134
Actions to Address Patient or Family Communication Needs 134
TELEHEALTH / TELEMEDICINE 135
EMERGENCY CARE POLICY 137
RESUSCITATION AND EMERGENCY SERVICES 139
ACTIONS TO BE TAKEN UPON DEATH OF A PATIENT 140
BACK-UP CARE POLICY 142
ADVANCED DIRECTIVES AND PATIENT ORDERS FOR LIFE-SUSTAINING TREATMENT (POLST) 144
NURSE DELEGATION OF TASKS 146

WAC 246-335-525 PERSONNEL, CONTRACTOR, AND VOLUNTEER POLICIES 148

NON-DISCRIMINATION/AFFIRMATIVE ACTION POLICY 148
REFERENCES FOR PERSONNEL, CONTRACTORS, AND VOLUNTEERS 150
Record Retention 150
CONTRACTOR POLICY 151
VERIFICATION OF HEALTHCARE PROFESSIONAL CREDENTIALS 153
BACKGROUND CHECKS FOR PERSONNEL, CONTRACTORS, VOLUNTEERS, AND OTHER INDIVIDUALS 155
CHARACTER, COMPETENCE, AND SUITABILITY DETERMINATIONS FOLLOWING BACKGROUND CHECKS 158
MANDATORY REPORTING 161
NEW HIRE ORIENTATION, SKILLS VERIFICATION, AND SUPERVISED PRACTICE 163
NURSING ASSISTANT REGISTRATION 164
SAFE FOOD HANDLING FOR PATIENT MEALS 167
CURRENT CARDIOPULMONARY RESUSCITATION (CPR) TRAINING 172
INFECTION CONTROL PRACTICES 174
Hand Hygiene Policy 178
HOME MEDICAL SUPPLIES AND EQUIPMENT 180
TUBERCULOSIS INFECTION CONTROL PROGRAM 184
ANNUAL PERFORMANCE EVALUATION 186

WAC 246-335-530 PERSONNEL, CONTRACTOR, AND VOLUNTEER RECORDS 188

PERSONNEL POLICIES AND RECORDS 188
Personnel Policies 188
Personnel Records Management 188
Personnel Record Content 188
Access 189
Employee Separation 189

PATIENT BILL OF RIGHTS 190

WAC 246-335-540 HOME HEALTH PLAN OF CARE 195

PLAN OF CARE 195

Development and Assessment 198

Required Plan of Care Content 200

Plan of Care Review Schedule 201

Documentation for One-Time Visits 203

WAC 246-335-545 SUPERVISION OF HOME HEALTH SERVICES 206

DIRECTOR OF CLINICAL SERVICES - TRAINING PLAN 206

WAC 246-335-550 PATIENT RECORDS 212

CLINICAL RECORD MAINTENANCE 213

Record Retention 214

Confidentiality/Agency Use of Records/Authority to Make Entries 215

One-time visit 216

Record Transport and Off-Site Storage 217

ACCESS TO CLINICAL RECORDS 218

Patient Access to Records 218

Release of Information 218

Authorizing Practitioner Access to the Record 219

Attorney Access to the Record 219

Third Party Payer Access to the Record 219

Other Parties Access to the Record 219

Record of Information Released 220

Deceased Patients 220

Subpoena of Records 220

WAC 246-335-555 QUALITY IMPROVEMENT PROGRAM 221

WAC 246-335-560 HOME MEDICAL SUPPLIES AND EQUIPMENT 224

HOME MEDICAL SUPPLIES AND EQUIPMENT 224

REVIEW OF POLICY MANUALS

All Policy Manuals are reviewed on a yearly basis.	
Reviewed:	Date:
Reviewed:	Date:
Reviewed:	Date:
Reviewed:	Date:
Reviewed:	Date:
Reviewed:	Date:
Reviewed:	Date:
Reviewed:	Date:
Reviewed:	Date:
Reviewed:	Date:

APPLICABILITY, SCOPE and PURPOSE of HOME HEALTH AGENCIES

WAC 246-335-300 through 246-335-355 / WAC 246-335-505 through WAC

246-335-560

WAC 246-335 Compliance

Golden Years Home Health Supported Living LLC acknowledges its review of WAC 246-335-300 through WAC 246-335-355, which outlines the general requirements for all in-home services agencies licensed in Washington State. We agree to follow these directives in all aspects of our operations.

Furthermore, **Golden Years Home Health Supported Living LLC** acknowledges its review of WAC 246-335-505 through WAC 246-335-560, which specifically applies to agencies licensed to provide home health services. We agree to follow these additional directives to ensure we meet the elevated standards of care required for home health services

Our Home Health Agency, **Golden Years Home Health Supported Living LLC**, implements programs and services to provide comprehensive, quality home health care in the patient's home or service site. All services are provided under the direction of the Administrator and the Director of Clinical Services.

Golden Years Home Health Supported Living LLC addresses the patient's unique psychological, safety, self-esteem, and self-actualization needs in a patient plan of home health care.

Golden Years Home Health Supported Living LLC serves as a resource for the patient, the patient's family, and the community. We provide health care education for the patient, the patient's family and the community. Skilled nursing services will be provided by Registered Nurses and Licensed Practical Nurses (if hired by the Agency) and Home Health Aides. Physical, Occupational, Speech, Respiratory therapy, Dietitian/Nutritionist as well as Social Work services will be provided to the extent that such professionals are either hired by the Agency or contracted for by the Agency.

Similarly, services that may be provided by Physical, Occupational and Speech Therapists, and Medical Social Workers. **Golden Years Home Health Supported Living LLC** implements methods of screening applicants and evaluating their progress regularly in order to be able to provide quality personnel.

Golden Years Home Health Supported Living LLC provides staff developmental opportunities and programs, including orientation and programs concerning current practices in nursing and allied health care.

Golden Years Home Health Supported Living LLC provides current and accurate personnel records, including verification of current license/certification and health records.

The Agency maintains open communication with patients, employees, and Authorizing Practitioners via frequent contact in order to achieve quality and cost-effective care. The counties that we serve are **King & Pierce** Our hours of operation are **are 9:00 am to 5:00 pm, Monday through Friday**

The Agency provides clinical services (either through its own staff or through contract) from Registered Nurses ("RNs"), Licensed Practical Nurses ("LPNs"), Home Health Aides ("HHAs") Therapy services will be provided by Physical Therapists ("PTs"), Occupational Therapists ("OTs"), Speech Therapists ("SPs"), Respiratory Therapists (RT), Dietitian/Nutritionist and Medical Social Workers ("MSWs") to the extent that the agency hires

or contracts for such services. **Golden Years Home Health Supported Living LLC** adheres to standards set forth State, and Local regulations.

The Agency has approved written vision and mission statements that contain the Agency's operating purpose, philosophy and operating ethics. The vision and mission statements are distributed to all employees, volunteers and members of the Agency's Governing Body. The mission may be amended only through proper approval by the Agency's governing authority.

WAC 246-335-515 PLAN OF OPERATION

ORGANIZATIONAL STRUCTURE

WAC 246-335-515

Statement of Purpose

The applicant or licensee must develop and implement a plan of operation which includes:

(1) A description of the organizational structure.

List of officers' names

President: Rose Mbote

Administrator- Disaster Coordinator - Privacy Officer: Eric Njoroge

Alternate Administrator - Eric Njoroge

Director of clinical services- Alternate Administrator - Office Staff: Rose Mbote

PERSONNEL JOB DESCRIPTIONS

WAC 246-335-525(2)

Statement of Purpose

The applicant or licensee must develop and operationalize personnel, contractor, and volunteer policies and procedures that describe:

(2) Job descriptions that contain responsibilities and are consistent with health care professional credentialing and scope of practice as defined in relevant practice acts and

TITLE OF POSITION: CHIEF EXECUTIVE OFFICER

TITLE OF IMMEDIATE SUPERVISOR: GOVERNING BODY

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS - LIMITED

DUTIES	
1.	Implementing governing body directives or policies
2.	Overseeing fiscal management
3.	Complying with applicable laws and regulations
4.	Monitoring quality and appropriateness of services and products and assuring their availability
5.	Program planning, development, administration and evaluation
6.	Overall administration of Agency and its affiliated companies
7.	Coordinating and liaising with appropriate affiliate departments and committees
8.	Representing the organization to other groups, agencies, and the general public
9.	Keeping the governing body and staff informed of current organizational, community, and industry trends
10.	Developing policies and guidelines of operations
11.	Seeking and encouraging input from other administrative and service personnel
12.	Providing professional input to related administrative systems
RESPONSIBILITIES	
1.	Participates in the formulation of a business plan strategic goals, resource allocation plan(s) and budgets
2.	Consults, plans and works with the Governing Body and Management Staff in policy formulation and program development and implementation including matters of staffing for the Agency
3.	Assists the Governing Body in planning activities to develop administrative policies and practices
4.	Coordinates with the Administrator, Director of Clinical Services and other management and program areas for appropriate services operations and administration according to the organizational structure
5.	Maintains liaison for data processing and record needs, including weekly, monthly utilization reports
6.	Keeps the Administrator, and other management staff informed of Agency and affiliate developments, needs, etc., and encourages their participation in problem solving at the organization and community levels
7.	Assists in the evaluation of community needs and plans appropriate programs/services
8.	Submits formal written proposals to the Governing Body for all new programs of Services with complete cost/benefit analysis
9.	Directs the implementation of service goals and objectives
10.	Consults, plans and works with the Governing Body to implement all support functions relating to Board activities and other committee activities, i.e., agenda, minutes, reports and yearly calendar of activities
11.	Submits monthly and other operation reports to the Governing Body on a timely basis, outlining the completion of goals and objectives including justification for those not attained
12.	Participates in selective conferences and workshops to promote knowledge in community health trends, job performance efficiency and overall professional growth
13.	Establishes personal goals and objectives for advancement

14.	Participates with other health, civic, educational and professional groups as directed
15.	Negotiates and enters into contractual agreements on behalf of the Agency
16.	Maintains sound fiscal management practices and the overall credit standing of the Agency
17.	On occasion, represents the Agency in community and civic organizations
18.	Completes job evaluations of subordinate staff` in accordance with organizational policy
19.	Directing and monitoring organizational improvement activities
20.	Responsible for the evaluation of direct subordinates.

JOB CONDITIONS	
1.	Position is stressful in terms of meeting deadlines.
2.	It requires minimal lifting of office records and printouts.
3.	It is primarily a desk job which essentially involves sitting, standing, stooping and walking, as well as an inordinate amount of telephone communication.
4.	Travel is required, by car or airplane to local, out-of-town or state seminars, conferences or meetings.
5.	The ability to communicate well, both verbally and in writing is required.

EQUIPMENT OPERATION
The job requires the ability to utilize a PC, calculator, multi- line telephone, and other related office equipment.

COMPANY INFORMATION
Has access to all patient medical records and patient financial accounts, personnel records and company financial records, which may be discussed with all management staff.

QUALIFICATIONS	
1.	Experience, business acumen, and professional relationships specific to the stated mission of the organization.
2.	Ability to demonstrate self-confidence and positive attitude toward self and others and maintain commitment and enthusiasm to goal achievement.
3.	Ability to develop and implement improved methods of operation.
4.	Ability to identify and evaluate personal strengths and weaknesses of self and others.
5.	Ability to supervise objectively without personal bias towards individuals.

ACKNOWLEDGMENT	
Employee Signature	Date

TITLE OF POSITION: CHIEF FINANCIAL OFFICER

TITLE OF IMMEDIATE SUPERVISOR: CHIEF EXECUTIVE OFFICER

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS - LIMITED

DUTIES	
Organizes and administers the business and financial activities of the Agency, advises the CEO and Administrator about financial trends affecting the Agency.	
RESPONSIBILITIES	
1.	Develops and maintains systems and procedures for Agency financial operations
2.	Prepares financial statements and reports
3.	Provides for a centralized filing system for all materials related to fiscal functions
4.	Provides for the collection and dissemination of statistical data
5.	Provides for the maintenance of all financial manuals
6.	Provides for a complete and accurate accounting of all business transactions
7.	Provides for billing services and collection of fees for service income
8.	Prepares monthly invoices and statements for contracts
9.	Assists with audits
10.	Performs other related functions as requested
JOB CONDITIONS	
1.	Position is stressful in terms of meeting deadlines.
2.	It requires minimal lifting of office records and printouts.
3.	It is primarily a desk job which essentially involves sitting, standing, stooping and walking, as well as an inordinate amount of telephone communication.
4.	Travel is required, by car or airplane to local, out-of-town or state seminars, conferences or meetings.
5.	The ability to communicate well, both verbally and in writing in English is required.
6.	The job requires the ability to utilize a PC, calculator, multi-line telephone, and other related office equipment.
COMPANY INFORMATION	
Has access to all patient medical records and patient financial accounts, personnel records and company financial records, which may be discussed with all management staff, including the Governing Body.	
QUALIFICATIONS	
1.	Related business experience or courses.
2.	Two years of accounting experience preferred.

3. Must have a criminal background check.	
ACKNOWLEDGMENT	
Employee Signature	Date

TITLE OF POSITION: DIRECTOR OF HUMAN RESOURCES

TITLE OF IMMEDIATE SUPERVISOR: ADMINISTRATOR

DUTIES	
Is responsible for managing the Human Resources Department and overseeing all personnel issues.	
RESPONSIBILITIES	
1.	Responsible for the recruitment and selection of employees based on applicant's credentials and qualifications including training and licensure
2.	Uses a consistent and non-discriminating process to select employees as follows: a. Conducts personal interviews on all applicants at the time of recruitment and selection b. Obtains two written or verbal follow-up of employment references and job history c. Verifies education, experience, training, licensure or certification, as appropriate to job responsibilities d. Selects and employs qualified individuals without regard to gender, race, age, creed, handicap, sexual orientation, or national origin
3.	Coordinates an orientation program for all new employees, including an introduction to the organization, staff and employee rules and responsibilities
4.	Ensures that all employees receive ongoing education, including in-services, training and other activities to maintain and improve staff competence
5.	Ensures that all employees meet their required mandatory training, keeps records of in-services and files employee's certificate of attendance in the employee personnel folder
6.	Ensures that all employees are meeting the performance expectations stated in his/her job descriptions by collaborating with the supervisors
7.	Ensures that all employees are in compliance with the mandatory requirements for employment
8.	Responsible for other miscellaneous duties including the following: a. Maintains strict professional ethics, confidentiality and privacy of the patient and/or family members b. Participates in personnel policies revisions, when requested c. Assumes responsibility for own personal and professional growth
9.	May be requested by the President/CEO to attend outside meetings and/or conferences
JOB CONDITIONS	
1.	Position is stressful in terms of meeting deadlines.
2.	It requires minimal lifting of office records and printouts.
3.	It is primarily a desk job which essentially involves sitting, standing, stooping and walking, as well as an inordinate amount of telephone communication.
4.	Travel is required, by car or airplane to local, out-of-town or state seminars, conferences or meetings.
5.	The ability to communicate well, both verbally and in writing in English is required.
EQUIPMENT OPERATION	
Utilization of calculator, multi-line telephone, copy machine, basic medical equipment	
COMPANY INFORMATION	
Has access to all patient medical records, personnel records and patient financial accounts which may be discussed with the Executive Director, Director of Reimbursement, Employee Relations and Quality Improvement department staff, as well as outside government agencies.	
QUALIFICATIONS	
1.	A college degree is preferable.

2.	Experience in interviewing and recruitment of qualified applicants is required.
3.	Strong leadership and excellent interpersonal relationship abilities are essential.

ACKNOWLEDGMENT

Employee Signature Date

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS - LIMITED

TITLE OF POSITION: QUALITY IMPROVEMENT MANAGER

TITLE OF IMMEDIATE SUPERVISOR: Director of Clinical Services

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS - LIMITED

DUTIES	
Assists in the development, implementation and evaluation of the Quality Assessment and Performance Improvement (QAPI) program and activities that support Agency's objectives in delivering quality patient care services.	
RESPONSIBILITIES	
1.	Assists the Quality Improvement (QAPI) Director in developing and implementing an Agency CQI program
2.	Assists in preparing monthly payer billing
3.	Assists QAPI Director in reviewing plans of care
4.	Assists QAPI Director with chart audits
5.	Assists QAPI Director with quarterly U.R. statistics
6.	Assists in the implementation of approved recommendations and assessing the Agency's success in correcting clinical record deficiencies
7.	Regularly participates in case conference activities and assures through attendance and record reviews that conferences are conducted and documented in accordance with Agency policy
8.	Reviews all reports of complaints, errors, incidents, and grievances to ensure that they are fully documented and properly handled in the timeframe and manner specified by the Agency's policy
9.	Coordinates a program for risk management which encompasses written guidelines and provides for supervision of workers on cases with risk factors, such as: contagious disease, infections, AIDs, hepatitis, high tech therapies, high tech equipment and supplies, and other situations in which the patient or worker is at risk
10.	Makes home visits regularly to observe care being rendered, to assess the condition and use of equipment and supplies, and to receive reports of the patient/family satisfaction with care
11.	Assists the Director in evaluating and resolving employee complaints, grievances and turnover issues
12.	Performs other tasks as assigned by the Administrator, DCS and QAPIDirector
13.	Assures the confidentiality of clinical records
14.	Promotes and maintains an environment that is compliant with state and local regulatory agencies
15.	Participates in development and implementation of a staff educational program for documentation and Quality Improvement
16.	Works with administrative and supervisory personnel regarding home care visits to assure high quality and proper follow-up patient care with regard to patient needs
17.	Consults with authorized practitioners when necessary to ensure and maintain safe, efficient, and quality patient care
18.	Performs chart audits and assists with case conferences
19.	Participates with other health, civic, education and professional groups as directed by the Director of Quality Improvement, the Administrator and the Director of Clinical Services

20.	Keeps current on local area issues and works with organizations that work to improve the health and welfare of the community as directed by the Administrator and the Director of Quality Improvement

JOB CONDITIONS	
1.	Position is stressful in terms of meeting deadlines.
2.	It requires minimal lifting of office records and printouts.
3.	It is primarily a desk job which essentially involves sitting, standing, stooping and walking, as well as an inordinate amount of telephone communication.
4.	Travel is required, by car or airplane to local, out-of-town or state seminars, conferences or meetings.
5.	The ability to communicate well, both verbally and in writing in English is required

EQUIPMENT OPERATION
Utilization of CRT, calculator, multi-line telephone, copy machine, basic medical equipment

COMPANY INFORMATION
Has access to all patient medical records, personnel records and patient financial accounts which may be discussed with the Executive Director, Director of Finance, Employee Relations and Quality Improvement department staff, as well as outside government agencies

QUALIFICATIONS	
1.	Must be a Washington licensed Registered Nurse or Licensed Practical Nurse holding a current state license.
2.	Must be adept at preparing patient charts.
3.	Must have experience in developing, implementing and evaluating continuous quality improvement programs and activities.
4.	Must have the ability to demonstrate self-confidence and a positive attitude and maintain enthusiasm and commitment with respect to goal achievement.

ACKNOWLEDGMENT	
Employee Signature	Date

TITLE OF POSITION: ADMINISTRATOR

TITLE OF IMMEDIATE SUPERVISOR: Governing Body

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS - LIMITED

DUTIES	
The Administrator is a qualified full-time employee appointed by the Governing Body who has full authority for management of the Agency operations that includes administering, directing and coordinating all administrative and financial activities of the home health agency.	
RESPONSIBILITIES Statement: WAC 246-335-515 Plan of Operation (12) The Administrator must continue to update its plan of operation to reflect current practice, services provided by the agency, and state and local laws.	
1.	Oversee the day-to-day operation and fiscal affairs of the agency
2.	Implement the state regulations
3.	Designate in writing an alternate to act in the Administrator's absence
4.	Provide management and supervision of services throughout all approved service areas
5.	Arrange for necessary services
6.	Keep contracts current and consistent with the state regulations.
7.	Serve as a liaison between the licensee, personnel and contractors
8.	Ensure personnel and contractors are currently credentialed by the state of Washington, when appropriate, according to applicable practice acts and consistent with the state regulations.
9.	Ensure personnel and contractors comply with the agency's policies and procedures
10.	Implement a quality improvement process consistent with the state regulations.
11.	Manage record keeping according to the state regulations
12.	Ensure supplies and equipment necessary to patient care are available, maintained, and in working order
13.	Ensure the accuracy of public information materials
14.	Serves as the agency's Disaster Coordinator
15.	Acts as corporate compliance officer and assumes responsibilities described in the agency corporate compliance plan
16.	Acts as the HIPAA Privacy Officer
CONTINUING PROGRAM EVALUATION	
1.	Participate in the review, analysis and appraisal of the effectiveness of the total Agency program
2.	Evaluate service policies and functions, and recommend proposals for changes or study of problems which affect the Agency
3.	Evaluate the performance of the individuals in the program to establish standards and the individual's professional development
4.	Implement all utilization review activities
5.	Coordinate the timely implementation of corrective action plans and controls
MEETS STATE REQUIREMENTS	
1.	Be familiar with state regulations and maintain them in the Agency

2.	Be responsible for familiarizing the employees with all laws and regulations, and make available copies of the rules available for their use
3.	Be responsible for the completion, maintenance and submission of such reports and records as required
4.	Designate a qualified employee to be his/her authorized representative in his/her absence
5.	Maintain a current organizational chart to show lines of authority to the patient level
6.	Clearly identify public services provided by the Agency in the geographic area in which these services are available
7.	Maintain office facility for the Agency which is large enough for efficient staff work, adequately equipped, and which provides for a safe working environment, meeting local ordinances and fire regulations
8.	Ensure the accuracy of public information materials and activities
ENSURE THE ACCURACY OF PUBLIC MATERIALS	
1.	Ensure the accuracy of public information materials and activities.
2.	Develops cooperative relationships with other agencies for the exchange of information and services and with community agencies to foster an understanding of Agency programs.
3.	Contact local, state and national associations and participate in meetings and conventions.
JOB CONDITIONS	
1.	Position is stressful in terms of meeting deadlines.
2.	It requires minimal lifting of office records and printouts.
3.	It is primarily a desk job which essentially involves sitting, standing, stooping and walking, as well as an inordinate amount of telephone communication.
4.	Travel is required, by car or airplane to local, out-of-town or state seminars, conferences or meetings.
5.	The ability to communicate well, both verbally and in writing in English is required.
6.	Must be able to read a type that is 12 points or larger.
EQUIPMENT OPERATION	
The ability to utilize a PC, calculator, multi-line telephone, and other related office equipment is required.	
COMPANY INFORMATION	
Has access to all patient medical records and patient financial accounts, personnel records and Agency financial records, which may be discussed with all management staff, including the Governing Body.	
QUALIFICATIONS	
1.	A professional with training and experience in health services with at least two (2) years of supervisory or administrative experience in home healthcare or related health programs and the experience, knowledge and ability to effectively administer the program is preferred.
2.	Must have a criminal background check.
ACKNOWLEDGMENT	
Employee Signature	Date

TITLE OF POSITION: ALTERNATE ADMINISTRATOR

TITLE OF IMMEDIATE SUPERVISOR: Administrator

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS - LIMITED

DUTIES	
Act as Administrator in the absence of the Administrator and directs and coordinates all administrative and financial activities of the professional home health care agency.	
RESPONSIBILITIES IN THE ABSENCE OF THE ADMINISTRATOR	
1.	Oversee the day-to-day operation and fiscal affairs of the agency
2.	Implement the state regulations
3.	Provide management and supervision of services throughout all approved service areas
4.	Arrange for necessary services
5.	Keep contracts current and consistent with the state regulations.
6.	Serve as a liaison between the licensee, personnel and contractors
7.	Ensure personnel and contractors are currently credentialed by the state of Washington, when appropriate, according to applicable practice acts and consistent with the state regulations.
8.	Ensure personnel and contractors comply with the agency's policies and procedures
9.	Implement a quality improvement process consistent with the state regulations.
10.	Manage record keeping according to the state regulations.
11.	Ensure supplies and equipment necessary to patient care are available, maintained, and in working order
12.	Ensure the accuracy of public information materials
13.	Serves as the agency's Disaster Coordinator
14.	Acts as corporate compliance officer and assumes responsibilities described in the agency corporate compliance plan
15.	Acts as the HIPAA Privacy Officer
CONTINUING PROGRAM EVALUATION	
1.	Participate in the review, analysis and appraisal of the effectiveness of the total Agency program
2.	Evaluate service policies and functions, and recommend proposals for changes or study of problems which affect the Agency
3.	Evaluate the performance of the individuals in the program to establish standards and the individual's professional development
4.	Implement all utilization review activities
5.	Coordinate the timely implementation of corrective action plans and controls
MEETS STATE REQUIREMENTS	
1.	Be familiar withstate regulations and maintain them in the Agency
2.	Be responsible for familiarizing the employees with all laws and regulations, and make available copies of the rules available for their use
3.	Be responsible for the completion, maintenance and submission of such reports and records as required

4.	Designate a qualified employee to be his/her authorized representative in his/her absence
5.	Maintain a current organizational chart to show lines of authority to the patient level
6.	Clearly identify public services provided by the Agency in the geographic area in which these services are available
7.	Maintain office facility for the Agency which is large enough for efficient staff work, adequately equipped, and which provides for a safe working environment, meeting local ordinances and fire regulations
8.	Ensure the accuracy of public information materials and activities

ENSURE THE ACCURACY OF PUBLIC MATERIALS	
1.	Ensure the accuracy of public information materials and activities.
2.	Develops cooperative relationships with other agencies for the exchange of information and services and with community agencies to foster an understanding of Agency programs.
3.	Contact local, state and national associations and participate in meetings and conventions.

JOB CONDITIONS	
1.	Position is stressful in terms of meeting deadlines.
2.	It requires minimal lifting of office records and printouts.
3.	It is primarily a desk job which essentially involves sitting, standing, stooping and walking, as well as an inordinate amount of telephone communication.
4.	Travel is required, by car or airplane to local, out-of-town or state seminars, conferences or meetings.
5.	The ability to communicate well, both verbally and in writing in English is required.
6.	Must be able to read a type that is 12 points or larger.

EQUIPMENT OPERATION
The ability to utilize a PC, calculator, multi-line telephone, and other related office equipment is required.

COMPANY INFORMATION
Has access to all patient medical records and patient financial accounts, personnel records and Agency financial records, which may be discussed with all management staff, including the Governing Body.

QUALIFICATIONS	
1.	A professional with training and experience in health services with at least two (2) years of supervisory or administrative experience in home healthcare or related health program and the experience, knowledge and ability to effectively administer the program is preferred.
2.	Must have a criminal background check.

ACKNOWLEDGMENT	
Employee Signature	Date

TITLE OF POSITION: DIRECTOR OF CLINICAL SERVICES

TITLE OF IMMEDIATE SUPERVISOR: Administrator

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS- HIGH

DUTIES OF POSITION
The Director of Clinical Services (DCS) is responsible for nursing, therapy, nutritional, social, and related services that support the plan of care provided by the agency.
The Director of Clinical Services (DCS), in accordance with policies and standards established by the Governing Body, and under the direct supervision of the Administrator, assists with the planning, coordination, leading, controlling and evaluation of home health services and is an employee of the agency.
The DCS is responsible for coordination and liaison with appropriate parent and affiliate Departments and committees, development of policies and guidelines for and obtaining input from service personnel and providing professional input to related administrative systems.
The DCS directs and supervises professional and auxiliary personnel rendering patient care services, performs marketing duties and fosters public relations for the company, consults and plans with health care facilities regarding staffing needs, and communicates the concept of quality patient care to the general community. Also remains available during all hours that services are offered.
The DCS interprets service needs to the Administrator and acts as a patient services administrative liaison.
Designates in writing a similarly qualified alternate to act in the absence of the Director of Clinical Services.
RESPONSIBILITIES OF POSITION
Ensures that patient care policies and procedures related to each service provided is followed.
Ensures that the supervision of all patient care provided by personnel is in accordance with the agency's policy.
Completes the evaluation of services provided by contractors.
Ensures the coordination of services when one or more licensed agencies are providing care to the patient.
Ensures compliance with the plan of care(s).
Ensures that all direct care personnel and contractors observe and recognize changes in the patient's condition and needs and report any changes to the Director of Clinical Services or designee.
Ensures that all direct care personnel and contractors initiate emergency procedures according to agency policy.
Completes a minimum of ten (10) hours of training annually related to clinical supervision duties and the delivery of home health services.
Coordinates with other program areas and senior management as appropriate according to the organizational structure.
Acts as liaison with Staff Development for program areas.
Participates in selected orientation programs.
Keeps staff informed of agency developments, needs, etc., and encourages their participation in problem solving at the organization and community levels.
Assists in the evaluation of community needs and plans programs and services to meet identified needs.
Submits formal written proposals to the Administrator for all new programs or services (with complete cost/benefit analysis).
Assists with QI and utilization review activities.

Participates in selective conferences and workshops to promote knowledge in community health trends, efficiency in job performance and overall professional growth.
Participates with other health, civic, education and professional groups as directed by the Administrator.
Serves on and periodically reviews policies relevant to patient care with the Quality Improvement Committee.
Assists in marketing the Agency and fostering community relations.
Consults with authorized practitioners in matters relating to patient care services.
Consults and plans with health care facilities regarding staffing needs.
Evaluates own job performance and utilization of resources in planning for professional growth.
Participates in state and national organizations, meetings, seminars, workshops and activities relating to the health professions and health care services, when appropriate.
PROMOTES STAFF DEVELOPMENT
Utilizes capabilities of professional and paraprofessional personnel in the development of orientation and staff education programs.
Recognizes leadership potential and offers opportunities for leadership training and development.
Encourages and assists personnel to continue self-improvement through formal education, educational meetings and active participation in professional and related organizations.
Assists with interviews and hires select home health care personnel.
JOB CONDITIONS
Position is stressful in terms of meeting deadlines.
It requires minimal lifting of office records and printouts.
It is primarily a desk job which essentially involves sitting, standing, stooping and walking, as well as an inordinate amount of telephone communication.
Travel is required, by car or airplane to local, out-of-town or state seminars, conferences or meetings.
Must be able to read 12 point and larger type.
The ability to communicate well, both verbally and in writing in English is required.
EQUIPMENT OPERATION
The job requires the ability to utilize a PC, calculator, multi-line telephone, and other related office equipment.
COMPANY INFORMATION
Has access to all patient medical records and patient financial accounts, personnel records and company financial records, which may be discussed with all management staff.
QUALIFICATIONS
A Registered Nurse currently licensed in Washington under the state regulations.
Must have two years of nursing experience, one of which has been preferably home health experience preferred.
A Bachelor’s degree is preferred.
Must have current CPR certification.
Must have a criminal history background clearance.
ACKNOWLEDGMENT

Employee Signature	Date

TITLE OF POSITION: ALTERNATE DIRECTOR OF CLINICAL SERVICES

TITLE OF IMMEDIATE SUPERVISOR: Director of Clinical Services

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS- HIGH

DUTIES OF POSITION
The Alternate Director of Clinical Services (ADCS) is responsible for nursing, therapy, nutritional, social, and related services that support the plan of care provided by the agency in the absence of the DCS.
The ADCS, in accordance with policies and standards established by the Governing Body, and under the direct supervision of the Administrator, assists with the planning, coordination, leading, controlling and evaluation of home health services and is an employee of the agency.
The ADCS is responsible for coordination and liaison with appropriate parent and affiliate Departments and committees, development of policies and guidelines for and obtaining input from service personnel and providing professional input to related administrative systems.
The ADCS directs and supervises professional and auxiliary personnel rendering patient care services, performs marketing duties and fosters public relations for the company, consults and plans with health care facilities regarding staffing needs, and communicates the concept of quality patient care to the general community. Also remains available during all hours that services are offered.
The ADCS interprets service needs to the Administrator and acts as a patient services administrative liaison.
RESPONSIBILITIES OF POSITION
Ensures that patient care policies and procedures related to each service provided is followed.
Ensures that the supervision of all patient care provided by personnel is in accordance with the agency's policy.
Completes the evaluation of services provided by contractors.
Ensures the coordination of services when one or more licensed agencies are providing care to the patient.
Ensures compliance with the plan of care(s).
Ensures that all direct care personnel and contractors observe and recognize changes in the patient's condition and needs and report any changes to the Director of Clinical Services or designee.
Ensures that all direct care personnel and contractors initiate emergency procedures according to agency policy.
Completes a minimum of ten (10) hours of training annually related to clinical supervision duties and the delivery of home health services.
Coordinates with other program areas and senior management as appropriate according to the organizational structure.
Acts as liaison with Staff Development for program areas.
Participates in selected orientation programs.
Keeps staff informed of agency developments, needs, etc., and encourages their participation in problem solving at the organization and community levels.
Assists in the evaluation of community needs and plans programs and services to meet identified needs.
Submits formal written proposals to the Administrator for all new programs or services (with complete cost/benefit analysis).
Assists with QI and utilization review activities.

Participates in selective conferences and workshops to promote knowledge in community health trends, efficiency in job performance and overall professional growth.
Participates with other health, civic, education and professional groups as directed by the Administrator.
Serves on and periodically reviews policies relevant to patient care with the Quality Improvement Committee.
Assists in marketing the Agency and fostering community relations.
Consults with authorized practitioners in matters relating to patient care services.
Consults and plans with health care facilities regarding staffing needs.
Evaluates own job performance and utilization of resources in planning for professional growth.
Participates in state and national organizations, meetings, seminars, workshops and activities relating to the health professions and health care services, when appropriate.
PROMOTES STAFF DEVELOPMENT
Utilizes capabilities of professional and paraprofessional personnel in the development of orientation and staff education programs.
Recognizes leadership potential and offers opportunities for leadership training and development.
Encourages and assists personnel to continue self-improvement through formal education, educational meetings and active participation in professional and related organizations.
Assists with interviews and hires select home health care personnel.
JOB CONDITIONS
Position is stressful in terms of meeting deadlines.
It requires minimal lifting of office records and printouts.
It is primarily a desk job which essentially involves sitting, standing, stooping and walking, as well as an inordinate amount of telephone communication.
Travel is required, by car or airplane to local, out-of-town or state seminars, conferences or meetings.
Must be able to read 12 point and larger types.
The ability to communicate well, both verbally and in writing in English is required.
EQUIPMENT OPERATION
The job requires the ability to utilize a PC, calculator, multi-line telephone, and other related office equipment.
COMPANY INFORMATION
Has access to all patient medical records and patient financial accounts, personnel records and company financial records, which may be discussed with all management staff.
QUALIFICATIONS
A Registered Nurse currently licensed in Washington under the state regulations.
Must have two years of nursing experience, one of which has been preferably home health experience preferred.
A Bachelor’s degree is preferred.
Must have current CPR certification.
Must have a criminal history background clearance.
ACKNOWLEDGMENT

Employee Signature	Date

TITLE OF POSITION: REGISTERED NURSE

TITLE OF IMMEDIATE SUPERVISOR: Director of Clinical Services

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS - HIGH

DUTIES	
To provide nursing care in accordance with the patient's plan of care to include comprehensive health and psychosocial evaluation, monitoring of the patient's condition, health promotion and prevention coordination of services, teaching and training activities and direct nursing care	
RESPONSIBILITIES	
1.	Coordinates total patient care by conducting comprehensive health and psychosocial evaluation, monitoring the patient's condition, promoting sound preventive practices, coordinating services and teaching and training activities
2.	Evaluates the effectiveness of nursing service to the patient and family on an ongoing basis
3.	Performs admission, re-assessments, transfer, resumption of care and discharge paperwork for the home care patient
4.	Prepares and presents patient's record to the Clinical Record Review Committee as indicated
5.	Consults with the attending authorized practitioner concerning alterations of Patient Care Plans, checks with the appropriate supervisor and makes changes, as appropriate
6.	Coordinates patient services
7.	Submits clinical notes no less often than weekly, and progress notes and other clinical record forms outlining the services rendered as indicated
8.	Submits a tally of patient care visits made each day
9.	Participates in case conferences, discusses with the supervisor problems concerning the patients and how they may best be handled
10.	Discusses with the appropriate supervisor the need for the involvement of other members of the health team
11.	Obtains orders for paraprofessional service and submits a referral to the appropriate personnel
12.	Participates in the patient's discharge planning process
13.	Cooperates with other agencies providing nursing or related services to provide continuity of care and to implement a comprehensive care plan
14.	Participates in staff development meeting
15.	Continually strives to improve his/her nursing care skills by attending in-service education, through formal education, attendance at workshops, conferences, active participation in professional and related organizations and individual research and reading
16.	Participates in the development and periodic revision of the authorized practitioner's Plan of Care and processes change orders as needed
17.	Submits timely clinical notes, progress notes and other clinical record forms outlining the services rendered
18.	Participates in the patient's discharge planning process
19.	Maintains an on-going knowledge of current drug therapy
20.	Adheres to Washington nurse practice act
21.	May be requested by Director of Clinical Services to fill in for the other nurses

COORDINATES THE ADMISSION OF A PATIENT TO THE AGENCY	
1.	Conducts an initial and ongoing comprehensive assessment of the patient's needs at appropriate time frames
2.	Obtains a medical history from the patient and/or a family member particularly as it relates to the present condition
3.	Conducts a physical examination of the patient, including vital signs, physical assessment, mental status, appetite and type of diet, etc.
4.	Evaluates the patient, family member(s) and home situation to determine what health teaching will be required
5.	Evaluates the patient's environment to determine what assistance will be available from family members in caring for the patient
6.	Evaluates the patient's condition and home situation to determine if the services of a Home Health Aide will be required and the frequency of this service
7.	Explains nursing and other Agency services to patients and families as a part of planning for care
8.	Develops and implements the nursing care plan
9.	May be requested by the Director of Clinical Services to fill in for other nurses who are on vacation or sick.
PROVIDES SKILLED NURSING CARE AS OUTLINED IN THE NURSING CARE PLAN	
1.	Nursing services, treatments and preventative procedures requiring substantial specialized skill and ordered by the authorized practitioner
2.	The initiation of preventive and rehabilitative nursing procedures as appropriate for the patient's care and safety
3.	Observing signs and symptoms and reporting to the authorized practitioner reactions to treatments, including drugs, as well as changes in the patient's physical or emotional condition
4.	Teaching, supervising and counseling the patient and caregivers regarding the nursing care needs and other related problems of the patient at home
ASSUMES RESPONSIBILITY FOR THE CARE GIVEN BY THE HOME HEALTH AIDE	
1.	Supervises and evaluates the care given by the Home Health Aide per agency policy
2.	Submits to the appropriate department/individual a written evaluation of the Home Health Aides who are providing service to the patients in his/her geographical area
3.	Participates in periodic conferences with the Home Health Aide supervisor concerning the Aide's performance
4.	Charts those services rendered to the patient by the staff nurse and changes that have been noted in the patient's condition and/or family and home situation, makes revisions in the nursing care plan as needed, records supervisory visits conducted with the Home Health Aide, evaluates patient care and progress and closes charts of discharged patients
5.	Evaluates the effectiveness of her nursing service to the individual and family
6.	Consults with the attending authorized practitioner concerning alteration of the plan of treatment in consultation with the supervisor
7.	Submits a tally of visits made each day
8.	Participates in case conferences
9.	Discusses with the supervisor problems concerning the patients and possible resolution
10.	Discusses with the supervisor the need for involvement of other members of the health team such as the home health aide, physical therapist, speech therapist, occupational therapist, social worker, etc.

11.	Obtains orders for paraprofessional service and submits referral to appropriate personnel
12.	Provides guidance and supervision to the LPN and supervises the LPN per agency policy
13.	Coordinates total patient care
14.	Cooperates with other agencies providing nursing or related services to provide continuity of care and to implement a comprehensive care plan
15.	Participates in staff development meetings
16.	Participates in the educational experiences for student nurses
17.	Continually strives to improve his/her nursing care by attending in-service education, through formal education, attendance at workshops, conferences, goal setting, active participation in professional and related organizations and individual research and reading
18.	Participates in the planning, operation and evaluation of the nursing service
19.	Participates in the development and periodic revision of the authorized practitioner's Plan of Treatment and processes change orders as needed
20.	Participates in the patient's discharge planning
21.	Maintains an on-going knowledge of current drug therapy
22.	Prepares the care plan for the Home Health Aide

JOB CONDITIONS

1.	Must have a driver's license and be willing and able to drive to patient's residences.
2.	The ability to access patients' homes which may not be routinely wheelchair accessible is required. Hearing, eyesight and physical dexterity must be sufficient to perform a physical assessment of the patient's condition and to perform and demonstrate patient care.
3.	Physical activities will include, walking, sitting, stooping, and standing and minimal to maximum lifting of patients and the turning of patients.
4.	The ability to communicate both verbally and in writing in English is required as frequent communication by telephone and in writing is involved.

EQUIPMENT OPERATION

Thermometer, B/P cuff, glucometer, penlight, hand washing materials

COMPANY INFORMATION

Has access to all patient medical records, personnel records and patient financial accounts which may be discussed with the Director of Clinical Services.

QUALIFICATIONS

1.	Must be currently licensed in Washington as a Registered Nurse under chapter 18.79 RCW.
2.	A minimum of one-year experience in community/home health agency or in a hospital setting is preferred.
3.	Must have a working knowledge of home health care and the principles and techniques of professional nursing and required documentation that pertains to it.
4.	Should be skillful in organization and in the principles of time management and have knowledge of management processes.
5.	Must be able to contribute to the quality of care being rendered through constructive communication with nursing managers and staff.
6.	Must have a criminal background clearance.

7.	Must have a current CPR certification.

ACKNOWLEDGMENT

Employee Signature Date

TITLE OF POSITION: PEDIATRIC REGISTERED NURSE

TITLE OF IMMEDIATE SUPERVISOR: Director of Clinical Services

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS - HIGH

DUTIES	
To provide nursing care is in accordance with the pediatric patient's plan of care,	
RESPONSIBILITIES	
1.	Conducts initial and ongoing comprehensive assessments of the patient's needs
2.	Comprehensive health and psychosocial evaluation, monitoring of the patient's condition, health promotion and prevention, coordination of services, teaching and training activities
3.	Initiates the plan of care and revises it as necessary
4.	Provides all services requiring specialized pediatric nursing skills
5.	Educates the patient and family regarding the disease process, self-care techniques and prevention procedures
6.	Initiates and maintains appropriate preventive and rehabilitative nursing procedures
7.	Prepares clinical and progress notes; evaluates the effectiveness and outcomes of care
8.	Coordinates services with other departments and other appropriate caregivers
9.	Analyzes and provides chart assessments for case conferences
10.	Acts as a liaison with authorized practitioners, hospitals and specialty organizations and provides them with information and instruction
11.	Supervises LPNs and paraprofessionals providing services to the patient
12.	Plans for the patient's discharge from the program
13.	Participates in in-service programs
JOB CONDITIONS	
1.	Must be able to access patients' homes which may not be routinely wheelchair accessible.
2.	Hearing, eyesight and physical dexterity must be sufficient to perform a physical assessment of the patient's condition and to perform patient care.
3.	Physical activities will include, walking, sitting, stooping, and standing and minimal to maximum lifting of patients and the turning of patients.
4.	The ability to communicate both verbally and in writing is required as frequent communication by telephone and in writing in English is involved.
EQUIPMENT OPERATION	
Thermometer, B/P cuff, glucometer, penlight, hand washing materials	
COMPANY INFORMATION	
Has access to all patient medical records, personnel records and patient financial accounts which may be discussed with the Director of Clinical Services.	
QUALIFICATIONS	
1.	Must be currently licensed in Washington as a Registered Nurse under chapter 18.79 RCW.

2.	Must have one-year Pediatric Nursing experience, preferably in a Home Health Agency; should also have expertise in any specialized nursing procedures and equipment used for infants, children and adolescents.
3.	Must have a criminal background clearance.
4.	Must have current CPR certification.

ACKNOWLEDGMENT

Employee Signature Date

TITLE OF POSITION: LICENSED PRACTICAL NURSE

TITLE OF IMMEDIATE SUPERVISOR: Director of Clinical Services or Registered Nurse

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – HIGH

DUTIES	
1.	Provides skilled nursing care under the direct supervision of the Registered Nurse.
2.	Performs selected skilled actions in the provision of curative, rehabilitative, palliative or preventative nursing care.
3.	Is responsible and accountable for making decisions that are based on the individual's nursing experience and educational preparation.
RESPONSIBILITIES	
1.	Assists the Registered Nurse in carrying out the plan of care
2.	Assists the Registered Nurse in performing specialized procedures as directed by the Registered Nurse under authorized practitioner's orders
3.	Assists with the preparation, implementation and continuing evaluation of the patient care plan
4.	Recognizes and understands the effects of social and economic problems upon patients and provides for the emotional and physical comfort and safety of patients
5.	Helps teach the patient the appropriate self-care techniques
6.	Fosters cooperative effort among personnel by understanding the functions of other persons involved in patient care and by active participation in team and staff conferences
7.	Assumes responsibility for personal and vocational growth and development through active participation in nursing organizations and participating in in-service programs and other on the job learning
8.	Prepares clinical and progress notes (not to include admission)
9.	Observes, records, and reports to the appropriate person patient symptoms, reactions and changes including: a. The general physical and mental condition of patients and signs and symptoms which may be indicative of untoward changes b. Stresses in human relationships between the patients and staff personnel and between patients and their families and visitors
10.	Observes, records and reports to the appropriate person the general physical and mental condition of patients and signs and symptoms which may be indicative of changes in condition
11.	Assists with the rehabilitation of patients according to the medical care plan by encouraging the interests and special aptitudes of the patient and encouraging patients to help themselves using their own capabilities
12.	Further assists in rehabilitation by reporting any abnormality in the patient’s range of motion, body mechanics or body alignment
13.	Actively participates in nursing organizations and promotes and participates in in-service programs
TREATMENTS AND PROCEDURES	
The Licensed Practical Nurse with the education and proven clinical competence may perform certain treatments/procedures provided the Home Health Agency’s policies and the State’s Regulatory Acts	
allow their performance.	
JOB CONDITIONS	
1.	The ability to communicate well, both verbally and in writing in English is required.

2.	The ability to access patients' homes which may not be routinely wheelchair accessible is required.
3.	Hearing, eyesight and physical dexterity must be sufficient to perform a physical assessment of the patient's condition and to perform patient care.
4.	Physical activities will include, walking, sitting, stooping, and standing and minimal to maximum lifting of patients and the turning of patients.

EQUIPMENT OPERATION

Utilization of calculator, multi-line telephone, copy machine, basic medical equipment.

COMPANY INFORMATION

Has access to all patient medical records, personnel records and patient financial accounts which may be discussed with the Executive Director, Director of Reimbursement, Employee Relations and Quality Improvement department staff.

QUALIFICATIONS

Must be currently licensed in Washington as a Licensed Practical Nurse under the state regulations.

One or more years in Home Health Agency or in a hospital setting, Home health experience preferred.

Must have a criminal background clearance.

Must have current CPR certification.

ACKNOWLEDGMENT

Employee Signature Date

TITLE OF POSITION: MEDICAL SOCIAL WORKER

TITLE OF IMMEDIATE SUPERVISOR: Director of Clinical Services

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – HIGH

DUTIES OF POSITION	
Provides medical social services to patients in their homes in accordance with authorized practitioner orders and under the direction and supervision of the Director of Clinical Services or another appropriate supervisor.	
POSITION RESPONSIBILITIES	
1.	Assists in the admission process of the patient to the Agency by performing an initial evaluation, assessing the patient's psychosocial status and evaluating the patient, family and home to identify socioeconomic/emotional factors that will affect the plan of care
2.	Assists in development and implementation of the interdisciplinary patient care plan as it pertains to medical social work
3.	Observes, records and reports changes in the patient's emotional and social factors that affect the patient's illness and his/her need for care and his/her response to treatment
4.	Consults with the attending authorized practitioner concerning alteration of the plan of treatment
5.	Maintains and submits written clinical records as deemed by the Agency, including the initial evaluation, the care plan and daily notes
6.	Evaluates the patients and family's response to, and effectiveness of, the medical social work intervention
7.	Confirms, on a weekly basis, the scheduling of visits with the Director of Clinical Services to facilitate coordination of other staff visits
8.	Participates in Agency activities and committees when appropriate to include staff development activities and in-service education
9.	Participates in discharge planning
10.	Assumes responsibility for self-development by continually striving to improve his/her Medical Social Worker practice through formal education, attendance at workshops and conferences, active participation in professional and related organizational meetings, and individual research and reading
SKILLED MEDICAL SOCIAL WORK SERVICES	
1.	Assesses the patient's potential to cope with his/her social and health problems
2.	Acts as consultant to the members of the health team, assists them in understanding the social, emotional and environmental factors related to the patient's health problems
3.	Helps patients to utilize the resources of their families and the community
4.	Provides rehabilitative and supportive casework
5.	Assists patients and their families in coping with personal and environmental difficulties which might predispose them toward illness or interfere with obtaining maximum benefits from medical care
JOB CONDITIONS	
1.	The ability to communicate well, both verbally and in writing in English is required.
2.	The ability to access patients' homes which may not be routinely wheelchair accessible.
3.	Hearing, eyesight and physical dexterity must be sufficient to perform a physical assessment of the patient's condition and to perform patient care.

4.	Physical activities will include, walking, sitting, stooping, and standing and minimal to maximum lifting of patients and the turning of patients.

EQUIPMENT OPERATION

Utilization of calculator, multi-line telephone, copy machine, hand washing materials.

COMPANY INFORMATION

Has access to all patient medical records which may be discussed with the Director of Clinical Services, the Administrator, the Executive Director, and the Quality Improvement department staff, including medical records and outside government agencies.

QUALIFICATIONS

1.	Have a degree from a social work educational program accredited and approved in accordance with the state regulations, or Meets the qualifications provided in 42 C.F.R. Section 418.114 as it existed on January 1. 2012: has a Master of Social Work (MSW) degree from a school of social work accredited by the Council on Social Work Education.
2.	Two years' experience, preferred, with at least one year of experience in a health care setting (hospital, clinic, rehabilitation center, etc.)
3.	Must have a criminal background clearance.
4.	Must have current CPR certification.

ACKNOWLEDGMENT

Employee Signature Date

TITLE OF POSITION: REGISTERED PHYSICAL THERAPIST

TITLE OF IMMEDIATE SUPERVISOR: Director of Clinical Services

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – HIGH

DUTIES	
Provides skilled Physical Therapy services on an intermittent basis to patients in their homes in accordance with authorized practitioner orders in accordance with the state regulations.	
RESPONSIBILITIES	
1.	Provides initial and ongoing comprehensive assessments of the patient's needs
2.	Establishes a treatment plan in consultation with the authorized practitioner and revises it with the approval of the authorized practitioner as necessary
3.	Assists in development and implementation of the interdisciplinary patient care plan as it pertains to Physical Therapy
4.	Assists the authorized practitioner in evaluating patients by applying diagnostic and prognostic muscle, nerve, joint and functional ability tests
5.	Treats patients to relieve pain, develop or restore functional and maintain maximum performance, using physical means, such as exercise, massage, heat, water, light and electricity
6.	Consults and familiarizes staff regarding the established plan
7.	Teaches, supervises, and counsels the family and patient in the physical therapy program
8.	Instructs patient in care and use of wheelchairs, braces, crutches, canes and prosthetic and orthotic devices
9.	Provides the equipment required to carry out the plan
10.	Arranges for out-patient service for procedures which cannot be given at home
11.	Records on a timely basis all evaluation data, treatments, and patient's response to therapy intervention also records Quality Improvement data
12.	Prepares an assessment of the patient's condition and periodic evaluations and summaries
13.	Supervises Physical Therapy Assistants and home health aides per agency policy
14.	Records and reports to the authorized practitioner the patient's reaction to treatment and /or any changes in the patient's condition
15.	Communicates with the authorized practitioner per agency policy or whenever changes occur regarding the patient's status and obtains additional orders, if necessary
16.	Communicates with other team members (including Home Health Aides) and, when appropriate, instructs them in certain phases of physical therapy that they may use while working with the patient
17.	Writes clinical/progress notes on the date of the visit and incorporates same in the patient's record per agency policy
18.	Participates in case conferences, physical therapy staff meetings, agency activities and in-service programs as necessary
19.	Supervises the home health aide per agency policy when physical therapy is the only professional discipline involved in the patient's care and supervises the Physical Therapy Assistant per agency policy
20.	Participates with staff, patients, and authorized practitioners in discharge planning activities and completes a Physical Therapy Discharge Summary

DUTIES	
Assists in the Agency Admission Process	
1.	Obtains a medical history, particularly as it relates to the present condition.
2.	Evaluates the patient, the patient's significant other(s) and the home situation to determine what instruction will be required, what assistance will be available from the family in caring for the patient, and what other Agency and community services will be required.
3.	Assures that the authorized practitioner's orders are appropriate and discusses necessary changes.
JOB CONDITIONS	
1.	The ability to drive and the ability to access patients' homes which may not be routinely wheelchair accessible are required.
2.	Hearing, eyesight and physical dexterity must be sufficient to perform a physical assessment of the patient's condition and to perform patient care.
3.	On occasion, may be required to bend, stoop, reach and move patient weight up to 250 pounds, lift and/or carry up to 30 pounds.
4.	Must be able to effectively communicate in English.
EQUIPMENT OPERATION	
Physical therapy equipment and other medical equipment will be utilized. Hand washing materials.	
COMPANY INFORMATION	
Has access to all patient medical records which may be discussed with the Director of Clinical Services, the Administrator, the Executive Director, and the Quality Improvement department staff.	
QUALIFICATIONS	
1.	Is currently licensed under chapter the state regulations.
2.	Must have at least one year of institutional/home health experience.
3.	Must have a criminal background clearance.
4.	Must have current CPR certification.
ACKNOWLEDGMENT	
Employee Signature	Date

TITLE OF POSITION: PHYSICAL THERAPY ASSISTANT

TITLE OF IMMEDIATE SUPERVISOR: REGISTERED PHYSICAL THERAPIST

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – HIGH

DUTIES	
Provides skilled Physical Therapy services on an intermittent basis to patients in their homes under direction from the Registered Physical Therapist in accordance with	
RESPONSIBILITIES	
1.	Under the direction of a Registered Physical Therapist (PT) provides patient services which have been delegated by the PT
2.	Under the supervision of the physical therapist as outlined in the state regulations, assists in evaluation and development of a rehabilitative plan of care and in periodic re-evaluation as required
3.	Participates in the preparation of clinical and daily progress notes, all notes are reviewed by the PT and submitted per agency policy, also prepares periodic summary reports
4.	As directed, performs routine strengthening and therapeutic exercises and assists the patient with the use of self-help devices
5.	Together with the PT, instructs the patient and family and assists in teaching exercise programs to the patient and family
6.	Identifies possible ineffective drug therapy or adverse reactions, significant side effects, drug allergies, and contra-indicated medication and reports same to the PT
7.	Observes, and records activities and findings in the clinical record and reports to the PT and the authorized practitioner the patient's reaction to treatment and any changes in the patient's condition
8.	Consults with PT regarding the outcome of home visits and reports physical findings in written form
9.	Under the supervision of the Physical Therapist, assists in evaluation and development of the rehabilitative plan of care and in periodic re-evaluation as indicated
10.	Instructs other health team personnel including, as appropriate, Home Health Aides, as well as family members with respect to certain phases of physical therapy, so that they may work with the patient
11.	Writes daily progress notes on patients, and submits same per agency policy, and prepares periodic summary reports as indicated
12.	Attends rehabilitation department staff meetings and case conferences
13.	Participates with staff, patients, and authorized practitioners in discharge activities and assists the Physical Therapist in completing the Physical Therapy Discharge Summary
14.	Confirms, on a weekly basis, the scheduling of visits with the Supervisor/Director of Clinical Services in order to coordinate Agency visits
15.	Assumes responsibility for self-development by continually striving to improve his/her health care knowledge through educational programs, attendance at workshops and conferences, active participation in professional and related organizations and individual research and reading
16.	Assumes other duties deemed appropriate and necessary by the PT and as designated by Registered Nurse Case Management and the Supervisor
ASSISTS IN THE AGENCY ADMISSION PROCESS	
1.	Obtains a medical history, particularly as it relates to the present condition.

2.	Evaluates the patient, the patient's significant other(s) and the home situation to determine what instruction will be required, what assistance will be available from the family in caring for the patient, and what other Agency and community services will be required.
3.	Assures that the authorized practitioner's orders are appropriate and discusses necessary changes.

JOB CONDITIONS	
1.	The ability to drive and the ability to access patients' homes which may not be routinely wheelchair accessible are required.
2.	Hearing, eyesight and physical dexterity must be sufficient to perform a physical assessment of the patient's condition and to perform patient care.
3.	On occasion, may be required to bend, stoop, reach and move patient weight up to 250 pounds; lift and/or carry up to 30 pounds.
4.	Must be able to effectively communicate in English.

EQUIPMENT OPERATION
Physical therapy equipment and other medical equipment will be utilized. Hand washing materials.

COMPANY INFORMATION
Has access to all patient medical records which may be discussed with the Director of Clinical Services, the Administrator, the Executive Director, and the Quality Improvement department staff.

QUALIFICATIONS	
1.	The Physical Therapy Assistant (PTA) is licensed as outlined in the state regulations.
2.	Must have a criminal background clearance.
3.	Must have current CPR certification.

ACKNOWLEDGMENT	
Employee Signature	Date

TITLE OF POSITION: OCCUPATIONAL THERAPIST

TITLE OF IMMEDIATE SUPERVISOR: Director of Clinical Services

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – HIGH

DUTIES	
Provides skilled occupational therapy service on an intermittent basis to patients in their homes in accordance with the authorized practitioner's orders in accordance with the state regulations.	
RESPONSIBILITIES	
1.	Assists the authorized practitioner in evaluating the patient's level of function by applying diagnostic and prognostic procedures
2.	Conducts initial and ongoing comprehensive assessments of the patient's needs
3.	Evaluates the patient's significant other(s), and the home situation to determine what instruction will be required and what family assistance will be available in caring for the patient, as well as what other Agency and community services will be required
4.	Assures that the authorized practitioner's orders are appropriate and discusses necessary changes
5.	Assists in development and implementation of the interdisciplinary patient care plan to include Occupational Therapy in consultation with the authorized practitioner
6.	Guides and instructs the patient in prescribed therapeutic self-care and creative activities that are directed toward improving independence and physical and mental functioning
7.	Instructs patient in care and use of wheelchairs, braces, splints, and prosthetic and orthotic devices
8.	Teaches, supervises and counsels the family and patient in the total Occupational Therapy program and other related problems of the patient at home
9.	Establishes goals based on the patient's current level of functioning and potential for improvement and performs re-evaluations as indicated
10.	Makes arrangements for outpatient services when procedures are required that cannot be given in the home
11.	Evaluates the effectiveness of occupational therapy intervention and updates the patient care plan as needed
12.	Prepares clinical/progress notes on the day of the visit which are incorporated into the chart per agency policies
13.	Communicates with the authorized practitioner on a regular basis regarding patient's status, obtaining additional orders, if necessary
14.	Communicates with other team members and, when appropriate, instructs them in Occupational Therapy techniques that they may use while working with the patient
15.	Supervises the Home Health Aide per agency policy when Occupational Therapy is the only professional discipline involved in the patient's care
16.	Supervises the Occupational Therapy Assistant per agency policy
17.	Participates in Agency activities, in-services, and meetings, when appropriate
18.	Confirms, on a weekly basis, the scheduling of visits with the Director of Clinical Services in order to coordinate Agency visits
19.	Participates with the staff, the patient, and the authorized practitioner in discharge activities and completes an Occupational Therapy Discharge Summary

20.	Assumes responsibility for self-development by continually striving to improve his/her health care knowledge through educational programs, attendance at workshops and conferences, active participation in professional and related organizations and individual research and reading

ASSISTS IN THE AGENCY ADMISSION PROCESS	
1.	Obtains a medical history, particularly as it relates to the present condition.
2.	Evaluates the patient, the patient's significant other(s) and the home situation to determine what instruction will be required, what assistance will be available from the family in caring for the patient, and what other Agency and community services will be required.
3.	Assures that the authorized practitioner's orders are appropriate and discusses necessary changes.

JOB CONDITIONS	
1.	The ability to drive and the ability to access patients' homes which may not be routinely wheelchair accessible are required.
2.	Hearing, eyesight and physical dexterity must be sufficient to perform a physical assessment of the patient's condition and to perform patient care.
3.	On occasion, may be required to bend, stoop, reach and move patient weight up to 250 pounds; lift and/or carry up to 30 pounds.
4.	Must be able to effectively communicate in English.

EQUIPMENT OPERATION
Occupational therapy equipment and other medical equipment will be utilized. Hand washing materials.

COMPANY INFORMATION
Has access to all patient medical records which may be discussed with the Director of Clinical Services, the Administrator, the Executive Director, and the Quality Improvement department staff and government agencies.

QUALIFICATIONS	
1.	The Occupational Therapist (OT) is currently licensed under the state regulations.
2.	Preferred to have at least one year of institutional/home health experience.
3.	Must have a criminal background clearance.
4.	Must have current CPR certification.

ACKNOWLEDGMENT	
Employee Signature	Date

TITLE OF POSITION: OCCUPATIONAL THERAPY ASSISTANT

TITLE OF IMMEDIATE SUPERVISOR: OCCUPATIONAL THERAPIST

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – HIGH

DUTIES OF POSITION	
Provides skilled occupational therapy service on an intermittent basis to patients in their homes under the supervision of the Occupational Therapist.	
POSITION RESPONSIBILITIES	
1.	Under the direction of the Occupational Therapist (OT) provides patient care services which have been delegated by the Occupational Therapist
2.	Under the direction of the OT assists in evaluation and development of the rehabilitative plan of care and in periodic re-evaluation as required
3.	Participates in the preparation of clinical and daily progress notes. All notes are reviewed by the OT and submitted per agency policy. Also prepares periodic summary reports
4.	Together with the OT, instructs the patient and family, assists in teaching them exercise programs
5.	Guides and instructs the patient in prescribed therapeutic self-care and creative activities that are directed toward improving independence and physical and mental functioning and assists the patient in the use of self-help devices
6.	Identifies possible ineffective drug therapy or adverse reactions, significant side effects, drug allergies, and contra-indicated medication and reports same to the OT
7.	Observes, and records activities and findings in the clinical record and reports to the OT and the authorized practitioner the patient's reaction to treatment and any changes in the patient's condition
8.	Consults with OT regarding the outcome of home visits and reports physical findings in written form
9.	Under the supervision of the OT, assists in evaluation and development of the rehabilitative plan of care and in periodic re-evaluation as indicated
10.	Instructs other health team personnel including, as appropriate, Home Health Aides, as well as family members with respect to certain phases of occupational therapy, so that they may work with the patient
11.	Writes daily progress notes on patients, and submits same per agency policy, and prepares periodic summary reports as indicated
12.	Attends rehabilitation department staff meetings and case conferences
13.	Instructs patient in care and use of wheelchairs, braces, splints, and prosthetic and orthotic devices
14.	Participates with staff, patients, and authorized practitioners in discharge activities and assists the Occupational Therapist in completing the Occupational Therapy Discharge Summary
15.	Confirms, on a weekly basis, the scheduling of visits with the Supervisor/Director of Clinical Services in order to coordinate Agency visits
16.	Assumes responsibility for self-development by continually striving to improve his/her health care knowledge through educational programs, attendance at workshops and conferences, active participation in professional and related organizations and individual research and reading
17.	Assumes other duties deemed appropriate and necessary by the OT
JOB CONDITIONS	

1.	The ability to drive and the ability to access patients' homes which may not be routinely wheelchair accessible are required.
2.	Hearing, eyesight and physical dexterity must be sufficient to perform a physical assessment of the patient's condition and to perform patient care.
3.	On occasion, may be required to bend, stoop, reach and move patient weight up to 250 pounds; lift and/or carry up to 30 pounds.
4.	Must be able to effectively communicate in English.

EQUIPMENT OPERATION

Occupational therapy equipment and other medical equipment will be utilized. Hand washing materials.

COMPANY INFORMATION

Has access to all patient medical records which may be discussed with the Director of Clinical Services, the Administrator, the Executive Director, and the Quality Improvement department staff and government agencies.

QUALIFICATIONS

1.	The Occupational Therapy Assistant (OTA) is currently licensed as outlined in the state regulations.
2.	Must have a criminal background clearance.
3.	Must have current CPR certification.

ACKNOWLEDGMENT

Employee Signature Date

TITLE OF POSITION: SPEECH THERAPIST

TITLE OF IMMEDIATE SUPERVISOR: Director of Clinical Services

DUTIES	
Provides skilled speech pathology service on an intermittent basis to patients in their homes in accordance with the authorized practitioner's orders in accordance with the state regulations.	
RESPONSIBILITIES	
1.	Evaluates the patient's level of functioning and recommends mechanisms to enhance the patient's hearing ability
2.	Provides initial and ongoing comprehensive assessments of the patient’s needs
3.	Establishes a Speech Therapy treatment plan in consultation with the authorized practitioner and revises it with the approval of the authorized practitioner as necessary
4.	Assists in development and implementation of the interdisciplinary patient care plan as it pertains to Speech Therapy
5.	Assures that the authorized practitioner's orders are appropriate and discusses necessary changes
6.	Communicates with other team members and, when appropriate, instructs them in speech pathology techniques that they may use while working with the patient
7.	Teaches, supervises, and counsels the family and patient in the total Speech Therapy program and other related problems of the patient at home
8.	Evaluates the patient, his/her significant other(s) and the home situation to determine what instruction will be required, what assistance will be available for the family in caring for the patient, and what other agency and community services will be required
9.	Makes arrangements for out-patient services for procedures which cannot be given in the home
10.	Records on a timely basis all evaluation data, treatments, and patient's response to therapy intervention
11.	Supervises the Home Health Aide per agency policy when Speech Therapy is the only professional discipline involved in the patient's care
12.	Records and reports to the authorized practitioner the patient's reaction to treatment and /or any changes in the patient's condition
13.	Participates in the development of periodic revision of the authorized practitioner's plan of treatment keeping him/her informed of the patient's status, and obtaining additional orders if necessary
14.	Communicates with the authorized practitioner per agency policy or whenever changes occur regarding the patient’s status and obtains additional orders, if necessary
15.	Evaluates the effectiveness of speech pathology intervention and updates the patient care plan as needed.
16.	Writes clinical/progress notes on the date of the visit and incorporates same in the patient's record per agency policy
17.	Assumes responsibility for self-development by continually striving to improve his/her health care knowledge through educational programs, attendance at workshops and conferences, active participation in professional and related organization meetings and individual research and reading
18.	Confirms, on a weekly basis, the scheduling of visits with the Director of Clinical Services in order to coordinate the patient’s visit schedule
19.	Participates in agency activities, case conferences and meetings, when appropriate

20.	Participates with staff, patients, and authorized practitioners in discharge planning activities and completes a Speech Therapy Discharge Summary

ASSISTS IN AGENCY ADMISSION PROCESS	
1.	Obtains a medical history, particularly as it relates to the present condition.
2.	Evaluates the patient, and the home situation to determine what instruction and assistance will be available from the family in caring for the patient, and what other Agency and community services will be required.
3.	Assures that the authorized practitioner's orders are appropriate and discusses necessary changes.

JOB CONDITIONS	
1.	The ability to drive and the ability to access patients' homes which may not be routinely wheelchair accessible are required.
2.	Hearing, eyesight and physical dexterity must be sufficient to perform a physical assessment of the patient's condition and to perform patient care.
3.	On occasion, may be required to bend, stoop, reach and move patient weight up to 250 pounds; lift and/or carry up to 30 pounds.
4.	Must be able to effectively communicate in English.

EQUIPMENT OPERATION
Speech therapy equipment and hand washing materials.

COMPANY INFORMATION
Has access to all patient medical records which may be discussed with the Director of Clinical Services, the Administrator, the Executive Director, and the Quality Improvement department staff and government agencies.

QUALIFICATIONS	
1.	The Speech Therapist (SLP) is currently licensed under chapter the state regulations.
2.	Preferred to have at least one year of institutional/home health experience.
3.	Must have a criminal background clearance.
4.	Must have current CPR certification.

ACKNOWLEDGMENT	
Employee Signature	Date

TITLE OF POSITION: NUTRITIONIST/DIETITIAN

TITLE OF IMMEDIATE SUPERVISOR: Director of Clinical Services

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – HIGH

DUTIES	
Directs, supervises, evaluates and provides nutritional services on an intermittent basis to patients in their homes in accordance with the authorized practitioner's orders	
RESPONSIBILITIES	
1.	Assesses and evaluates the nutritional status of patients
2.	Assists the authorized practitioner and nursing personnel to determine the dietary needs of the patient
3.	Prepares and calculates a therapeutic diet to meet specific requirements of the patient and confers with the authorized practitioner to establish dietary treatment
4.	Assists the patient and family to understand and follow dietary modifications ordered by the authorized practitioner. Instructs on dietary restrictions and drug/nutrient interactions as appropriate
5.	Observes, and reports to the authorized practitioner and nurse supervisor the patient's response to dietary treatment and any related changes in the patient's condition and revises nutritional plan as indicated by patient's condition
6.	Participates in care coordination activities, and acts as a resource to other health care staff, as requested
7.	Participates in Quality Improvement activities as assigned and communicates information that enables collection and analysis of data to identify opportunities for improvement
8.	Participates in Quality Improvement activities as assigned and communicates information that enables collection and analysis of data to identify opportunities for improvement
9.	Participates in the development of periodic revision of the authorized practitioner's plan of treatment keeping him/her informed of the patient's status, and obtaining additional orders if necessary
10.	Communicates with other team members and, when appropriate, instructs them in nutritional, dietary teaching that they may use while working with the patient
11.	Maintains and submits written clinical and agency records as deemed necessary
12.	Participates with staff, patient, and authorized practitioner in discharge activities and completes a Dietary Discharge Summary
13.	Assumes responsibility for self-development by continually striving to improve his/her health care knowledge through educational programs, attendance at workshops and conferences, active participation in professional and related organization meetings and individual research and reading
14.	Confirms, on a weekly basis, the scheduling of visits with the Director of Clinical Services to coordinate Agency visits
15.	Performs additional duties assigned in response to patient and/or Agency needs
ASSISTS IN THE ADMISSION PROCESS	
1.	Obtains a medical history, particularly as it relates to the present condition.
2.	Evaluates the patient, the patient’s significant other(s) and the home situation to determine what instruction will be required, what assistance will be available from the family in caring for the patient, and what other Agency and community services will be required.
3.	Assures that the authorized practitioner’s orders are appropriate and discusses necessary changes.

JOB CONDITIONS	
1.	The ability to drive and the ability to access patients' homes which may not be routinely wheelchair accessible are required.
2.	Hearing, eyesight and physical dexterity must be sufficient to perform a physical assessment of the patient's condition and to perform patient care.
3.	On occasion, may be required to bend, stoop, reach and move patient weight up to 250 pounds; lift and/or carry up to 30 pounds.
4.	Must be able to effectively communicate in English.

EQUIPMENT OPERATION
Use of nutritional teaching literature; Hand washing materials

COMPANY INFORMATION
Has access to all patient medical records which may be discussed with the Director of Clinical Services, the Administrator, the Executive Director, and the Quality Improvement department staff and government agencies.

QUALIFICATIONS
The Dietitian or Nutritionist will be: Certified under the state regulations or, Registered by the Academy of Nutrition and Dietetics as a registered dietitian nutritionist or, Certified by the board for certification of nutrition specialists as a certified nutrition specialist or, Certified by the American Clinical Board of Nutrition as a diplomate of the American Clinical Board of Nutrition.
Must have at least one year of experience.
Must have a criminal background clearance.
Must have current CPR certification.

ACKNOWLEDGMENT	
Employee Signature	Date

TITLE OF POSITION: RESPIRATORY THERAPIST

TITLE OF IMMEDIATE SUPERVISOR: Director of Clinical Services

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – HIGH

DUTIES	
Provides skilled respiratory therapy service on an intermittent basis to patients in their homes in accordance with the authorized practitioner's orders and in accordance with the state regulations.	
RESPONSIBILITIES	
1.	Assesses the patient's condition and level of function to determine appropriateness of equipment provided.
2.	Assesses the home environment including safety, storage facilities, cleanliness and ability to support the equipment.
3.	Participates with the authorized practitioner and Agency personnel in development of the interdisciplinary care plan as it pertains to respiratory therapy. Prepares an assessment of the patient's condition and periodic evaluations and summaries.
4.	Provides prescribed equipment and instructs patient/family in care and use of respiratory equipment.
5.	Consults and trains patient and caregiver regarding use of respiratory equipment, supplies and techniques. Teaches, supervises, and counsels the family and patient in the respiratory therapy program.
6.	Participate in care coordination activities, and act as a resource to other health care staff, as requested.
7.	Participate in Quality Improvement activities as assigned and communicate information that enables collection and analysis of data to identify opportunities for improvement.
8.	Provides technical assistance with respiratory equipment. Monitors the continued effectiveness, safety and appropriateness of the respiratory equipment.
9.	Participate in the development of periodic revision of the authorized practitioner's plan of treatment keeping him/her informed of the patient's status and obtaining additional orders if necessary.
10.	Communicate with other team members and, when appropriate, instructs them in respiratory teaching that they may use while working with the patient.
11.	Maintain and submit written clinical and agency records as deemed necessary. Records on a timely basis all evaluation data, treatments, and patient's response to therapy intervention and quality assurance data.
12.	Participate with staff, patient, and authorized practitioner in discharge activities and complete a Discharge Summary.
13.	Assume responsibility for self-development by continually striving to improve his/her health care knowledge through educational programs, attendance at workshops and conferences, active participation in professional and related organization meetings and individual research and reading.
14.	Participates in case conferences, staff meetings, agency activities and in-service programs as appropriate. Follows company policies and procedures.
15.	Performs Pulse Oximetry testing when ordered by the authorized practitioner.
ASSISTS IN THE ADMISSION PROCESS	
1.	Obtain a medical history, particularly as it relates to the present condition.
2.	Evaluate the patient, the patient’s significant other(s) and the home situation to determine what instruction will be required, what assistance will be available from the family in caring for the patient, and what other Agency and community services will be required.

3.	Assure that the authorized practitioner's orders are appropriate and discuss necessary changes.

JOB CONDITIONS
The ability to drive and the ability to access patients' homes which may not be routinely wheelchair accessible are required.
Hearing, eyesight and physical dexterity must be sufficient to perform a physical assessment of the patient's condition and to perform patient care.
On occasion, may be required to bend, stoop, reach and move patient weight up to 250 pounds; lift and/or carry up to 30 pounds.
Must be able to effectively communicate in English.
EQUIPMENT OPERATION
Respiratory therapy equipment and other medical equipment will be utilized. Use of respiratory teaching literature and hand washing materials.
COMPANY INFORMATION
Has access to all patient medical records which may be discussed with the Director of Clinical Services, the Administrator, the Executive Director, and the Quality Improvement committee.
QUALIFICATIONS
The Respiratory Therapist (RT) is currently licensed under chapter 18.89 RCW.
Preferred to have at least one year of experience.
Must have a criminal background clearance.
Must have current CPR certification.
ACKNOWLEDGMENT
Employee Signature Date

TITLE OF POSITION: HOME HEALTH AIDE

TITLE OF IMMEDIATE SUPERVISOR: Registered Nurse

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – HIGH

DUTIES	
1.	Home health aide services are services provided under the supervision of an RN, PT. OT or ST.
2.	The Home Health Aide (HHA) is an individual who is a nursing assistant certified or nursing assistant registered under the state regulations.
3.	HHA services may also be provided by an LPN or RN as needed.
4.	Provides personal care and related services in the home, under the direction, instruction and supervision of the RN, PT, OT or ST.
5.	RN delegated tasks to be performed by an HHA must be assigned by and performed under the supervision of an RN who will be responsible for the patient care provided by the HHA.
RESPONSIBILITIES	
1.	Assists the patient with ambulation as outlined in the care plan.
2.	Assists the patient with exercise as outlined in the care plan.
3.	Assists the patient with self-administered medications as outlined in the care plan.
4.	Reports changes in patient condition and needs to the supervisor.
5.	Completes appropriate records per agency policy.
6.	Provide personal care to the patient as outlined in the care plan.
7.	Completes homemaker services as outlined in the care plan.
8.	Completes other tasks as outlined on the patient care plan.
JOB CONDITIONS	
1.	The ability to drive and the ability to access clients' homes which may not be routinely wheelchair accessible are required.
2.	Hearing, eyesight and physical dexterity must be sufficient to perform a physical assessment of the client's condition and to perform client care.
3.	On occasion, may be required to bend, stoop, reach and move client weight up to 250 pounds; lift and/or carry up to 30 pounds.
4.	Must be able to communicate clearly, both verbally and in writing in English.
EQUIPMENT OPERATION	
Use of BP cuff, thermometer and stethoscope. Hand washing materials.	
COMPANY INFORMATION	
Has access to all patient medical records which may be discussed with the Registered Nurse and the Supervisor.	
QUALIFICATIONS	
1.	The Home Health Aide (HHA) is a nursing assistant currently certified or registered under the state regulations.
2.	Must have current CPR certification.
3.	Must have criminal background clearance.

ACKNOWLEDGMENT

Employee Signature	Date

TITLE OF POSITION: STAFFING COORDINATOR

TITLE OF IMMEDIATE SUPERVISOR: Director of Clinical Services

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – LIMITED

DUTIES OF POSITION	
1.	Is responsible for the coordination of patient visits, maintenance and upkeep of scheduling records and logbooks.
2.	Staffing coordination duties include the accurate and timely communication of scheduling changes between office and field staff.
POSITION RESPONSIBILITIES	
1.	Maintains a current patient roster with necessary information
2.	Updates logbooks and schedule sheets
3.	Prepares weekly schedule for field staff and provides copies for distribution
4.	Assists in coordinating services provided to patients
5.	Controls and monitors schedule changes
6.	Checks compliance of visits done (on master schedule) after all notes are matched to charges, brings any scheduling problems to the supervisor's attention immediately
7.	Assists in taking referrals, makes copies of referral sheets for Payroll/Billing Manager and on-call coordinator when requested
8.	Calls hospital Social Services department to notify them that an Agency patient has been admitted to their hospital
9.	Obtains weekly visit count and report results to the Director of Clinical Services
10.	Assists in relaying messages to field staff, office staff and community liaisons, gives and takes reports from the on-call coordinator
11.	Performs other duties as assigned by the Supervisor, DCS or Administrator
JOB CONDITIONS	
1.	Position is stressful in terms of meeting deadlines.
2.	It is primarily a desk job which essentially involves sitting, standing, stooping and walking, as well as an inordinate amount of telephone communication.
3.	Travel is required, by car or airplane to local, out-of-town or state seminars, conferences or meetings.
4.	It requires minimal lifting of office records and printouts.
5.	The ability to read 12 point and larger types is required.
6.	One must be able to hear adequately on the telephone with no more than an amplifier and be able to communicate both verbally and in writing in English.
EQUIPMENT OPERATION	
The job requires the ability to utilize a PC, calculator, multi- line telephone, and other related office equipment.	
COMPANY INFORMATION	
Has access to all patient medical records which may be discussed with management.	

QUALIFICATIONS	
1.	High school diploma or equivalent.
2.	Preferably an LPN with a Home Health experience.
3.	Should be a skilled organizer able to manage office files, logbooks and staff schedules.
4.	Must possess light secretarial skill and have a polite telephone manner.
5.	Must have a criminal background clearance.

ACKNOWLEDGMENT

Employee Signature Date

TITLE OF POSITION: MEDICAL RECORDS MANAGER

TITLE OF IMMEDIATE SUPERVISOR: Administrator

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – LIMITED

DUTIES	
Is responsible for maintaining records for past and present patients.	
RESPONSIBILITIES	
1.	Supervise Medical Records department clerical staff
2.	Maintain medical records in locked cabinets to assure confidentiality of patient information
3.	Set up new patient clinical records
4.	File documentation on patient charts in chronological order in appropriate locations, weekly
5.	Monitor check-in and check-out of patient charts by office and field staff
6.	Assist staff in locating charts and documentation
7.	Log in and out authorized practitioner orders and mail orders to the appropriate authorized practitioners
8.	Develop and implement systems to assure organization and timely retrieval of documentation
9.	Break down discharged patients' records and give them to the QAPI Department for review discharge
10.	Maintain and store all discharged patient charts
11.	Assist with the orientation of new employees as requested
12.	Perform other duties as assigned by the Administrator
JOB CONDITIONS	
1.	Position is stressful in terms of meeting deadlines.
2.	It is primarily a desk job which essentially involves sitting, standing, stooping and walking, as well as an inordinate amount of telephone communication.
3.	Travel is required, by car or airplane to local, out-of-town or state seminars, conferences or meetings.
4.	It requires minimal lifting of office records and printouts.
5.	The ability to read 12 point and larger types is required.
6.	One must be able to hear adequately on the telephone with no more than an amplifier and be able to communicate both verbally and in writing in English.
EQUIPMENT OPERATION	
The job requires the ability to utilize a PC, calculator, multi-line telephone, and other related office equipment.	
COMPANY INFORMATION	
Has access to all patient medical records which may be discussed with management.	
QUALIFICATIONS	
1.	High school diploma or equivalent.
2.	Experience as a record manager.
3.	Should be a skilled organizer able to manage office files, logbooks and staff schedules.
4.	Must possess light secretarial skill and have a polite telephone manner.

ACKNOWLEDGMENT

Employee Signature	Date

TITLE OF POSITION: OFFICE MANAGER

TITLE OF IMMEDIATE SUPERVISOR: ADMINISTRATOR

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – LIMITED

DUTIES
Oversees the operations of the office, including maintaining current employee personnel files, answering all phone calls, preparing and maintaining the Agency's payroll system, and compiling statistics necessary for the Administrator.

RESPONSIBILITIES	
1.	Maintains all staff and contract personnel files
2.	Maintains master files of personnel evaluations and makes copies on monthly basis for the appropriate department supervisor
3.	Keeps all evaluation forms current
4.	Sends deficiency notices to personnel
5.	Acts as receptionist for the office, answering inquiries of general nature form applicants, visitors, and professional staff, assisting them in a friendly and cooperative manner
6.	Assists in miscellaneous bookkeeping functions
7.	Submits bills to appropriate health care agencies
8.	Assists the full-time and part-time payroll procedures
9.	Assumes various duties as directed by the Administrator in the area of personnel and accounting

JOB CONDITIONS	
1.	Position is stressful in terms of meeting deadlines.
2.	It is primarily a desk job which essentially involves sitting, standing, stooping and walking, as well as an inordinate amount of telephone communication.
3.	Travel is required, by car or airplane to local, out-of-town or state seminars, conferences or meetings.
4.	It requires minimal lifting of office records and printouts.
5.	The ability to read 12 point and larger types is required.
6.	One must be able to hear adequately on the telephone with no more than an amplifier and be able to communicate both verbally and in writing in English.

EQUIPMENT OPERATION
The job requires the ability to utilize a PC, calculator, multi-line telephone, and other related office equipment.

COMPANY INFORMATION
Has access to all patient medical records and patient financial accounts, personnel records and company financial records, which may be discussed with all management staff, including Governing Body.

QUALIFICATIONS	
1.	High school diploma or equivalent.
2.	Experience as a coordinator or office manager, preferably in the health field.
3.	Should be a skilled organizer able to manage office files, logbooks and staff schedules.

4.	Must possess light secretarial skill and have a polite telephone manner.

ACKNOWLEDGMENT

Employee Signature Date

TITLE OF POSITION: PAYROLL/BILLING/RECEIVABLES MANAGER

TITLE OF IMMEDIATE SUPERVISOR: ADMINISTRATOR

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – LIMITED

DUTIES	
Manages the Agency's payroll, bill payments and receivable collections	
RESPONSIBILITIES	
1.	Supervises staff payroll, accounts payable and third-party billing
2.	Posts workers' salaries on appropriate records, prepares all information needed to prepare checks and payroll reports, verifies all payroll deductions and reconciles cash payments related to those deductions
3.	Processes accounts payable according to Agency policy, posts expense and income to appropriate accounts, reviews all phone charges monthly
4.	Assists in preparation of entry forms for general ledger
5.	Posts all payments in cash payments journal
6.	Bills outside agencies and third-party payers
7.	Supervises and coordinates credit and collection to include completion of all necessary paperwork and data entry
8.	Manages credit and collection activities on past due accounts participates in any and all collection activities of problem accounts
9.	Supervises the completion of all required insurance forms in a timely manner
10.	Supervises and follows up on receiving signed service agreements and deposits
11.	Reviews and analyzes all financial reports with the Administrator
12.	Assists with audits
13.	Verifies and assures time slips are signed and in the office on time, assures there are clinical notes for each entry on the time slips
14.	Takes care of any necessary payroll and billing adjustments
15.	Assures the correct data entry of patient information and payroll and billing information into the system
JOB CONDITIONS	
1.	Position is stressful in terms of meeting deadlines.
2.	It is primarily a desk job which essentially involves sitting, standing, stooping and walking, as well as an inordinate amount of telephone communication.
3.	Travel is required, by car or airplane to local, out-of-town or state seminars, conferences or meetings.
4.	It requires minimal lifting of office records and printouts.
5.	The ability to read 12 point and larger types is required.
6.	One must be able to hear adequately on the telephone with no more than an amplifier and be able to communicate both verbally and in writing in English.
EQUIPMENT OPERATION	
The job requires the ability to utilize a PC, calculator, multi-line telephone, and other related office equipment.	

COMPANY INFORMATION	
Has access to all patient financial accounts, personnel records and company financial records, which may be discussed with all management staff, including the Governing Body.	
QUALIFICATIONS	
1.	High school diploma or equivalent.
2.	Experience as a payable manager, preferably in the health field.
3.	Should be a skilled organizer able to manage office files, logbooks and “T accounts”.
4.	Must possess light secretarial skill and have a polite telephone manner.
ACKNOWLEDGMENT	
Employee Signature	Date

TITLE OF POSITION: PAYROLL/BILLING/RECEIVABLES CLERK

TITLE OF IMMEDIATE SUPERVISOR: PAYROLL/BILLING/RECEIVABLES MANAGER

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – LIMITED

DUTIES	
Assists the Payroll/Billing/Receivables Manager in all aspects of payroll and billing.	
RESPONSIBILITIES	
1.	Prepares and submits payroll and billing
2.	Assists with accounts receivable and collection activity
3.	Complies and completes all statistical reports as requested by the Manager
4.	Prepares and submits cash receipt reports, employee invoices, and mileage sheets
5.	Compiles and completes patient visit log on a weekly basis
6.	Compiles and completes monthly visit report
7.	Prepares and submits supply lists for billing
8.	Performs other duties as assigned by the Manager
JOB CONDITIONS	
1.	Position is stressful in terms of meeting deadlines.
2.	It is primarily a desk job which essentially involves sitting, standing, stooping and walking, as well as an inordinate amount of telephone communication.
3.	It requires minimal lifting of office records and printouts.
4.	The ability to read 12 point and larger types is required.
5.	One must be able to hear adequately on the telephone with no more than an amplifier and be able to communicate both verbally and in writing in English.
EQUIPMENT OPERATION	
The job requires the ability to utilize a PC, calculator, multi-line telephone, and other related office equipment.	
COMPANY INFORMATION	
Has access to all patient financial accounts, personnel records, which may be discussed with all management staff, including the Governing Body.	
QUALIFICATIONS	
1.	High school diploma or equivalent.
2.	Minimum of one-year experience in bookkeeping or related field or evidence of concentrated training.
3.	Must be able to handle heavy telephone work.
4.	Must be able to type 40 words per minute accurately.

ACKNOWLEDGMENT	
Employee Signature	Date

TITLE OF POSITION: BUSINESS CLERK

TITLE OF IMMEDIATE SUPERVISOR: OFFICE MANAGER OR ADMINISTRATOR

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – LIMITED

DUTIES	
Performs routine office clerical tasks.	
RESPONSIBILITIES	
1.	Does all necessary copying and all necessary filing
2.	Opens mail, sorts it and distributes it to the proper person
3.	Tracks verbal orders and all forms signed by authorized practitioners, patients and staff
4.	Mails all in-office mail including all plans of care, verbal order forms to appropriate authorized practitioners and notifies the DCS of their return
5.	Inputs patient and staff schedules on the computer and inputs new patient and new employee information
6.	Maintains confidentiality of all office and record information
7.	Performs other duties as assigned
JOB CONDITIONS	
1.	Position is stressful in terms of meeting deadlines.
2.	It is primarily a desk job which essentially involves sitting, standing, stooping and walking, as well as an inordinate amount of telephone communication.
3.	It requires minimal lifting of office records and printouts.
4.	The ability to read 12 point and larger types is required.
5.	One must be able to hear adequately on the telephone with no more than an amplifier and be able to communicate both verbally and in writing in English.
EQUIPMENT OPERATION	
The job requires the ability to utilize a PC, calculator, multi-line telephone, and other related office equipment.	
COMPANY INFORMATION	
Has access to all patient medical and financial records, which may be discussed with all management staff, including the Governing Body.	
QUALIFICATIONS	
1.	High school graduate or equivalent.
2.	Should have knowledge of office manager procedures and bookkeeping.
3.	Needs to be able to type accurately, use a computer and office machines.

ACKNOWLEDGMENT	
Employee Signature	Date

TITLE OF POSITION: COORDINATOR

TITLE OF IMMEDIATE SUPERVISOR: Director of Clinical Services

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – LIMITED

DUTIES	
Directs the coordination of patient care under the supervision of the Director of Clinical Services	
RESPONSIBILITIES	
1.	Coordinates patient care among disciplines
2.	Confers with Director of Clinical Services on problems and complaints
3.	Liaison between coordinating team and the office
4.	Performs all coordinating duties when necessary
5.	Reviews and directs case management notes, documenting change of aide, change in service, telephone complaints, etc.
6.	Supervises the completion of intake forms and their appropriate routing for admission approval
7.	Supervises the scheduling of appropriately skilled person to fill the case; the in-putting of the scheduling information on the computer and continue to supervise the coordination of the scheduling
8.	Supervises the scheduling of replacement workers as needed and the input of the scheduling information
9.	Communicates with patients/families on care related issues
10.	Interview, test and check references of field staff being considered for hire and supervises the set-up of personnel files
11.	Takes telephone referral information
12.	Handles matters related to staff and the patient they serve
13.	Supervises the maintenance of case management notes related to personal problems and/ other situations that arise with staff including those relating to patient care
14.	Communicates on a routine basis with contract agencies to maintain smooth relations and to obtain needed information
15.	Advises the Director of Clinical Services of all new cases to be filled and supervises the review of cases on a weekly basis
16.	Performance of other duties as required
JOB CONDITIONS	
1.	Position is stressful in terms of meeting deadlines.
2.	It is primarily a desk job which essentially involves sitting, standing, stooping and walking, as well as an inordinate amount of telephone communication.
3.	It requires minimal lifting of office records and printouts.
4.	The ability to read 12 point and larger types is required.
5.	One must be able to hear adequately on the telephone with no more than an amplifier and be able to communicate both verbally and in writing in English.
EQUIPMENT OPERATION	

The job requires the ability to utilize a PC, calculator, multi-line telephone, and other related office equipment.
COMPANY INFORMATION
Has access to all patient medical and financial records, which may be discussed with all management staff.
QUALIFICATIONS

1.	A Washington licensed nurse is preferred.
2.	A minimum of one-year experience as a Coordinator, preferably in a home health setting.
3.	A good working knowledge of home health care.

ACKNOWLEDGMENT
Employee Signature Date

TITLE OF POSITION: UNLICENSED INTAKE COORDINATOR

TITLE OF IMMEDIATE SUPERVISOR: Coordinator/ Director of Clinical Services

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – LIMITED

DUTIES OF POSITION	
Assists in the coordination of patient care under the direction of the Coordinator	
POSITION RESPONSIBILITIES	
1.	Answers phones, receives all intake information for paraprofessional cases and enter the appropriate information in the computer
2.	Completes intake forms and route them appropriately for admission approval
3.	Assists in the scheduling of appropriately skilled person to fill the case, inputs scheduling information on the computer and continues to assist in the coordination of the scheduling
4.	Schedules replacement workers as needed and input scheduling information
5.	Assists in communication with patients/families on home health aide related issues
6.	Assists in the establishment of personnel files and in the checking of references for paraprofessionals being considered for hire
7.	Takes telephone referral information if no nursing staff is available and passes referrals as soon as possible
8.	Handles matters related to staff and the patient they serve, with the exception of issues that require professional/clinical judgment
9.	Assists in the maintenance of case management notes related to personal problems and/other situations that arise with staff including those relating to patient care
10.	Communicates on a routine basis with contract agencies to maintain smooth relations and to obtain needed information
11.	Documents and reports changes in patient status, in service requirements, in and employee staff assignment and in particular, any unsatisfactory performance
12.	Performs other duties as directed
JOB CONDITIONS	
1.	Position is stressful in terms of meeting deadlines.
2.	It is primarily a desk job which essentially involves sitting, standing, stooping and walking, as well as an inordinate amount of telephone communication.
3.	It requires minimal lifting of office records and printouts.
4.	The ability to read 12 point and larger types is required.
5.	One must be able to hear adequately on the telephone with no more than an amplifier and be able to communicate both verbally and in writing in English.
EQUIPMENT OPERATION	
The job requires the ability to utilize a PC, calculator, multi- line telephone, and other related office equipment.	
COMPANY INFORMATION	
Has access to all patient medical and financial records, which may be discussed with all management.	
QUALIFICATIONS	

1.	High school graduate.
2.	Experience in a Home Health Care setting preferred.
3.	Must be computer literate and able maintain simple records in English.

ACKNOWLEDGMENT

Employee Signature Date

TITLE OF POSITION: COMMUNITY LIAISON

TITLE OF IMMEDIATE SUPERVISOR: ADMINISTRATOR

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – LIMITED

DUTIES OF POSITION	
Is responsible for interaction in the community and promoting company services.	
POSITION RESPONSIBILITIES	
1.	Analyzes the potential of the company's service area to determine target markets
2.	Visits Doctor' offices, hospital discharge desks, Assisted Living facilities and other possible sources of referrals to present Agency credentials and obtain patient referrals
3.	Analyzes the company's organization to determine its strengths and weaknesses
4.	Analyzes past and current marketing data
5.	Completes an analysis of the company's "product" line
6.	Analyzes patient/company relationships
7.	Develops sales/marketing objectives and sales projections
8.	Develops a marketing plan, identifying priorities and sets a reasonable timetable
9.	Implements marketing plan staying within established timetable
10.	Reviews and evaluates the analyses and plan on an established basis
JOB CONDITIONS	
1.	Position is stressful in terms of meeting deadlines.
2.	It is primarily a desk job which essentially involves sitting, standing, stooping and walking, as well as an inordinate amount of telephone communication.
3.	It requires minimal lifting of office records and printouts.
4.	Must have the ability to read 12 point and larger type, communicate clearly and be able to hear adequately on the telephone with no more than an amplifier.
EQUIPMENT OPERATION	
Must be able to use a PC, calculator, multi-line telephone, and other office equipment.	
COMPANY INFORMATION	
Has access to patient medical and financial records, which may be discussed with management staff.	
QUALIFICATIONS	
1.	At least 1-year experience in marketing, nursing or social work interacting with health agencies/professionals.
2.	Must be organized and detail oriented.
3.	Must be able to communicate effectively in writing, orally in English.
4.	Must be able to interact with health professionals at all levels.

ACKNOWLEDGMENT	
Employee Signature	Date

TITLE OF POSITION: RECEPTIONIST

TITLE OF IMMEDIATE SUPERVISOR: OFFICE MANAGER/ADMINISTRATOR

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – LIMITED

DUTIES OF POSITION	
Greets people, answers the phones, and does light clerical duties	
POSITION RESPONSIBILITIES	
1.	Greets people as they enter the Agency, answers calls, answers general information questions, screens visitors and delivers messages
2.	Types memos, correspondence, agendas and reports as required
3.	Assists with accounting functions as assigned, e.g., ledger entering, account processing, etc.
4.	Makes appointments and informs staff members of meetings
5.	Maintains files for the Agency
6.	Opens and sorts mail
7.	Does photocopying as required
8.	Performs other duties as required
JOB CONDITIONS	
1.	Position is stressful in terms of meeting deadlines.
2.	It is primarily a desk job which essentially involves sitting, standing, stooping and walking, as well as an inordinate amount of telephone communication.
3.	It requires minimal lifting of office records and printouts.
4.	The ability to read 12 point and larger types is required.
5.	One must be able to hear adequately on the telephone with no more than an amplifier and be able to communicate both verbally and in writing in English.
EQUIPMENT OPERATION	
The job requires the ability to utilize a PC, calculator, multi-line telephone, and other related office equipment.	
COMPANY INFORMATION	
Has access to all patient medical and financial records, which may be discussed with all management staff.	
QUALIFICATIONS	
1.	Must have a High school education.
2.	Must have a minimum of six months office experience.
3.	Must possess typing and other clerical skills.

ACKNOWLEDGMENT	
Employee Signature	Date

TITLE OF POSITION: DATA ENTRY OPERATOR

TITLE OF IMMEDIATE SUPERVISOR: OFFICE MANAGER/ADMINISTRATOR

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – LIMITED

DUTIES	
Has responsibility for inputting data to the computer terminal	
RESPONSIBILITIES	
1.	Inputs patient visit information to the Patient Management System on a timely basis and review data regularly
2.	Inputs and reviews the following clinical information on a timely basis to include, referrals, admissions, readmissions and reinstatements, care plans, verbal orders, re-certifications, discharge and patients not admitted
3.	Maintains confidentiality of patient medical information
4.	Is cross trained for all MIS functions
5.	Performs offices procedures, such as sorting and filing records and photocopying
6.	Prints outputs on a daily basis
7.	Performs other duties as directed
JOB CONDITIONS	
1.	Position is stressful in terms of meeting deadlines.
2.	It is primarily a desk job which essentially involves sitting, standing, stooping and walking, as well as an inordinate amount of telephone communication.
3.	It requires minimal lifting of office records and printouts.
4.	The ability to read 12 point and larger types is required.
5.	One must be able to hear adequately on the telephone with no more than an amplifier and be able to communicate both verbally and in writing in English.
EQUIPMENT OPERATION	
The job requires the ability to utilize a PC, calculator, multi-line telephone, and other related office equipment.	
COMPANY INFORMATION	
Has access to all patient medical and financial records, which may be discussed with all management staff.	
QUALIFICATIONS	
1.	High school education.
2.	Minimum of six months office experience.
3.	Must possess typing and other clerical skills.

ACKNOWLEDGMENT	
Employee Signature	Date

TITLE OF POSITION: COURIER/CLERK

TITLE OF IMMEDIATE SUPERVISOR: OFFICE MANAGER/ADMINISTRATOR

RISK OF EXPOSURE TO BLOODBORNE PATHOGENS – LIMITED

DUTIES	
Acts as a courier for the Agency delivery and picking up packages as needed provides other clerical support.	
RESPONSIBILITIES	
1.	Delivers and picks up at various locations correspondence, medical documentation, and supplies
2.	On a daily basis, delivers Plans of Care to the authorized practitioners, at their offices, for their signature, returns signed forms to supervisor
3.	Maintains logs and notes dates when POCs were mailed to authorized practitioners, the follow-up planned, the action taken, and the disposition
4.	Follows-up by telephone if authorized practitioners do not return POCs within seven days after mailing
5.	Sends second copy of POC to authorized practitioner if he/she fails to send original back within 7 days
6.	Maintains communication with the Office Manager and the DCS to plan further follow-up for outstanding orders or POCs. with all management staff, including the Administrator
JOB CONDITIONS	
1.	Position is stressful in terms of meeting deadlines.
2.	It is primarily a desk job which essentially involves sitting, standing, stooping and walking, as well as an inordinate amount of telephone communication.
3.	It requires minimal lifting of office records and printouts.
4.	The ability to read 12 point and larger type is required.
5.	One must be able to hear adequately on the telephone with no more than an amplifier and be able to communicate both verbally and in writing in English.
EQUIPMENT OPERATION	
The job requires the ability to utilize a PC, calculator, multi-line telephone, and other related office equipment.	
COMPANY INFORMATION	
Has access to all patient medical and financial records, which may be discussed with all management staff.	
QUALIFICATIONS	
1.	High school education.
2.	Minimum of six months office experience.

3. Must possess typing and other clerical skills.	
ACKNOWLEDGMENT	
Employee Signature	Date

RESPONSIBILITIES OF CONTRACTORS AND VOLUNTEERS

WAC 246-335-515

Applicability: This policy applies to all staff members, contractors, and volunteers of **Golden Years Home Health Supported Living LLC**, a skilled home care agency providing services in the state of Washington.

PURPOSE

This policy outlines the responsibilities of contractors and volunteers who provide services on behalf of the agency. It ensures they understand their roles, contribute to quality care, and comply with all applicable regulations as mandated by the state regulations.

Responsibilities

A. General Requirements

All contractors and volunteers will be required to:

1. **Meet Employment Criteria:**
   - Comply with **Golden Years Home Health Supported Living LLC**'s non-discrimination policy as outlined in Chapter 49.60 RCW.
   - Demonstrate qualifications relevant to their assigned tasks.
2. **Complete Application Process:**
   - Submit a formal application.
   - Provide professional references.
   - Undergo a background check in accordance with WAC 246-335-425(6).
3. **Sign Agreements:**
   - Sign a written agreement outlining:
     - Specific services to be provided.
     - Expected work hours.
     - Commitment to adhering to agency policies and procedures, including confidentiality.
4. **Attend Orientation and Training:**
   - Participate in an orientation to **Golden Years Home Health Supported Living LLC**, including our mission, values, and client population.
   - Receive training specific to their roles and responsibilities, ensuring competency in delivering safe and effective care.
   - Complete ongoing training opportunities as required.
5. **Supervision:**
   - Work under the supervision of a qualified staff member, with the level of supervision dependent on the complexity of the service provided.
   - Caregivers will always be directly supervised by the clinical manager.
6. **Client Care:**
   - Provide quality care that aligns with client needs and care plans.

- Respect client dignity, independence, and privacy.

7. **Communication:**
   - Maintain clear communication with agency staff.
   - Report any changes in client condition, concerns about client safety, or difficulties fulfilling assigned tasks.
8. **Compliance:**
   - Adhere to all applicable laws and regulations, including infection control protocols, HIPAA regulations, and **Golden Years Home Health Supported Living LLC**'s non-discrimination policy.
   - Maintain confidentiality of all client information.

B. Background Checks and Suitability Determinations (WAC 246-335-425(6) & (7))

1. **Background Checks:**
   - All contractors, volunteers, and anyone with direct contact with vulnerable clients will undergo a criminal history background check processed through the Washington State Patrol (WSP).
   - Disclosure statements will be completed and approved by the Department of Social and Health Services.
   - Background checks and disclosure statements will be renewed every two years.
2. **Suitability Determinations:**
   - If a background check reveals non-disqualifying convictions, pending charges, or negative actions, **Golden Years Home Health Supported Living LLC** will conduct a character, competence, and suitability determination.
   - This determination will consider factors outlined in WAC 246-335-425(7) to assess the individual's ability to provide safe and appropriate care.

SERVICES UNDER CONTRACT

WAC 246-335-525

Statement of Purpose

The applicant or licensee must develop and operationalize personnel, contractor, and volunteer policies and procedures that describe:

(3) References for personnel, contractors and volunteers.

Policy

1. Agency will use contracted personnel in instances when the type of work, working schedule, and duration of employment permit.
2. All contractors will have a personnel file that mirrors the direct hire employee.

Special Instructions

1. Written service contracts are signed and dated by Administrator and Contractor and are reviewed annually.
2. The contract includes:
    a. Duties to be performed
    b. Services to be provided.
    c. Assurance by the contractor of education, training, and qualifications needed to provide the service.
    d. Contractor parties may participate in Quality Improvement as applicable.
    e. Documentation of the client is submitted per contract.
    f. Bills for reimbursement are submitted per contract.
    g. The contractor will not bill the client for any services.
    h. Contractor is responsible to withhold necessary taxes.
    i. Contractor is responsible to follow all agency policies and procedures.
    j. The contract will have effective dates and terms for renewal and/or termination.

SERVICES TO BE PROVIDED

WAC 246-335-515

Statement of Purpose

The applicant or licensee must develop and implement a plan of operation which includes the (4) Services to be provided; (5) The days and hours of agency operation.

Principal Purpose of the Agency

1. The principal purpose of the Agency shall be to provide skilled nursing and other therapeutic and paraprofessional services in their place of residence.
2. At least one of the services shall be provided directly by Agency employees.
3. The Agency shall at all times comply with all applicable regulations of the United States Department of Health and Human Services, and all rules, regulations and guidelines as set forth by the state and local governments.
4. The Agency shall refuse neither service nor employment nor discriminate against any person because of age, race, color, creed, sex, national origin or handicap(s).

Definition of Term Care

1. Care will generally be provided for the time, duration and frequency specified by the Authorizing Practitioner and agreeable with the patient.
2. Care will be provided in the patient's place of residence.

Availability of Care

1. Services shall be available seven (7) days per week, twenty-four (24) hours per day.
2. Services shall be provided in the evening and on weekends and the Agency shall provide for an after-business hours answering service to handle inquiries and emergencies.
3. The Director of Clinical Services or designee must be available during all hours that patient care is being provided.

Services Provided

- RNs (LPN under the direction of RN)
- Home Health Aides
- Physical Therapy

- Occupational Therapy
- Speech Therapy
- Medical Social Worker
- Respiratory Therapy
- Nutritional/Dietitian

1. We also provide assistance in obtaining medical equipment and supplies, nutritional guidance, and laboratory & respiratory services.
2. Orders from the attending Authorizing Practitioner are required for performance of care and services except for home care aide, homemaker, chore or companion services only, if the same are provided by the Agency.
3. Care must conform to current standards of practice for the respective discipline and should be reasonable and necessary to the treatment of a medical or psychiatric disorder.

Remuneration Policy

1. This Agency prohibits any illegal remuneration for securing or soliciting patients or patronage.
2. The Agency ensures that clinical decisions regarding the provision of care are not compromised by the provision of incentives to staff be the incentive financial or otherwise.
3. This is monitored closely by our Quality Improvement Program and as part of our Clinical Record Review Committee.

MANAGEMENT AND SUPERVISION

WAC 246-335-515

Statement of Purpose

The applicant or licensee must develop and implement a plan of operation which includes:

(6) Criteria for management and supervision of home health services throughout all approved service areas, which include:

(b) How supervision of personnel and volunteers and monitoring of services provided by contractors will occur which meet the requirements of WAC 246-335-545;

(c) How performance evaluations for personnel and volunteers and evaluation of services provided by contractors will be conducted per WAC 246-335-525 (16) and (17).

Administrator

1. The Governing Body shall appoint a full-time Administrator who shall have full authority for management of Agency operations.
2. The Administrator must be a home care employee.
3. The Administrator is responsible to:
    a. Oversee the day-to-day operation and fiscal affairs of the agency;
    b. Implement the state regulations;
    c. Designate in writing an alternate to act in the Administrator's absence;
    d. Provide management and supervision of services throughout all approved service areas;
    e. Arrange for necessary services;
    f. Keep contracts current and consistent with WAC 246-335-425 (4);
    g. Serve as a liaison between the licensee, personnel and contractors;
    h. Ensure personnel and contractors are currently credentialed by the state of Washington, when appropriate, according to applicable practice acts and consistent with WAC 246-335-425 (5);
    i. Ensure personnel and contractors comply with the agency's policies and procedures;
    j. Implement a quality improvement process consistent with WAC 246- 335-455;
    k. Manage recordkeeping according to WAC 246-335-450;
    l. Ensure supplies and equipment necessary to patient care are available, maintained, and in working order;
    m. Ensure the accuracy of public information materials; and
    n. Ensure current written policies and procedures are accessible to personnel and contractors during hours of operation

Management and Supervision Criteria

1. **Qualifications:** All management and supervisory staff must possess the necessary education, training, and experience to effectively manage and supervise home health services. This may include:
    - A registered nurse (RN) license
    - A bachelor's degree in a related field (e.g., healthcare administration, social work)
    - Experience in home health care management or supervision

2. **Supervision of Staff:** Supervisors must ensure that all staff members are adequately trained, competent, and supervised in their roles. This includes:
    - Providing orientation and ongoing training
    - Monitoring staff performance
    - Addressing any performance issues or concerns
    - Ensuring compliance with agency policies and procedures
3. **Development and Implementation of Service Plans:** Supervisors must ensure that comprehensive service plans are developed for each client in accordance with WAC 246-335-540. This includes:
    - Conducting initial assessments to determine client needs
    - Collaborating with clients and their families to develop personalized care plans
    - Ensuring that service plans are updated as needed to reflect changes in the client's condition
4. **Quality Assurance:** Supervisors must implement and maintain a quality assurance program to monitor the quality of care provided and identify areas for improvement. This includes:
    - Conducting regular reviews of client records
    - Monitoring staff performance
    - Addressing client complaints and concerns
    - Implementing corrective actions as needed
5. **Compliance:** Supervisors must ensure that the agency is in compliance with all applicable state regulations, including WAC 246-335-540. This includes:
    - Maintaining accurate and up-to-date documentation
    - Ensuring that staff are properly licensed and certified
    - Adhering to all safety and infection control standards

SUPERVISION OF PERSONNEL, VOLUNTEERS, AND CONTRACTORS

WAC 246-335-545

Purpose:

This policy outlines the procedures for supervising personnel, volunteers, and contractors who provide home health services. It ensures all services meet the high-quality standards required by WAC 246-335-545.

1. **Supervision of Personnel and Volunteers**
   - **Qualified Supervisors:** A qualified healthcare professional, as defined by WAC regulations, will supervise all personnel and volunteers providing direct care services. This supervisor may be an RN, LPN, therapist (PT, OT, Speech), or other qualified professional according to the services provided.
   - **Training and Competency:** Supervisors will ensure all personnel and volunteers under their supervision:
     - Receive proper training on relevant policies and procedures, including the plan of care for each client.
     - Demonstrate competency in performing assigned tasks before providing independent care.
     - Participate in ongoing training to maintain and update their skills and knowledge.
   - **Supervision Methods:** Supervision will be conducted through a combination of methods, including:
     - Regular in-person or virtual meetings to discuss client care and address any concerns.
     - On-site visits by the supervisor to observe care delivery and provide feedback.
     - Review of documentation completed by personnel and volunteers.
     - Availability for consultation and problem-solving throughout the service delivery period.
2. **Monitoring of Services Provided by Contractors**
   - **Contractor Selection:** The agency will establish a process for selecting qualified and reputable contractors to provide home health services. This process will consider factors such as licensure, certifications, experience, and references.
   - **Contracts:** All contracts with home health service providers will be in writing and clearly define:
     - The specific services to be provided.
     - Performance expectations and quality standards.
     - Supervision and communication protocols.
     - Reporting requirements.
   - **Monitoring Activities:** The agency will monitor the services provided by contractors through various methods, including:
     - Regular review of contractor performance reports.
     - Client feedback surveys.
     - Random on-site visits to observe service delivery.
     - Review of contractor documentation, including care plans and progress notes.
   - **Corrective Action:** If any deficiencies are identified in the services provided by a contractor, the agency will take appropriate corrective action. This may include retraining, revising the contract, or terminating the contract if necessary.

3. **Documentation**
    - All supervision and monitoring activities will be documented in the client's medical record. Documentation will include:
        - Dates and types of supervision provided to personnel and volunteers.
        - Notes from supervisory meetings and on-site visits.
        - Documentation of training completion.
        - Contractor performance reports.
        - Records of communication with contractors.
        - Any corrective actions taken.
4. **Compliance**
    - This policy ensures compliance with WAC 246-335-545 by establishing a framework for effective supervision of personnel, volunteers, and contractors.
    - Staff will be trained on this policy and will be updated on any regulatory changes.

PERFORMANCE EVALUATION FOR PERSONNEL, VOLUNTEERS, AND CONTRACTORS

Purpose

This policy outlines the procedures for conducting performance evaluations for personnel and volunteers employed directly by the agency, and for evaluating services provided by contractors. It ensures all staff and contractors meet the high-quality standards required by WAC 246-335-525(16) and (17).

1. **Performance Evaluation for Personnel and Volunteers**
   - **Frequency:** Performance evaluations will be conducted for all personnel and volunteers at least annually. More frequent evaluations may be conducted for new staff, staff in probationary periods, or those requiring additional support.
   - **Evaluation Process:** The evaluation process will involve:
     - **Setting Performance Objectives:** Supervisors will collaborate with each staff member and volunteer to establish clear and measurable performance objectives aligned with their job descriptions and the client's plan of care.
     - **Evaluation Tools:** A standardized evaluation form will be used to assess performance in key areas, such as:
       - Knowledge and skills related to assigned tasks
       - Quality of care delivery
       - Adherence to policies and procedures
       - Communication and collaboration
       - Professionalism and dependability
     - **Self-Evaluation:** Staff and volunteers will have the opportunity to complete a self-evaluation reflecting on their performance and areas for improvement.
     - **Feedback Meeting:** A supervisor will meet with each staff member and volunteer to discuss the evaluation results, provide feedback, and establish development goals for the next evaluation period.
   - **Documentation:** All evaluation documents, including performance objectives, completed evaluation forms, and notes from feedback meetings, will be maintained in the employee or volunteer's personnel file.
2. **Evaluation of Services Provided by Contractors**
   - **Frequency:** The agency will evaluate the services provided by contractors at least annually. More frequent evaluations may be conducted based on the complexity of services, past performance, or identified concerns.
   - **Evaluation Methods:** Contractor evaluations will utilize various methods, including:
     - **Reviewing Contractor Performance Reports:** Contractors will be required to submit periodic reports outlining their performance against agreed-upon metrics and quality standards.
     - **Client Feedback Surveys:** Clients receiving services from contractors will be given the opportunity to provide feedback on their satisfaction with the care provided.
     - **On-Site Visits:** The agency may conduct random or scheduled on-site visits to observe service delivery by contractors and assess compliance with contractual obligations.

    - **Review of Contractor Documentation:** The agency may review contractor documentation, such as care plans, progress notes, and incident reports, to evaluate the quality of care provided.
- **Corrective Action:** If any deficiencies are identified in the services provided by a contractor, the agency will take appropriate corrective action. This may include retraining, revising the contract, or terminating the contract if necessary.

3. **Compliance**
    - This policy ensures compliance with WAC 246-335-525(16) and (17) by establishing a systematic process for evaluating the performance of personnel, volunteers, and contractors.
    - Staff will be trained on this policy and will be updated on any regulatory changes.

QUALITY IMPROVEMENT PROGRAM

WAC 246-335-555

This policy outlines the implementation of a quality improvement program (QIP) throughout all our approved home health service areas. This program ensures we continuously monitor, evaluate, and improve the quality of care and services we provide, adhering to WAC 246-335-555.

1. **QIP Structure**

Our QIP will consist of the following core elements:

- **Quality Improvement Committee:** A designated committee comprised of qualified healthcare professionals, staff representatives, and, when appropriate, consumer representatives, will oversee the QIP.
- **Performance Measures:** We will identify key performance indicators (KPIs) aligned with national and state quality standards for home health care. These KPIs may include:
  - Client satisfaction with care
  - Readmission rates to hospitals
  - Rates of hospital-acquired infections
  - Medication adherence rates
  - Pain management effectiveness
  - Client functional outcomes
- **Data Collection and Analysis:** We will establish a system for collecting data on our identified KPIs. Data collection methods may involve client surveys, chart reviews, incident reports, and staff feedback. The QIP committee will regularly analyze this data to identify trends and areas for improvement.
- **Quality Improvement Activities:** Based on data analysis, the QIP committee will recommend and implement quality improvement activities. These activities may involve:
  - Updating policies and procedures.
  - Implementing new training programs for staff.
  - Piloting new care models or interventions.
  - Revising plans of care to address identified needs.
- **Communication and Reporting:** The QIP committee will communicate program findings and improvement activities to all staff and leadership. Regular reports on the QIP will be submitted to the appropriate regulatory agencies.

2. **Application Across Service Areas**

The QIP will be applied consistently across all our approved service areas:

- **Service Area Integration:** KPIs and data collection methods will be adapted to address the specific needs and services provided in each area.
- **Communication and Collaboration:** Staff from all service areas will be encouraged to participate in the QIP process by sharing best practices and feedback.
- **Benchmarking:** We will benchmark our performance against national and state quality standards, as well as compare data across service areas to identify areas for improvement.

3. **Continuous Improvement**

The QIP is an ongoing process committed to continuous improvement in the quality of care provided by our agency. The program will be reviewed and updated regularly to reflect changes in regulations, best practices, and client needs.

4. **Compliance**

This policy ensures compliance with WAC 246-335-555 by establishing a comprehensive QIP that monitors, evaluates, and improves the quality of care across all our approved service areas.

TRANSFER OF CARE

WAC 246-335-520; 246-335-550

Statement of Purpose

The applicant or licensee must develop and operationalize delivery of services policies and procedures that describe:

(7) A process to inform patients of **alternative services** prior to **ceasing operation** or when the licensee is unable to meet the patient's needs; and

(8) A plan for preserving records, including the process to preserve or dispose of records prior to **ceasing operation** according to **WAC 246-335-550(7)** and **(8)**

Purpose

To ensure continuity of care and provide a written plan for orderly transfer of care of patients and clinical records in the event of dissolution of the Agency.

Policy:

1. Supervisor must ensure that a contingency plan will be followed in the event of dissolution of the Agency to assure continuity of patient care, assure the notification of patients of the agency's dissolution and documentation of the notification and compliance with State procedures.
2. In order to minimize the possibility of patient abandonment, patients will be given at least a forty-eight (48) hour written or verbal notice prior to discharge that will be documented in the patient record.
3. The patient or legal representative and the ordering Authorizing Practitioner will be notified of closure, names of providers that care may be transferred to and all participants will be allowed to decide who the patient's care will be transferred to.
4. All documentation will be placed in the patient's record.
5. Upon request, in accordance with agency policy, will provide patient information or a summary of care when the patient is transferred or discharged to another agency or facility.

Procedure:

1. The agency will notify the department in writing within thirty (30) days of ceasing operation of home care services.
2. The department will be informed of the location of patient's records (active and inactive) and the name and address of the patient record custodian.
3. Copies if active patient records will be transferred to the receiving agency, with patient permission, to which arrangements have been made to transfer care.
4. The transfer of patient information will be shared in order to ensure continuity of care and services to the patient.
5. A transfer form will be completed by the primary nurse or supervisor and placed in the patient record.

PRESERVING RECORDS

(7) The licensee must keep patient records for:

(a) Adults - Three years following the date of termination of services;

(b) Minors - Three years after attaining age eighteen, or five years following discharge, whichever is longer; and

(c) Patient death - Three years following the last date or termination of services if patient was on services when death occurred.

(8) The licensee must:

(a) Store patient records in a safe and secure manner to prevent loss of information, to maintain the integrity of the record, and to protect against unauthorized use;

(b) Maintain or release records in accordance with chapter **70.02** RCW; and

(c) After ceasing operation, retain or dispose of patient records in a confidential manner according to the time frames in subsection (7) of this section.

HOME HEALTH AGENCY PATIENT RECORD DOCUMENTATION

Purpose

This policy outlines the timeframes for filing documents in patient records for **Golden Years Home Health Supported Living LLC**, a home health agency licensed in Washington State. It adheres to the Washington Administrative Code (WAC) 246-335-515(9), which emphasizes the importance of maintaining accurate and complete patient records.

Policy Statement

Golden Years Home Health Supported Living LLC is committed to maintaining comprehensive and timely patient records that accurately reflect the care provided to each patient. All documentation must be completed, signed, and dated by the appropriate personnel within the following timeframes:

- **Initial Assessment:** All documents related to the initial assessment, including the patient medical history, plan of care, and physician orders, must be filed in the patient record **within 24 hours** of completion.
- **Ongoing Progress Notes:** Progress notes documenting the patient's condition, interventions provided, and response to care will be completed and filed in the patient record **within 48 hours** of the visit.
- **Discharge Summary:** A comprehensive discharge summary outlining the patient's progress, medications, and recommendations for continued care will be completed and filed in the patient record **within seven (7) days** of discharge.
- **All Other Documentation:** All other documents pertinent to the patient's care, such as laboratory results, physician reports, and medication administration records, must be filed in the patient record **within ten (10) business days** of receipt.

Procedures

1. All staff involved in patient care will be trained on this policy and the importance of timely documentation.
2. Standardized forms with clear instructions for completion will be readily available for all documentation needs.
3. Electronic documentation systems, if utilized, will be designed to enforce timeframes for required entries.
4. The role of the Clinical Director and Administrator will be to regularly review patient records to ensure compliance with this policy and address any discrepancies.

Consequences of Non-Compliance

Failure to adhere to these timeframes could result in:

- Inaccurate or incomplete patient records, hindering effective care delivery.
- Delays in care coordination and communication with other healthcare providers.
- Potential for regulatory citations or penalties from the Washington State Department of Health.

Retention and Disposal of Records

Patient records will be maintained in accordance with state regulations. The agency will have a separate policy outlining the specific retention and disposal procedures for patient records.

EMERGENCY PREPAREDNESS DISASTER PLAN

WAC 246-335-515

Statement of Purpose

The applicant or licensee must develop and implement a plan of operation which includes

(10) Emergency preparedness that addresses service delivery when natural disasters, man-made incidents, or public health emergencies occur that prevent normal agency operation. Include, at minimum: Risk assessment and emergency planning, communication plan, coordination of service delivery with emergency personnel to meet emergent needs of patients, and staff training

Policy

1. In depth policies are located in a separate Disaster Manual.
2. The Emergency/Disaster Plan provides an orderly back-up care procedure, which will be implemented in an emergency to assure that the health care needs of patients continue to be met.
3. All employees shall be oriented to the plan and their responsibilities.
4. Possible emergency or risk factors will be identified for each patient, and an appropriate emergency plan discussed with the patient and/or the responsible person at the time of admission as appropriate.
5. A current patient roster will be maintained that includes the classification assigned to the patient.
6. The name and telephone number of an emergency contact will be obtained.

Procedure

The Administrator or designee shall be responsible for implementing the plan as follows:

1. If an emergency occurs, either within the Agency causing staffing limitation (such as labor disputes or staff illness) or within the environment (such as floods, blizzards, hurricanes, fires or other natural disasters), the Director of Clinical Services or his/her designee will be responsible to review and prioritize patients according to the following classifications:

   Class 1 When the patient has a condition that is potentially life threatening,

   requires ongoing medical treatment, or requires assistance of a medical device to sustain life, the home environment and support system will be reviewed. When appropriate, arrangements for evacuation to an acute care facility will be made. These patients will be seen immediately. The Agency will obtain assistance from emergency personnel as necessary.

   Class 2 Patients with the greatest need for care will be seen as soon as

   possible by available staff. These include patients requiring daily insulin injections, IV medications, and sterile wound care of a wound with a large amount of drainage.

   Class 3 Services can be postponed 24-48 hours without adverse effects on

   the patient (e.g., an insulin dependent diabetic able to self-inject, a patient requiring a cardiovascular and/or respiratory assessment, a patient requiring sterile wound care to a wound with minimal amount of drainage or no drainage.)

   Class 4 Services could be postponed 72-96 hours without adverse effect on

the patient (e.g., a postoperative patient with no open wound, patients who are expected to be discharged within the next 10-14 days, patients requiring routine catheter changes).

2. Those patients with a "Class 1" designation shall be the first priority. The Administrator will immediately arrange with staff, the police, and/or fire department to dispatch assistance as soon as possible to these patients.
3. The Administrator shall contact all office staff to declare the emergency and have them contact their designated field staff. Staff safety is a primary concern. Staff will not be asked to jeopardize their safety if the situation becomes potentially dangerous. If the situation becomes potentially dangerous for staff, the authorities will be notified and, if necessary, an ambulance will be called for the patient.
4. All case managers/coordinators will contact their patients by priority to determine their status then report patient needs to the Administrator.
5. The Administrator or his or her designee is responsible for the following:
    a. Arranging for the orientation and periodic in-service for all staff to their respective responsibilities.
    b. Contacting the Civil Defense Department to inform them of the Agency's plan, if indicated.
    c. Receiving communication regarding a disaster and disseminating the same.
    d. Activating, implementing and discontinuing the plan.
    e. Developing an information/notification plan.
    f. Coordinating the activity of all key personnel and staff.
    g. Maintaining a list of all active employees and their home phone numbers.
    h. Maintaining a list of all active patients and their phone numbers.
    i. Notifying the telephone answering service of implementation of the emergency plan.
    j. Ensuring staff safety.
6. The Agency will be available to assist with triage, transportation, staffing, home care, and follow-up. The Emergency/Disaster Plan may be evoked for the following:
    a. Natural Disaster – blizzards, tsunami, floods, fires, etc.
    b. Civil Defense Disaster – war, poisonous gas, nuclear accident, bioterrorism.
    c. Community Emergency – strike, transportation accident, black-out, etc.
7. The Administrator shall be responsible for declaring that the plan is to go into effect. He/she will notify the supervising nurse who shall identify all patients in life threatening situations. Those, whose lives are threatened such as ventilator-dependent cases and bed-confined persons living alone, shall be the Agency's first priority. The Administrator will immediately arrange with staff, the police, and/or fire department to dispatch assistance as soon as possible. The Administrator shall notify all coordinators to begin identifying the location and availability of all staff. Coordinators will contact staff by phone, if possible, to instruct them when and where to report. All staff will also be advised that they must phone the office to report their whereabouts and availability. The coordinators shall maintain constant contact with the Administrator and supervising nurse to advise them of the staff status and shall assign or reassign cases as directed by the Administrator.
8. The coordinators and/or their designees shall utilize the patient roster to notify all patients of the disaster arrangements.

9. Notification shall be by priority as above. The Agency shall maintain a list or roster of personnel including their current telephone number. A similar list of patients and telephone numbers shall be maintained. Any person believed to qualify as the highest priority in a disaster shall be identified as such on the list. Staff shall be advised to routinely and continually notify the coordinators of such patients.
10. To the extent possible, record keeping during a disaster shall be thorough and complete. Attempts to notify staff and patients and the outcome, including alternate plans, should be documented.
11. All disaster/emergency activities will be documented by the Administrator in Agency records. Contacts with the patients shall be documented in the patient's clinical record. As indicated, contacts with staff may be documented in the employee's personnel record.
12. The Administrator shall declare when the emergency/disaster plan is discontinued. The Administrator shall periodically review the plan and propose any recommendations for change to the Governing Body.

Staff Emergency Preparedness Plan

Established a family preparedness plan

- Have a family communication plan
- Identify a point of contact that is out-of-town or in another state
- Escape routes
- Evacuation plan
- Plan for pets

Know your agency's emergency preparedness plan

- Know who to report to and procedures to follow
- Be prepared to assume tasks/roles out of your ordinary job description
- Ensure credentials (Identification cards, professional license, any local or state credential needed to move around restricted areas) are up to date and with you
- Know how supplies will be procured for patients
- Know the agencies communication tree

Have the automobile equipped

- Full tank of gas - identify gas stations that have emergency/backup power
- Maps of the area
- Shovel
- Blankets
- Portable battery operated or crank radio
- Cell phone charger
- Portable battery operated or crank flashlight
- Booster cables
- Bottled water and non-perishable high energy foods, such as granola bars, raisins and peanut butter
- Flares
- Tie repair kit
- Fire extinguisher
- First aid kit

Have alternative communication devices available for use

- charged cell phone
- portable phone
- satellite phone

ACCIDENT PREVENTION

If you are over age 65, your chances of dying from an accident are almost twice as likely as that of any other age group.

By taking the right precautions, you can protect yourself and those around you and prevent serious injury.

Why is injury a common problem among older people?

1. During the aging process certain physical, mental and emotional changes occur:
2. Less physical strength
3. Impaired eyesight
4. Impaired hearing
5. Slower physical reaction
6. Poor balance and coordination

What was considered a "minor" accident in your younger years may be serious now due to

1. Lower resistance to disease
2. Slower healing
3. Bones that are brittle
4. Your particular illness or disease
5. Physical limitations

The most common and dangerous cause of injury is falls.

Here are measures you can take to reduce falls

1. Remove scatter rugs or use non-skid tape or backing on throw rugs
2. Tack down the edges of all carpets
3. Never leave articles around beds, stairs, or in hallways
4. Do not use a doorway, halls or stairs for storage
5. Keep pathways clear of furniture, electric cords, space heaters, etc.
6. Don't rush when climbing up or down stairs
7. Stairs should have non-skid treads and a solid, easy to grasp handrail
8. If you must climb, use a solid step or ladder rather than a chair or box
9. When carrying objects, make sure you:
    a. Can see
    b. Get a firm grip
    c. Move slowly and evenly
    d. Lift with your legs (knees bent, back straight)

e. Ask for help with heavy or awkward objects

General Safety Tips

1. Avoid wearing only socks, smooth-soled shoes or slippers on non-carpeted floors
2. Avoid wet floors – wipe all spills up immediately
3. Keep kitchen floor free of grease and scraps
4. Household pets should be kept under control and out of pathways
5. To avoid dizziness, get out of your bed or chair slowly.
6. In the bathroom:
   a. Be sure mats are non-skid and there are treads in the tub or shower to prevent slips.
   b. Install “grab bars”. Towel racks should not be used as grab bars as they are not secure enough to support body weight.

Adequate lighting will help prevent accidents

1. Keep a lamp near the bed so you will not have to get up in the dark.
2. Keep a night light in the bathroom.
3. Keep hallways and steps well-lit.
4. Keep a flashlight handy in case of power failure

Additional Important Safety Tips

1. Post Emergency Numbers By Your Phone
2. If you live alone, ask a neighbor, friend, or family member to check on you each day
3. Take your time
4. Be safe
5. Do not take unnecessary risks

EMERGENCY AND DISASTER PROCEDURES

WAC 246-335-515

Statement of Purpose

The applicant or licensee must develop and implement a plan of operation which includes

(10) Emergency preparedness that addresses service delivery when natural disasters, man-made incidents, or public health emergencies occur that prevent normal agency operation. Include, at minimum: Risk assessment and emergency planning, communication plan, coordination of service delivery with emergency personnel to meet emergent needs of patients, and staff training

Policy

1. To ensure the continuation of agency operation and patient care, as normal as possible, during an emergency or disaster event by having a defined emergency management plan.
2. The plan will include at a minimum the following:
   a. Risk assessment
   b. Emergency planning
   c. Communication plan
   d. Coordination of service delivery with emergency personnel to meet the emergent needs of clients
   e. Staff training

Procedure

Inclement Weather

1. All administrative and supervisory staff are expected to contact the Administrator directly for instructions regarding operations and patient care.
2. All employees are expected to call the answering service regarding the opening of the Agency.
3. All office staff who can report are expected to do so.
4. If inclement weather conditions exist prior to the office opening, the Administrator will advise the answering service, at least one hour before the office opens if possible, if the office will be open, closed, or the opening delayed.
5. If the office is open, all staff are expected to report to work unless otherwise directed by their supervisor. If the employee chooses not to report to work, the supervisor must be notified in order that patient visits may be arranged.
6. The employee may utilize annual leave for the missed time. Absences without proper notice will not be paid.
7. If inclement weather develops during the work day the Administrator will make a decision regarding closing the Agency early.
8. Staff in the field are expected to call their supervisor for further instructions.
9. According to the disaster plan (#1.45), high risk patients will be notified of the emergency situation to arrange for supervision/care of patients. Instructions or assistance may be provided with transportation to hospitals or shelters.

10. If the office telephone service is out of order, operations will be maintained from an alternate location designated by the Administrator.
11. The answering service will be alerted regarding the need for priority service.
12. If no telephone service is available in the area, state and local police, fire stations, hospitals and patient's Authorizing Practitioners and emergency contacts will be notified of high-risk patients.
13. The Agency will attempt to assist high risk patients to obtain shelter and safety.

Interruption of Public Transportation

1. Staff members who use public transportation will be instructed to arrange for assistance from relatives or friends or carpool to patient's homes if possible.
2. At the discretion of the Administrator, staff may be assisted with transportation by supervisors, taxi, and/or state police to high risk patients.
3. Patients who rely on public transportation will be assisted by the Agency with alternate arrangements.
4. State and local police, emergency medical services and fire departments will be notified to assist with emergency services to meet patient's needs.

Work Stoppage

1. In the event of a work stoppage by Agency staff, patients will be contacted by a supervisor and appropriate arrangements will be made for service, i.e., family/friends, local hospitals, and other home care providers.
2. The Agency will immediately implement efforts to recruit new employees to meet staffing needs, and the Administrator will begin negotiations to resolve the situation as quickly as possible.
3. If a work stoppage occurs at a facility resulting in increased referrals and demands for service, the Administrator will immediately contact the facility to identify how the Agency can assist during the crisis.

SNOW STORMS/ BLIZZARDS/SNOW EVENTS

Definitions

Winter Storm Watch: This means a winter storm is possible.

Winter Storm Warning: This means a storm is occurring or soon will.

Before the Storm

- Know your risk.
- Make sure your disaster kit is available including emergency phone numbers.
- Charge your cell phone.
- Get some food and water into your home to sustain you for a few days.
- Ensure that you have back up batteries, flashlights and if you are using an oxygen concentrator, have your back up system accessible.
- If you have oxygen, contact your oxygen provider and arrange for back up supplies. If you have backup tanks, check your backup tanks to make sure they are full and that you can switch over to the backup.

- Listen to a local radio and television station for official announcements issued from the Emergency Operations Center.
- If the storm is reported to actually be happening, obtain assistance from family and friends to stay with you and assist you or make arrangements to stay with them. If you change your location, let the Agency know the address at which you will be staying.
- Place a shovel at the door to your home and make sure you have made arrangements for someone to assist you with snow removal.

During the Disaster

- STAY CALM
- Inside: Keep the home warm and avoid the use of kerosene heaters inside the house.
- Stay indoors and dress warmly. Dressing in layers of loose fitting clothing is best.
- Make sure that you keep up your nutrition and hydration status.
- Outside: winds and snow accumulation can build up on the tops/peaks of home, avoid walking under these. Stay inside and warm. You can take care of issues outside after the storm.
- On the road: Be safe and stay home. If you need to be removed from the home you are encouraged to be in a vehicle that has 2-4 wheel drive and snow tires.

After the Disaster

- Call for assistance from snow removal personnel.
- If you have to go out, dress warmly and be careful not to over exert if shoveling snow.
- Turn on the radio and listen for instructions on travel and safety instructions.

Other Important Information

- Plan for reuniting family.

HURRICANE INFORMATION

HURRICANE: IT'S NOT JUST ANOTHER STORM

Hurricane Survival Checklists

Before the Storm

- Know your risk
- How high is your home from sea level? Consult your home's building for your first floor elevation.
- Listen to a local radio and television station for official announcements issued from the Emergency Operations Center.

Special Circumstances (anything requiring additional preparation and/or evacuation time)

- Mobile Home/recreational vehicle
- People with special needs (medical or physical condition)
- Pets
- Boats

Know the Strength of the Hurricane

- Category One: 74-95 mph sustained winds
- Category Two: 96-110 mph sustained winds
- Category Three: 111 -130 mph sustained winds
- Category Four: 131-155 mph sustained winds
- Category Five: above 155 mph sustained winds

Determine where you will seek shelter if you have to leave and select an alternate

- Friend’s house, if located away from risk area
- Hotel or Motel located Inland
- Emergency Public Shelter operated by the American Red Cross

OTHER IMPORTANT THINGS TO CONSIDER

- Take a drive to your shelter choice so you know where it is located. Time the trip and multiply the time by three (3) to account for pre-storm road traffic conditions.
- Make the commitment now to evacuate when you are told to do so by local or state officials.
- If you do not have flood insurance, consult your insurance agent purchase. There is a five day wait before coverage begins.
- Prepare your hurricane evacuation kit.

DURING THE STORM, REMAIN INSIDE

- Blowing debris can injure or kill. Travel is extremely dangerous. Stay inside until authorities have announced your area is safe.

- Stay away from windows. Avoid using all electrical appliances. Seek refuge in a small interior, windowless area such as a closet, hallway or bathroom.

AFTER THE STORM

- Expect the worst. Be careful of downed power lines, gas leaks, weakened structures and dangerous animals.
- Do NOT drink the water. Eat only foods you're sure are absolutely safe.
- Be extra careful in handling power tools, generators, candles, matches and gas lanterns.
- Ask your Insurance Company for financial help. Listen to local radio stations for official relief information and instructions.

STAY SAFE

EARTHQUAKES/TORNADOES/OTHER DISASTERS

You may face potential threats from Earthquakes, Tornadoes or Other Disasters

Disaster Information

We are constantly aware of the potential of an earthquake/tornado/ other disasters creating damage and creating dangerous conditions. We need to properly prepare so that a disaster of any type will not cause greater personal damage than necessary. The items listed below may help you survive the disaster in a better way.

During the Disaster

- STAY CALM
- Inside: Stand in an internal hallway or crouch under a desk or table, away from windows or glass dividers
- Outside: Stand away from buildings, trees, telephone and electric lines
- On the road: Drive away from underpasses/overpasses; stop in a safe area; stay in the vehicle. Be aware of road conditions and do not take risks.

After the Disaster

- Check for injuries - provide first aid
- Check for safety - check for gas, water, sewage breaks, downed electric lines and shorts; turn off appropriate utilities; check for building damage and potential safety problems
- Aftershocks from an earthquake or storm resurgence can cause cracks around the chimney, foundation, stairs
- Clean up dangerous spills
- Wear shoes
- Turn on radio and listen for instructions on travel and safety instructions
- Don't use the telephone except for emergency use

Other Important Information

- How to turn off gas, water and electricity
- Do NOT drink water. Eat only foods you are sure are safe
- Plan for reuniting family
- Contact your insurance company regarding damage

FIRE

Protect yourself, your family and your home against fire or burns.

- Be prepared!
- Make a fire escape route and practice it.
- Make sure fire exits are free of clutter.
- Keep a fire extinguisher charged and handy. Know how to use it.
- Install smoke detectors and keep them in working order.

- Don’t smoke in bed or when sleepy.
- Use space heaters according to the manufacturer's instructions. Keep them free from clutter, paper, curtains, etc.
- Keep flammable liquids outside of home in approved safety containers.
- Have your home electrical system checked if there are signs of a wiring problem.
- Keep all electrical appliances in good working order.
- Use extension cords properly. Do not overload them and keep them away from sinks or water.
- Keep towels, curtains, and other flammables a safe distance from the stove.

To prevent burns

- Always check the hot water temperature. Experts suggest setting hot water heaters at 120 degrees F or below.
- Wear tight fitting or short sleeves when cooking.
- Keep pot handles away from the front of the stove.
- Use potholders.

COMMUNICATION PLAN

Goals

- Ensure timely and accurate information dissemination during emergencies.
- Minimize confusion and anxiety among staff, clients, and families.
- Facilitate coordinated response efforts with emergency responders and other healthcare providers.

Communication Channels

The agency will utilize a variety of communication channels to reach staff, clients, and families depending on the nature and severity of the emergency:

- **Landline Phones:** Primary communication method for non-critical emergencies or situations where internet/cell service might be disrupted.
- **Cell Phones:** Secondary method for contacting staff, clients, and families already identified with cell phone numbers.
- **Text Messages:** Can be used for quick updates and short, essential information. (Ensure prior consent for text message communication)
- **Email:** Suitable for detailed information updates or sharing resources after an emergency has passed.
- **Social Media (Optional):** Can be used to broadcast general updates to a wider audience, but should not be the primary communication channel for individual clients or families.

Target Audience and Communication Strategies

- **Staff:**
  - Establish a clear chain of command for emergency communication.
  - Utilize landline phones, cell phone calls, and text messages for initial contact and updates.
  - Develop a staff call tree to ensure all personnel are notified.
  - Utilize email for detailed instructions and resource sharing.
  - Conduct regular emergency preparedness drills to familiarize staff with communication protocols.
- **Clients and Families:**
  - Identify a primary contact person for each client and obtain their preferred communication method (phone, email, etc.).
  - Develop a client/family contact list to facilitate communication during emergencies.
  - Prioritize contacting primary caregivers first, followed by additional family members as needed.
  - Use clear, concise, and empathetic language when communicating with clients and families.
  - Provide updates on the situation, safety protocols, and next steps.
  - Offer emotional support and resources as needed.

Plan Implementation and Maintenance

- This communication plan will be distributed to all staff members for review and understanding.
- Staff will be trained on the communication protocols outlined in the plan.
- The plan will be reviewed and updated annually or more frequently if necessary, considering lessons learned from drills or actual emergencies.

- The agency will maintain a communication log to document all communication efforts during an emergency.

Additional Considerations

- **Clients with Special Needs:** Develop specific communication strategies for clients with disabilities or language barriers.
- **Power Outages:** Have a backup plan for communication in case of power outages (e.g., battery-powered radios, pre-designated meeting locations).
- **Public Information:** Consider how the agency will communicate with the general public, if necessary (e.g., press releases, social media updates).

PATIENT INSTRUCTIONS IN THE EVENT OF AN EMERGENCY

Emergencies include natural and manmade disasters. This may include hurricanes, tornadoes, earthquakes, severe weather or other natural disasters or it may include man made disasters such as bio-terrorism, Terrorism, Radiation, Chemical Spills, Nuclear Accidents and Hazardous Material.

We are faced with many types of emergency situations that may cause an interruption in services.

In the event you are faced with a natural disaster, inclement weather, and/or other emergency situation, to ensure the highest level of patient care & continuity of services, it is the policy of the agency that a patient immediately call the agency to advise of the emergency situation. Please utilize the following to contact the agency:

AGENCY PHONE NUMBER: (253) 487-7217

If you choose to evacuate during an emergency, you must take provisions with you. The following suggested items will make your temporary stay more comfortable:

- Foods that do not need cooking and Drinking water (1 gallon per person per day)
- Special dietary food if required
- Identification, valuable papers and photos, including medical information
- Personal hygiene items, such as: soap, deodorant, shampoo, toothbrush, toothpaste, aspirin, antacid, incontinent supplies, washcloth, towels etc.
- Utensils, such as: manual can opener, disposable plates, cups, forks, knives, spoons, napkins
- Prescription medicines, written prescription for refills & list of medications
- Books, magazines, cards, toys, and games for adults and children
- Infant care items such as formula, food, disposable diapers and toys
- Battery operated radio flashlight & lantern, extra batteries & earphones
- First aid kit including: betadine solution, bandages, adhesive tape, band-aids, bandages, safety scissors, non-prescription medicines
- Personal aids such as: eyeglasses, hearing aids & prosthetic devices
- Change of clothing and rainwear; sleeping bag or blanket, sheet & pillow

REMEMBER

ALL ALCOHOLIC BEVERAGES, ILLEGAL DRUGS, PETS, AND WEAPONS

ARE PROHIBITED WITHIN EMERGENCY PUBLIC SHELTERS.

For more information, see the community reference pages in your telephone

directory, or call

The State Emergency Management Agency

Address

COORDINATION OF SERVICES DELIVERY

Effective communication isn't the only crucial aspect of emergency preparedness. Equally important is establishing a strong plan for coordinating service delivery with emergency personnel. Here's what our agency can do:

1. **Building Relationships:**
   - **Establish Contacts:** Proactively develop relationships with local emergency response agencies such as fire departments, emergency medical services (EMS), and local public health departments.
   - **Joint Training:** Participate in joint training exercises with emergency personnel. This helps both parties understand each other's capabilities and limitations, fostering collaborative response during actual emergencies.
   - **Regular Communication:** Maintain regular communication with emergency personnel. This could involve quarterly meetings or information exchanges to ensure everyone has updated contact information and is aware of any changes within your agency or their services.
2. **Client Needs Assessment:**
   - **Vulnerability Assessment:** Conduct a vulnerability assessment for all your clients to identify those at higher risk during emergencies. This could consider factors like:
     - Medical conditions requiring ongoing care
     - Dependence on medical devices
     - Mobility limitations
     - Living alone
     - Lack of transportation
   - **Emergency Plans for Clients:** Develop individual emergency plans for high-risk clients in collaboration with the client, their family (if applicable), and their primary care physician. These plans should include:
     - Emergency contact information
     - Evacuation procedures
     - Alternative care locations (if necessary)
     - Medications and medical equipment needs
3. **Communication During Emergencies:**
   - **Notification Protocols:** Establish clear protocols for how your agency will notify emergency personnel of clients who require assistance during an emergency.
   - **Information Sharing:** Develop a system for sharing essential client information with emergency personnel, following HIPAA privacy regulations. This could include:
     - Medical conditions
     - Medications and allergies
     - Any special needs
4. **Staff Training:**
   - Train your staff on how to handle emergencies in the client's home environment. This could include:
     - Recognizing and responding to various emergencies (e.g., fires, floods)
     - Basic first aid and CPR training

- Proper evacuation procedures
- Communication protocols with emergency personnel

5. **Utilizing Technology:**

- Consider using technology to facilitate communication and coordination with emergency personnel. This could involve:
  - Secure online platforms for sharing client information (HIPAA compliant)
  - Mobile applications for real-time updates and communication
  - Electronic medical records access for emergency personnel (with patient authorization)

STAFF TRAINING

Training Frequency

- **Upon Hire:** New staff should receive emergency preparedness training as part of their onboarding process.
- **Every Two Years:** All staff should participate in refresher training on emergency preparedness every two years to ensure knowledge retention and address any updates to protocols.

Training Objectives

- Equip staff with the knowledge and skills to respond effectively during emergencies in client homes.
- Foster clear communication and coordination with emergency personnel.
- Ensure the safety and well-being of clients and staff during emergencies.

Training Content

- **Types of Emergencies:**
  - Familiarize staff with potential emergencies in Washington State (e.g., earthquakes, floods, power outages, hazardous materials spills).
- **Client Vulnerability Assessment:**
  - Train staff on identifying clients at higher risk during emergencies and understanding their specific needs.
- **Emergency Response Procedures:**
  - Train staff on proper procedures for handling various emergencies in the client's home environment, including:
    - Evacuation procedures (considering client mobility limitations)
    - Basic first aid and CPR
    - Operating medical equipment (if applicable)
    - Securing the client's home
- **Communication Protocols:**
  - Train staff on communication protocols for notifying emergency personnel and the agency during emergencies.
  - Emphasize the importance of clear and concise communication, following HIPAA regulations when sharing client information.
- **Resource Management:**
  - Train staff on utilizing available resources during emergencies, such as:
    - Emergency preparedness kits
    - Communication tools (landlines, cell phones)
    - Backup power sources (if available)
- **Client Communication:**
  - Train staff on effectively communicating with clients and families during emergencies, including:
    - Providing clear instructions and emotional support
    - Respecting client preferences and limitations

Training Methods

- A variety of training methods can be used to engage staff and enhance learning:
  - **Lectures and Presentations:** Delivering key information on emergency preparedness concepts.
  - **Interactive Workshops:** Providing hands-on practice through simulations or case studies.
  - **Video-based Learning:** Utilizing videos to depict emergency scenarios and response strategies.
  - **Online Modules:** Offering online modules for flexible learning and knowledge reinforcement.

Evaluation

- Evaluate the effectiveness of the training by incorporating assessments like:
  - Quizzes or written tests
  - Skills demonstrations
  - Feedback surveys

Additional Considerations

- **Documentation:** Maintain records of all staff emergency preparedness training, including dates, topics covered, and staff attendance.
- **Accessibility:** Ensure training materials and methods are accessible to staff with disabilities.
- **Updates:** Keep training content current by incorporating best practices and any changes in state regulations or agency protocols.

EMERGENCY/DISASTER INFORMATION

HOME HEALTH PATIENTS — PATIENT FORM (PAGE 1 OF 2)

Keep this plan where it can easily be located.

General Instructions to Patient on Use of This Form

This information is provided to you as a quick reference source in case any emergency occurs. Keep this document where it can easily be found. Inform other persons close to you (relative, neighbor, etc.) of its location.

1. **Golden Years Home Health Supported Living LLC** has a nurse on call 24 hours a day. You can reach the nurse through **(253) 487-7217**. After office hours and on weekends an answering service will reach the nurse and he/she will return your call, come see the patient if necessary, or simply answer any questions you may have.
2. In case of a serious medical emergency, call 911. The agency does not operate as an emergency service; therefore, valuable time may be lost by contacting the Agency for an emergency such as a diabetic coma, severe chest pain, unconsciousness, etc.

Name:	Date:
Allergies:	EDP Classification:

In case of Medical Emergency Dial 911

The Emergency Medical Service Dispatcher will need to know:

Your Name: _____

Your Telephone Number: _____

Your Address: _____

List of My Current Medications	List of My Supplies / DME

EMERGENCY/DISASTER INFORMATION — HOME HEALTH PATIENTS (PAGE 2 OF 2)

Emergency Contact Information

In case of emergency please notify the following individual:	
Name:	Phone:
Relationship:	

Disaster Plan Code

Level 1 – 1: Must be seen and/or evacuated ASAP.

My Emergency Plan

I will:	Plan	Details
	☐ Stay home	
	☐ Stay with family or friend	Name and telephone #:
	☐ Evacuate to a shelter	Shelter's name and address:
	☐ Standard	
	☐ Special Needs Registry	
	☐ MMF	
	☐ Evacuate to hospital	

Comments

Describe how services will continue in the event of an emergency:

Physician	Pharmacy
Name:	Name:
Telephone Number:	Telephone Number:
Address:	Address:

ANALYSIS OF RISK ASSESSMENT FOR POTENTIAL DISASTERS

Rate each potential disaster: **None, Low, Moderate** or **High**.

A. Natural Disasters	**Risk Level**
1. Flood	
2. Hurricane	
3. Tornado	
4. Extreme Heat	
5. Winter Storms	
6. Fires – internal	
7. Wildfires	
8. Other – Hail, High Wind, etc.	
B. Technological Hazards	**Risk Level**
1. Widespread Power Outages	
2. Hazardous Materials Incidents	
3. Nuclear Power Plants	
C. Epidemic	**Risk Level**
D. Terrorism	**Risk Level**
1. Foreign	
2. Domestic	

Completed by: _____ **Date:** _____

AGENCY EMERGENCY PREPAREDNESS PLAN EVALUATION

EMERGENCY PREPAREDNESS PLAN TASKS EVALUATED			
Initial Preparation	**Yes**	**No**	**N/A**
Plan has been reviewed within the last 12 months.			
All employees and their alternates responsible for executing work-around for a mechanized process have been identified in the Agency Business Recovery Plan (ABRP) and properly trained.			
All employees responsible for the execution of the ABRP have received ongoing training in Disaster Recovery and Emergency Management.			
The ABRP has been approved by the owner(s) and/or Administrator, and Governing Body.			
The physical and logical security at the alternate site is at least as stringent as the security at the disaster location.			
Preparedness	**Yes**	**No**	**N/A**
Administrative Responsibilities			
Communication:			
• Current list of staff with all applicable contact members in place.			
• Disaster Calling Tree for Patient Emergencies in place and up-to-date.			
• Administrative call in place and effective.			
• Backup communication plan for staff and patients sufficient.			
Patient Triage:			
• Triage list current and correct.			
Secure Office Building:			
• Fire extinguishers and smoke detectors in place and working properly.			
• Exits marked accurately and clearly for emergency routes.			
• Mail safety measures maintained.			
• Essential equipment for keeping the agency open identified.			
• Knowledge of repair or replace damaged equipment demonstrated.			
• Extra supplies stored as planned.			
• Knowledge of shutdown of heating/air condition system demonstrated.			
• All aspects of physical and logical security at the alternate site conform to current regulatory security procedures.			
Information Technology System:			
• Antivirus software and firewall in place.			
• Computer data backed up daily.			

Evaluated by: _____ **Date:** _____

Hazard Vulnerability Assessment (HVA) for a Home Health Agency

Introduction

A Hazard Vulnerability Assessment (HVA) is a critical tool for home health agencies to identify and evaluate potential risks that could impact their operations and the safety of their clients and staff. By conducting a thorough HVA, agencies can develop effective mitigation strategies to minimize the impact of hazards and ensure business continuity.

Steps

1. **Identify Hazards:**
   - **Natural Hazards:** Earthquakes, floods, hurricanes, wildfires, tornadoes, winter storms, etc.
   - **Man-Made Hazards:** Power outages, chemical spills, terrorism, active shooter incidents, etc.
   - **Operational Hazards:** Equipment failures, data breaches, staffing shortages, etc.
2. **Assess Vulnerability:**
   - Evaluate the agency's susceptibility to each identified hazard. Consider factors such as location, infrastructure, resources, and emergency preparedness plans.
   - Determine the potential impact of each hazard on the agency's operations, clients, and staff.
3. **Prioritize Risks:**
   - Rank the identified hazards based on their likelihood of occurrence and potential severity.
   - Focus on mitigating risks that pose the greatest threat to the agency's mission and continuity of operations.
4. **Develop Mitigation Strategies:**
   - Create strategies to reduce the likelihood of hazards occurring or to minimize their impact.
   - Consider measures such as emergency preparedness plans, evacuation procedures, backup systems, and staff training.
5. **Implement Mitigation Strategies:**
   - Ensure that mitigation strategies are implemented effectively and regularly tested.
   - Document all actions taken to address identified risks.
6. **Monitor and Review:**
   - Regularly review and update the HVA to reflect changes in the agency's operations, environment, and regulatory requirements.
   - Continuously monitor for new or emerging hazards and adjust mitigation strategies accordingly.

Key Areas for Assessment

- **Physical Infrastructure:** Assess the vulnerability of buildings, equipment, and infrastructure to natural and man-made hazards.
- **Staff Safety:** Evaluate the agency's ability to protect staff from hazards and ensure their well-being.
- **Client Safety:** Assess the potential risks to clients, including those with mobility limitations or health conditions.
- **Operational Continuity:** Evaluate the agency's ability to maintain essential services during and after a disaster.

- **Data Security:** Assess the risk of data breaches and implement measures to protect sensitive information.

HAZARD VULNERABILITY ASSESSMENT WORKSHEET

Agency: Golden Years Home Health Supported Living LLC **Date:** _____

Mark one box in each group. Score = Probability × Vulnerability × Preparedness (highest possible 27). Review at least annually and after any event.

Event	Probability			Vulnerability / Degree of disruption			Preparedness			Score
	High (3)	Mod. (2)	Low (1)	High (3)	Mod. (2)	Low (1)	Low (3)	Mod. (2)	High (1)	
Ice / Snow										
Flooding										
Earthquake										
Hurricane										
Hazardous Material Accident										
Fire										
Tornado										
Volcano										
Civil Disturbance										
Mass Casualty Event										
Terrorist Attacks										
Pandemic / Infectious Disease										
Electrical Failure										
Communications Failure										
Information System Failure										
Water Failure										
Transportation Interruption										
Environmental / Altered Air Quality Pollution										

Completed by: _____ **Title:** _____

CHECKLIST FOR AGENCY BUSINESS RECOVERY PLAN

CONTINUITY OF OPERATIONS

Agency Business Recovery Plan (ABRP) – Level I (Executive Awareness/Authority)	Yes	No	N/A
ABRP has been developed?			
ABRP has been updated within the last 6 months?			
Agency Business Recovery Plan (ABRP) – Level II (Plan Development and Documentation)			
A classification (level of risk – low, moderate, high) has been assigned to the Business Process/Function/Component that this Agency supports			
ABRP has been documented			
ABRP has been maintained			
ABRP includes the following sections:			
1. Identification			
2. Incident Management			
• Responsible agency officer			
• Personnel responsible for updates			
3. Response			
4. Recovery			
5. Restoration			
6. Plan Exercise			
7. Plan Maintenance			
Business Recovery Personnel/Teams and Contact Information			
ABRP identifies necessary support equipment (documentation/forms, equipment, spare parts, etc.) to recover the Agency and/or functions			
ABRP has an alternative site for recovery			
All critical or important data required to support the agency is being backed up			
Critical/important data is being stored in a protected location off-site			
Agency conducts a walk-through exercise of the Plan at least annually			
The walk-through element exercises have a prepared plan that includes:			
• Description			
• Scope			
• Objective			
A current copy of the ABRP is maintained off-site			
All users of the ABRP have ready access to a current copy at all times			
There is an audit trail of the changes made to the ABRP			

Reviewed by: _____ **Date:** _____

ADMINISTRATOR RESPONSIBILITIES

Purpose

This policy outlines the responsibilities of the Administrator for **Golden Years Home Health Supported Living LLC**, a home health skilled agency licensed in Washington State, in accordance with Washington Administrative Code (WAC) 246-335-515(11) and (12).

Administrator Qualifications and Appointment

- The Administrator will be a home health agency employee possessing the education and experience required by the agency's policies.
- The Administrator will be identified and appointed in writing by the agency's licensee.

Administrator Responsibilities

The Administrator is responsible for the overall leadership and management of the agency, ensuring compliance with WAC 246-335-515 and other applicable regulations. These responsibilities include:

Operational and Financial Management (WAC 246-335-515(11)(a))

- Overseeing the day-to-day operations of the agency, including staffing, scheduling, and service delivery.
- Managing the agency's fiscal affairs, including budgeting, financial reporting, and ensuring sound financial practices.

Plan of Operation Implementation and Updates (WAC 246-335-515(11)(b) & (12))

- Implementing the provisions of WAC 246-335-515 and ensuring all agency operations adhere to its policies and procedures.
- Leading the ongoing review and update of the agency's plan of operation to reflect current practice, services offered, and state and local laws.

Delegation and Supervision (WAC 246-335-515(11)(c) & (d))

- Designating in writing a qualified alternate to act in the Administrator's absence.
- Providing overall management and supervision of all agency services throughout approved service areas, as outlined in WAC 246-335-515(6).

Service Coordination and Contracting (WAC 246-335-515(11)(e) & (f))

- Arranging for necessary services that complement the agency's core offerings and ensure comprehensive patient care.
- Ensuring all contracts with other providers or vendors are current, meet the requirements of WAC 246-335-525(4), and align with the agency's quality standards.

Liaison and Communication (WAC 246-335-515(11)(g))

- Serving as the primary liaison between the agency licensee, personnel, contractors, and volunteers, fostering open communication and collaboration.

Personnel Management and Credentialing (WAC 246-335-515(11)(h))

- Ensuring all agency personnel, contractors, and volunteers possess the necessary current credentials required by the state of Washington, adhering to applicable practice acts and WAC 246-335-525(5).
- Overseeing the development and implementation of programs to ensure continuous competency and skill improvement for agency personnel.

Policy and Procedure Compliance (WAC 246-335-515(11)(i))

- Implementing processes to ensure all personnel, contractors, and volunteers are familiar with and comply with the agency's policies and procedures.

Quality Improvement (WAC 246-335-515(11)(j))

- Leading the implementation and ongoing evaluation of a quality improvement process that aligns with WAC 246-335-555, fostering continuous improvement in the delivery of care.

Recordkeeping (WAC 246-335-515(11)(k))

- Maintaining a comprehensive recordkeeping system that ensures all patient records and agency documentation are accurate, complete, and meet the requirements outlined in WAC 246-335-550.

Supplies and Equipment Management (WAC 246-335-515(11)(l))

- Ensuring the availability, proper maintenance, and functioning order of all supplies and equipment necessary for safe and effective patient care.

Public Information Accuracy (WAC 246-335-515(11)(m))

- Overseeing the accuracy and transparency of all public information materials, including brochures, website content, and marketing materials.

Policy and Procedure Accessibility (WAC 246-335-515(11)(n))

- Ensuring that all current written policies and procedures are readily accessible to personnel, contractors, and volunteers during regular business hours.

POLICY DURING THE ABSENCE OF THE ADMINISTRATOR

WAC 246-335-515

Statement of Purpose

(11) The applicant or licensee must identify an administrator. The administrator must be a home health employee and possess education and experience required by the agency's policies. The administrator is responsible to:

(c) Designate in writing an alternate to act in the administrator's absence;

Policy During the absence of the Administrator,

During the absence of the Administrator, will be the designated person who will assume all administrative responsibilities for the operation of the Agency.

Ahmed M Ahmed MD MPH, Alternate Administrator, will carry a cell phone to assure availability when needed.

Assignment of responsibility during the absence of the Administrator

I,, Administrator for the Agency, do hereby give full administrative authority to Ahmed M Ahmed MD MPH, Alternate Administrator, during my absences from the premises of the Agency.

I,, Administrator, have been appointed Disaster Coordinator and has been appointed Alternate Disaster Coordinator.

, Administrator, Disaster Coordinator

Accepted:, Alternate Administrator, Alternate Disaster Coordinator

Assignment of Responsibility during the absence of the Director of Clinical Services

I,, Director of Clinical Services, do hereby give clinical authority to, Alternate Director of Clinical Services, during my absences from the premises of the Agency.

, Director of Clinical Services

Accepted:, Alternate Director of Clinical Services.

WAC 246-335-520 DELIVERY OF SERVICES

ADMISSION, TRANSFER & DISCHARGE POLICY

WAC 246-335-520

Statement of Purpose

1) Admission, transfer, discharge, and referral processes: (a) In order to minimize the possibility of patient abandonment, patients must be given at least a f**orty-eight-hour written or verbal notice prior to discharge** that will be documented in the patient record.

Notice Delivery to Representatives

1. If the provider is unable to personally deliver a notice of non-coverage to a person legally acting on behalf of a beneficiary, then the provider should telephone the representative to advise him/her when the beneficiary's services are no longer covered.
2. The beneficiary's appeal rights must be explained to the representative, and the name and telephone number of the appropriate quality improvement organization (QIO) should be provided.

The date of the conversation is the date of the receipt of the notice. Confirm the telephone contact, by written notice, mailed on that same date.

1. Place a dated copy of the notice in the beneficiary's medical file, and document the telephone contact to include, name of person initiating the contact, name of the representative contacted, date and time of the contact, and the telephone number called.
2. When direct phone contact cannot be made, send the notice to the representative by certified mail, return receipt requested.
3. The date that someone at the representative's address signs (or refuses to sign) the receipt, is the date of receipt.
4. When notices are returned by the post office with no indication of a refusal date, then the beneficiary's liability starts on the second working day after the provider's mailing date.

Patients Admission

1. **Time Frame for Initiation of Services:** Supervisor will ensure timely and efficient patient admissions while adhering to regulations and quality standards. We strive to initiate the services within seven calendar days of receiving and accepting a physician or practitioner referral for services. The supervisor will **review and approve** referrals to ensure they meet eligibility criteria and contain all necessary information for a proper initial assessment.
2. The supervisor will **monitor** the progress of the admission process to ensure services are initiated within the seven-day timeframe whenever possible.
3. **Exceptions to Time Frame:** A longer timeframe for initiating services may be permitted when documented in writing and justified by one or more of the following reasons:

a. Physician or Practitioner Request: The physician or practitioner may request a delay in service initiation due to changes in the patient's medical condition or a need for additional testing or evaluation.
b. Patient or Representative Request: The patient, designated family member, legal representative, or referral source may request a delay in service initiation for various reasons, such as scheduling conflicts, coordination with other services, or time needed to prepare the home environment.
c. Difficulty Contacting Patient or Representative: In rare instances, delays may occur due to challenges contacting the patient, designated family member, or legal representative for essential information or consent. We will document all attempts made to contact the patient or representative.

4. **Communication:** We are committed to clear communication with all parties involved.
   a. We will promptly notify the physician or practitioner of any anticipated delays in service initiation.
   b. We will keep the patient or representative informed about the expected start date for services and any potential delays.
   c. We will document all communication and justifications for delays in the patient's medical records.

Admission of Patients for Home Health Services

This policy outlines the criteria and procedures for admitting patients to our home health services program, ensuring compliance with WAC regulations and timely service initiation.

1. **Referral and Eligibility**
   - Patients can be admitted to our home health services program upon receiving a valid referral from a physician or practitioner.
   - The referral must document the patient's medical condition, need for home health services, and anticipated plan of care.
   - A qualified staff member will assess the patient's eligibility for services based on:
     - **Medical Necessity:** The patient requires skilled nursing care, therapy services, or other home health services to safely manage their condition at home.
     - **Homebound Status:** The patient's medical condition limits their ability to leave home for outpatient services without significant difficulty.
     - **Insurance Coverage:** The patient has insurance coverage that benefits home health services, or they qualify for financial assistance programs.
2. **Timeframe for Initiation of Services**
   - Home health services will generally be initiated within **seven calendar days** of receiving and accepting a valid referral.
   - This timeframe emphasizes prompt service delivery to meet the patient's needs.
3. **Exceptions: Physician/Practitioner Requested Delay**
   - Longer timeframes for initiating services may be permitted, but must be documented in writing by the referring physician or practitioner.
   - Acceptable justifications for a delay may include:
     - Waiting for additional diagnostic test results.
     - Coordinating with other healthcare providers involved in the patient's care.

- Allowing time for the patient's condition to stabilize.
- Scheduling services around planned procedures or hospital discharges.

4. **Admission Process**
   - Upon receiving a referral, a qualified staff member (RN, LPN, Social Worker) will conduct a comprehensive initial intake assessment.
   - The assessment will gather information about the patient's:
     - Medical history
     - Current health status
     - Activities of daily living (ADLs)
     - Instrumental activities of daily living (IADLs)
     - Social determinants of health
     - Functional limitations
     - Preferences and goals
   - Based on the initial intake assessment, a qualified healthcare professional (RN, PT, OT, Speech Therapist) will develop a client-centered plan of care (POC) in collaboration with the client, their family (when appropriate), and other healthcare providers.
   - The POC will include:
     - Measurable goals and objectives
     - Specific interventions and services needed
     - Frequency and duration of services
     - Disciplines involved in providing care
     - Discharge criteria
   - The patient or their legal representative will be informed of their rights and responsibilities, and will be asked to sign a consent form for services.
5. **Communication**
   - The home health agency supervisor will communicate effectively with the patient, their physician or practitioner, and family members throughout the admission process.
   - This communication may involve phone calls, written correspondence, or secure electronic messaging.
6. **Compliance**
   - This policy ensures compliance with WAC regulations regarding timely initiation of home health services, while also acknowledging physician or practitioner discretion in certain situations.
   - Staff will be trained on this policy and will be updated on any regulatory changes.

Discharge / Transfer Policy

In the following policy, the term “Discharge” is synonymous with “Termination of Services.”

1. The Agency must develop and implement an effective transfer and discharge planning process for patients who are transferred to another agency or who are discharged to a Skilled Nursing Facility (SNF), Inpatient Rehabilitation Facility (IRF), or Long-Term Care Hospital (LTCH).

2. The Agency must assist patients and their caregivers in selecting a post-acute care provider by using and sharing data that includes, but is not limited to HHA, SNF, IRF, or LTCH data on quality measures and data on resource use measures. The Agency must ensure that the post-acute care data on quality measures and data on resource use measures is relevant and applicable to the patient's goals of care and treatment preferences.
3. The HHA must send all necessary medical information pertaining to the patient's current course of illness and treatment, post discharge goals of care, and treatment preferences, to the receiving facility or health care practitioner to ensure the safe and effective transition of care.
4. This broad, flexible requirement allows home health agencies to tailor the exchange of information to the exact circumstances and needs of the care transition in order to support the patient's post-discharge goals.
5. Continue to send Discharge summary to post-discharge provider.
6. Agencies will be required to provide data to the patient and caregiver on quality and resource use measures that are relevant to the patient's goals of care and treatment preferences. This applies for patients discharged to a skilled nursing facility (SNF), inpatient rehabilitation facility (IRF) or long-term care hospital (LTCH). **Transfers to hospitals are not included in the requirement for home health agencies.**
    d. Instead of a specified list, the Agency must send necessary medical information pertaining to the patient's current course of illness and treatment, post- discharge goals of care, and treatment preferences to the receiving facility of the health care practitioner to ensure the safe and effective transition of care.
    e. The Agency is required to comply with requests for additional necessary clinical information made by the receiving facility or health care practitioner, which may include items such as a copy of the patient's current plan of care or latest physicians 'orders.
    f. Information you have to share outcomes with:
        a. Home Health Agencies
        b. Skilled Nursing Facilities
        c. Inpatient Rehabilitation Facilities
        d. Long Term Acute Care Hospital
7. In order to minimize the possibility of patient abandonment, patients must be given at least a forty-eight-hour written or verbal notice prior to discharge that will be documented in the patient record. Forty-eight-hour notice is not required if home health agency worker safety, significant patient noncompliance, or patient's failure to pay for services rendered are the reason(s) for the discharge.
8. The Agency will maintain a process for the ongoing assessment of each patient/client's continuing care and discharge planning needs. This is required to ensure that patient/client discharges are adequately planned to terminate services when the patient/client no longer has a need, or desire, for care, and to ensure that the patient/client's continuing care needs are met, and that the patient/client participates in the discharge planning process. The reason for discharge is discussed and charted.
9. The Agency determines, under a policy, the circumstances when a patient would be transferred to another organization or discharged. The patient, and patient representative (if any), have the right to be informed of the Agency's policies and procedures on transfers and discharges. The Agency may only discharge, or transfer a patient, from the Agency, if:

a. The transfer, or discharge, is necessary for the patient's welfare because the Agency, and the physician who is responsible for the Home Health plan of care, agree that the Agency can no longer meet the patient's needs, based on the patient's acuity. The Agency must arrange a safe, and appropriate, transfer to other care entities when the needs of the patient exceed the Agency's capabilities;
b. The patient moves out the Agency's geographic service;
c. The patient requires care/service not provided by the Agency, or the Agency is not a preferred provider, by the patient's insurance company;
d. The patient, or payor, will no longer pay for the services provided by the Agency;
e. The transfer, or discharge, is appropriate because the physician, who is responsible for the Home Health plan of care, and the Agency, agree that the measurable outcomes and goals, set forth in the plan of care, have been achieved, and the Agency, and the physician who is responsible for the Home Health plan of care, agree that the patient no longer needs the Agency's services;
f. The patient refuses services, or elects to be transferred, or discharged;
g. The patient is receiving the same services from another Agency. The Agency will notify the referring, or ordering, physician of the existence of the other agency providing care;
h. The physician consistently fails to sign the plan of treatment in the required time period, does not renew the Plan of Care at the 60-day interval, or gives orders that are not consistent with the stated diagnosis;
i. The patient is no longer homebound.

10. **Discharge for Cause:** The Agency determines, under a policy set by the Agency, for the purpose of addressing discharge for cause, that the patient's (or other persons in the patient's home) behavior is disruptive, abusive, or uncooperative, to the extent that delivery of care to the patient, or the ability of the Agency to operate effectively, is seriously impaired. The Agency must do the following before it discharges a patient for cause:
    a. Advise the patient, representative (if any), the physician(s) issuing orders for the Home Health plan of care, and the patient's primary care practitioner, or other healthcare professional, who will be responsible for providing care and services to the patient, after discharge from the Agency (if any), that a discharge for cause is being considered;
    b. Make efforts to resolve the problem(s) presented by the patient's behavior, the behavior of other persons in the patient's home, or situation;
    c. Provide the patient, and representative, if any, with contact information for other Agencies, or providers, who may be able to provide care; and
    d. Document the problem(s) and efforts made to resolve the problem(s) and enter this documentation into its clinical records.
11. The patient dies; or
12. The Agency ceases to operate.

Discharge/Transfer Procedure

1. Upon termination of services, the Agency will:
    a. Identify goals achieved for the patient/client.
    b. Consult with the physician prior to discharging patient/client who has not attained goals due to:

   i. Non-compliance.
   ii. The patient is no longer eligible for treatment.
c. Discharge must be approved by the Director of Nursing.
d. A discharge summary note is written, and a copy is sent to the physician within 5 days. The physician is also notified of discharge by phone call to the office or by fax.
e. Discharge date is the date of the last chargeable visit.
f. Patient/client/family must be adequately informed of discharge, as noted in the clinical narrative.
g. Patient/client who is hospitalized- several factors, including where it falls in the episode, and length of hospital stay, will determine if the patient is placed on hold or discharged.
h. A plan shall be developed, or a referral made, for any continuing care indicated.

2. Discharge planning shall begin at the time of admission, with patient/client included in the process, and being advised as to the expected duration of treatment. Re-evaluation by the RN, and additional planning with the patient/client, shall occur throughout the course of care, and shall include documentation of specific plans and the expected date of discharge.
3. Patient/family will be informed of the discharge, orally and in writing, at least five (10) days prior to discharge. The following will be discussed:
   a. The date of the discharge
   b. The reason for the discharge
   c. Contact information for the receiving facility if the patient is to be transferred.
   d. Expectation that they will participate in their discharge planning process.
   e. To receive at least 10 calendar days advance written notice of the intent of the home care agency to terminate services. Less than 10 days advance written notice may be provided in the event the consumer has failed to pay for services, despite notice, and the consumer is more than 14 days in arrears, or if the health and welfare of the direct care worker is at risk. Written notice must include the reason for termination/transfer.
4. Discharge plans will be coordinated with other care/service providers, as applicable.
5. All patients/clients will receive discharge instructions, regarding their ongoing care needs, prior to the final visit.
6. All discharged patients will have required documentation to ensure appropriate communication is provided to the physician, as requested, and to the receiving Agency.
7. Prior to discharging the patient, the attending physician shall be notified. A written discharge summary, which must be completed, and a copy sent to the physician within 5 business days of discharge, will be maintained in the clinical record. The discharge summary will include:
   a. Patient identifying information
   b. Patient's physician and phone number
   c. The reason for discharge
   d. The date of discharge
   e. The patient's physical and psychosocial status at the time of discharge
   f. The patient's diagnosis
   g. A summary of the care and services provided

   h. Patient progress toward desired goals
   i. Instructions, and referrals, provided to the patient
   j. Contact information for the receiving facility if the patient is to be transferred
   k. Expectation that the patient will participate in their discharge planning process
8. At least, 2 days before the discharge (final) visit, the patient/client will receive the notice to discharge, with instructions on the purpose of the form.
9. Home health services starting within seven calendar days of receiving and accepting a physician or practitioner referral for services. Longer time frames are permitted when one or more of the following is documented:
   a. Longer time frame for the start of services is requested by physician or practitioner
   b. Longer time frame for the start of services is requested by the patient, designated family member, legal representative, or referral source
   c. Start of services was delayed due to agency having challenges contacting patient, designated family member, or legal representative
10. When the patient/client is transferred to another organization or facility, the patient/client is informed, in a timely manner, of the need for transfer and/or level of care and of the alternatives. The patient/client, and family, have input into these decisions. They are notified of any financial benefit to the referring home care. Relevant information regarding the patient/client's condition and care requirements will be provided verbally, and in writing, to the facility, within 48 hours of the Agency becoming aware of the transfer.

Transfer Summary

1. A transfer summary is completed, and maintained, in the patient record, and a copy is forwarded to the receiving organization. A transfer summary includes, but is not limited to:
   a. Date of transfer
   b. Patient identifying information and emergency contact
   c. Destination of patient transferred
   d. Emergency contact
   e. Destination of patient transferred
   f. Date and name of person receiving report
   g. Patient's physician and phone number
   h. Diagnosis related to the transfer
   i. Significant health history
   j. Transfer orders and instructions
   k. A brief description of services provided, and ongoing needs that cannot be met
   l. Status of patient, at the time of transfer
2. A completed transfer summary is sent within two business days of a planned transfer, if the patient's care will be immediately continued in a healthcare facility; or
3. A completed transfer summary is sent within two business days of becoming aware of an unplanned transfer, if the patient is still receiving care in a healthcare facility, at the time, when the Agency becomes aware of the transfer.

Interruption of Services

When an interruption of services occurs during a patient/client's certification period, a form is to be completed and placed in the clinical record, with previous progress notes, to clarify why visits have not occurred. This will apply to instances in which the patient/client has been admitted to the hospital, or to any other situation that creates the need for a brief, temporary hold on Agency services. It does not apply to a one-time missed visit occurrence.

1. When an interruption of services occurs, the following procedure should be followed:
    a. Complete the 'Interruption of Services' form, which includes an explanation as to why services must be temporarily halted, and anticipated length of hold, if known.
    b. The Agency will maintain a log of hospitalized patients/clients.
    c. Request notification of patient/client discharge from the hospital, in order to resume home care, if appropriate.
2. Once the patient/client has been released from the hospital, and Home Health services have resumed, the form is completed, and an interim nursing assessment is performed by an RN to determine whether or not there has been a significant change in the patient/client's needs.
3. If the patient/client is in the hospital at the time of recertification, the patient/client will be discharged from Home Health services and then readmitted to Home Health when discharged from the hospital, in order to continue home care services.
4. If the patient/client expires while in the hospital, the discharge will be completed to reflect this occurrence, and the clinical record will be closed.

Completion of Discharged Records

In order to verify closure of inactive records, all clinical records will be completed and audited within thirty days of discharge as follows:

1. The clinical record of a discharged patient/client is fully completed, including a discharge summary, in five working days.
2. Each clinical record is audited, within 30 days, and returned to the secretary for filing.

DISCHARGE OF PATIENTS IN UNSAFE SITUATIONS

The safety of field staff is of primary importance. If, in any way, this safety would be compromised, the case (after all efforts to resolve the issues have been exhausted) will be closed, and the patient, and/or responsible caregivers, will be notified.

1. In any situation where a field staff person feels immediate danger, and/or a threat to safety, the home, or area, should be left, at once, and the supervisor notified.
2. If there are ongoing unsafe situations in the home, or area, which the field staff observes, this should be brought to the attention of the supervisor, as soon as possible. Examples of unsafe situations include:
    a. Drug dealing
    b. Firearms, which are visible and available
    c. Persons in the home, or proximity, who exhibit violent, agitated, or threatening behavior

d. Environmental issues, e.g., vermin, open flames near oxygen cylinders, animal droppings, etc.
e. Animals that are not locked away. If injury occurs involving an animal attacking a patient/client or employee, the following procedure will be implemented:
    i. The employee will immediately alert his/her supervisor regarding the injury
    ii. Report the incident to the county animal control office
    iii. Notify the patient's physician (if the patient is injured) and carry out orders if any
    iv. Document the incident in the patient/client's clinical record (if patient is injured)
    v. Complete an Incident/Accident Report form and submit it to the Administrator
    vi. If the employee is injured, the employee is sent to the emergency room, or the hospital of choice, for treatment.

3. A patient care conference, with the participation of all appropriate disciplines, will be held to discuss the situation and any appropriate actions that could be taken.
4. Documentation will include a description of the situation, any discussions and communications with the patient, caregiver, organizational staff, community resources, etc., and any actions to be taken.
5. **If the decision is made to discharge the patient/client, the following steps will**

be taken:

a. The patient/client will be notified, by administration, of the decision to discharge, the reason, and the date of the last visit, by certified mail. Whenever possible, the patient/client will be given time to secure other modes of care or placement.
b. The physician involved, and any appropriate referral source or community resources, will also be informed by telephone and certified mail.
c. A summary of the situation attempts to resolve it, and the action taken will be documented and placed in the chart.

Care of the Dying Patient

1. The Agency is sensitive to the needs of the dying patient/client, and his/her caregivers, as a family unit, and responds to the patient/client's unique, and individual, needs within the home care setting. To ensure respectful, responsive care of the dying patient/client, and his/her caregivers, care is directed at optimizing comfort, and dignity, as well as responding to the psychosocial, emotional, and spiritual needs of the patient/client and his/her caregivers.
2. Agency staff members assist the patient/client in managing pain, and symptoms, by implementing interventions that appropriately alleviate, and/or control, pain and by monitoring the patient/client's level of control. The Wong-Baker Faces pain rating scale will be utilized to rate pain.
3. The Agency will identify secondary symptoms, determine if these symptoms are responsive to treatments, and then take the actions necessary to manage the symptoms.
    a. Consider the need for the social worker to council family on the death process.
    b. Inform patient/client and family about Hospice Services.

CONSENT PRACTICES

WAC 246-335-550

Statement of Purpose

The licensee must, (3) Except as provided in subsection (4) of this section, include documentation of the following in each record:

(b) Patient's consent for services, care, and treatments;

Purpose:

To provide a consistent process for obtaining authorizations for treatment and related services.

Policy:

1. Authorizations are obtained prior to the initiation of treatment from the patient or his or her legal representative in a manner that is consistent with state law.
2. Patients will be provided and sign a consent, on or before the start of care, that includes consent for services, care and treatments, the payment source and the patient's responsibility for payment, if any.
3. All consents will be retained in the patient record.

Procedure:

1. Treatment authorizations are obtained prior to the initiation of care from either the patient or a legal representative.
2. Treatment authorizations address the following items:
   a. The nature and purpose of the proposed care.
   b. The probable risks and probable benefits of proposed care.
   c. The availability, if any, of treatment alternatives and their attendant risks and benefits.
   d. The risks of foregoing proposed care and alternative forms of treatment.
   e. Informed consent.
   f. Proposed frequency/duration.
3. Treatment authorization practices are modified to meet state regulatory requirements.
4. A request is made to the patient or legal representative to involve family caregivers in the consent process.
5. A consent document is signed by the patient or legal representative including, where appropriate, the authorization to involve family caregivers in the consent process.

REFUSAL OF CARE

WAC 246-335-535

Statement of Purpose

A home health agency at the time of admission must provide each patient, designated family member, or legal representative with a written bill of rights affirming each patient's right to:

(6) Refuse specific treatments or services

Purpose:

To establish a process for the patient to exercise the right to refuse service or treatment.

Policy:

The right of the patient to refuse treatment is respected and documented in a manner that facilitates communication of such a decision.

Procedure:

1. When a patient declines care or treatment, the determination is made after an attempt has been made to explain the risks and benefits of such a decision and the consequences of it.
2. An explanation of the risks, benefits, and consequences of a refusal of care is provided to the patient and documented in the clinical record.
3. When treatment decisions are made by a legal representative, an explanation of the risks, benefits, and consequences are provided and documented in the clinical record.
4. The Authorizing Practitioner is notified of the patient's decision to decline care or treatment.
5. Refusal of care determinations are respected to the extent permitted under applicable state law.

SPECIFIC HOME HEALTH SERVICE INCLUDING ANY NON MEDICAL SERVICES

Purpose

This policy outlines the home health services, including non-medical services, offered by **Golden Years Home Health Supported Living LLC**, a home health skilled agency licensed in Washington State. These services are designed to meet the individual needs of our patients and their families, as identified in their personalized plans of care.

Scope of Services

We offer a comprehensive range of home health services to support patients in their homes and promote their independence. These services can be categorized as:

- **Skilled Services:** Provided by licensed healthcare professionals, such as nurses, therapists, and social workers, to address specific medical needs and treatment plans.
- **Non-Medical Services:** Designed to assist patients with daily living activities and enhance their overall well-being.

Non-Medical Services may include

- **Personal Care:** Assistance with bathing, dressing, grooming, toileting, and other activities of daily living (ADLs).
- **Homemaker Services:** Light housekeeping tasks such as laundry, meal preparation, and errands.
- **Companionship:** Providing emotional support and social interaction to reduce isolation.
- **Respite Care:** Providing temporary relief for family caregivers.
- **Safety Monitoring:** Checking on patients at regular intervals to ensure their well-being.
- **Nutritional Counseling:** Guidance on maintaining a healthy diet to promote healing and well-being.
- **Education and Support:** Providing patients and families with information and resources related to their condition.

Determining Service Needs

The specific services provided will be determined based on the individual needs of each patient, as identified during the initial assessment and documented in the plan of care. This plan is developed collaboratively by the patient, their family (if applicable), the physician, and the home health team. The plan of care will be reviewed and updated regularly to reflect any changes in the patient's condition or needs.

Additional Considerations

- Non-medical services may be provided by trained home health aides or personal care assistants.
- The agency will work with patients and their families to identify and secure additional resources or services not directly provided by the agency, if needed.
- Cultural and religious preferences will be considered when developing the plan of care and selecting services.

The agency is committed to providing a comprehensive range of home health services that meet the individual needs of our patients and families. This policy ensures that all personnel involved in patient care understand the services offered and the importance of person-centered care.

TIMELY INITIATION OF HOME HEALTH SERVICES

WAC 246-335-520

This policy outlines the procedures for initiating home health services in accordance with regulations, ensuring services begin within a timely manner while considering physician or practitioner requests.

1. **Timeframe for Initiation of Services**
   - Home health services will generally be initiated within **seven calendar days** of receiving and accepting a valid referral from a physician or practitioner.
   - This timeframe emphasizes prompt service delivery to meet the patient's needs.
2. **Exceptions**
   - Longer timeframes for initiating services may be permitted when documented in writing by the referring physician or practitioner.
   - Acceptable justifications for a delay may include:
     - Waiting for additional diagnostic test results.
     - Coordinating with other healthcare providers involved in the patient's care.
     - Allowing time for the patient's condition to stabilize.
     - Scheduling services around planned procedures or hospital discharges.
     - Longer time frame for the start of services is requested by the patient, designated family member, legal representative, or referral source.
     - Agency having challenges contacting patient, designated family member, or legal representative
3. **Documentation Requirements**
   - If a delay in service initiation is requested by the physician or practitioner, the following documentation must be included in the patient's medical record:
     - A written justification for the delay, including the specific reason and anticipated start date for services.
     - The physician's or practitioner's signature and date.
4. **Communication**
   - The home health agency will communicate effectively with the physician or practitioner regarding the referral and any potential delays in service initiation.
   - This communication may involve phone calls, written correspondence, or secure electronic messaging.
5. **Compliance**
   - This policy ensures compliance with regulations requiring timely initiation of home health services, while also acknowledging physician or practitioner discretion in certain situations.

Staff will be trained on this policy and will be updated on any regulatory changes.

ROLES & RESPONSIBILITIES WITH MEDICATION ASSISTANCE

Medication Assistance

- **Reviewing Medication Schedule:** Caregivers can review a client's medication schedule with them to ensure they understand the medications they are taking, the dosage, and the frequency.
- **Preparing Medication:** Caregivers can help clients prepare their medications for easy access, such as sorting pills into pill boxes or using blister packs.
- **Reminding Clients to Take Medications:** Caregivers can remind clients it's time to take their medications, but cannot pressure them.
- **Reporting Concerns:** Caregivers can report any concerns they have about a client's medication use to a supervisor or registered nurse (RN) who can then assess the situation.

Important Note: Caregivers **cannot** perform the following medication administration tasks:

- **Dispensing Medications:** Caregivers cannot physically remove medications from a prescription bottle or packaging.
- **Administering Medications:** Caregivers cannot give medications to clients orally, topically, or through injections.

Who Can Administer Medications?

- **Registered Nurses (RNs):** RNs employed by the home care agency or working independently can administer medications based on a doctor's orders.
- **Licensed Practical Nurses (LPNs):** LPNs can administer medications under the supervision of an RN, following a doctor's orders and the agency's policies.

Additional Responsibilities

- **Maintaining Documentation:** Caregivers may be responsible for documenting medication assistance activities, such as reminding clients or reporting concerns.
- **Client Education:** Caregivers can help educate clients about the importance of medication adherence and potential side effects (but should not provide medical advice).

Management of Patient Medications and Treatments

Medication Management

- **Medication Orders:** Home health nurses will only administer medications as ordered by a licensed physician or practitioner.
- **Verification:** Nurses will verify the following information before administering each medication:
  - Patient name, medication name, dosage, route, and frequency.
  - Any allergies or contraindications documented in the patient's medical record.
  - Expiry date of the medication.
- **Administration:** Medications will be administered using the prescribed route (e.g., oral, injection) and documented in the patient's medication record.
- **Patient Education:** Nurses will educate patients and their caregivers about their medications, including expected benefits, potential side effects, and proper storage.

- **Reporting:** Any adverse medication reactions or medication errors will be reported immediately to the supervising physician or practitioner and documented in the patient's medical record.

Treatment Management

- **Treatment Orders:** Home health staff will only provide treatments as ordered by a licensed physician or practitioner.
- **Competency:** Staff will perform treatments only within their scope of practice and competency.
- **Documentation:** All treatments provided will be documented in the patient's medical record, including the date, time, and details of the treatment performed.
- **Patient Education:** Patients and their caregivers will be educated about the purpose of each treatment and any potential risks or side effects.
- **Communication:** Staff will communicate any concerns regarding a patient's progress or ability to tolerate treatments to the supervising physician or practitioner.

Collaboration

- We will collaborate with the patient's physician or practitioner to ensure continuity of care and medication management.
- We will communicate regularly with the patient and their family/representatives regarding their medications and treatments.
- We will work with the pharmacy to ensure timely medication delivery and address any medication dispensing issues.

Compliance with Practice Acts

- We are committed to adhering to all applicable Washington State practice acts and regulations governing medication administration and treatment provision by home health professionals.
- Staff will receive ongoing training to stay updated on current regulations and best practices in medication and treatment management.

COORDINATION OF CARE

WAC 246-335-545

Coordination Among Services

In the event an agency offers both home health, home care, and hospice services, ensuring seamless coordination among these services is crucial. Here's how:

- Interdisciplinary Care Team (IDT): Establish a team with representatives from all service categories (RNs, LPNs, CNAs, social workers, therapists, home health aides, hospice aides, chaplains).
- Comprehensive Care Plans: Develop integrated care plans for each client, incorporating services from all relevant categories.
- Communication Strategies: Facilitate communication through:
  - Regular IDT Meetings: Discuss cases, coordinate plans, and address challenges.
  - Electronic Medical Records (EMRs): Use a shared system for real-time information access and updates.
  - Standardized Protocols: Establish clear communication protocols for frequency, format, and reporting.

Coordination with Other Agencies

Effective care often involves collaboration with external agencies. Here's how to achieve this:

- Identify Relevant Agencies: Consider entities like hospitals, specialists, community centers, and mental health providers based on client needs.
- Develop Referral Processes: Establish clear procedures for referring clients to external agencies when necessary.
- Communication Channels: Maintain communication channels with these agencies through secure methods like faxes, emails, or electronic health information exchange (HIE) systems (with patient authorization).
- Information Sharing: Share necessary client information with external agencies, following HIPAA privacy regulations.

Actions to Address Patient or Family Communication Needs

Clear communication is vital for patient and family satisfaction. Here are some key actions:

- Client/Family Preferences: Identify preferred communication methods (phone, email, in-person meetings) for each client/family.
- Language Needs: Accommodate language barriers by providing translated materials or interpreters.
- Accessibility: Ensure communication materials are accessible to individuals with disabilities (large print, Braille).
- Culturally Sensitive Communication: Respect cultural backgrounds and beliefs when communicating with clients and families.
- Open Communication: Encourage open communication and address client/family questions and concerns promptly and honestly.
- Care Plan Involvement: Involve clients and families in developing and understanding the care plan.
- Regular Updates: Provide regular updates on the client's condition and any changes in care.
- Emotional Support: Offer emotional support and address anxieties or fears related to the client's care.

TELEHEALTH / TELEMEDICINE

WAC 246-335-525

Statement of Purpose

The applicant or licensee must develop and operationalize personnel, contractor, and volunteer policies and procedures that describe: (7) **Utilization** of **telehealth or**

telemedicine for patient consultation purposes or to acquire patient vitals and

other health data in accordance with state laws and (11) Training on the use of telehealth or telemedicine for patient consultation and the transmission of health data

Policy Statement

Our agency is committed to leveraging technology to expand access to quality healthcare. We achieve this by ensuring our personnel are proficient in using telehealth and telemedicine platforms for patient consultations and the transmission of health data while adhering to all applicable regulations.

Applicability

This policy applies to all personnel involved in providing patient care through telehealth or telemedicine services, including but not limited to:

- Physicians
- Advanced Practice Providers
- Nurses
- Therapists
- Other healthcare professionals involved in patient consultations

Training Requirements

1. **Core Telehealth and Telemedicine Training:**
    - All personnel involved in telehealth or telemedicine must complete a core training program before initiating patient care using these technologies.
    - The core training will cover, at a minimum:
        - Definition and types of telehealth and telemedicine
        - Benefits and limitations of telehealth
        - Legal and ethical considerations for telehealth encounters (e.g., informed consent, privacy regulations)
        - Establishing and maintaining patient-provider relationships in a virtual setting
        - Technical skills for using telehealth platforms (e.g., video conferencing, secure messaging)
        - Troubleshooting common technical issues
        - Data security and privacy protocols for telehealth interactions
        - Emergency procedures and contingency plans for technical failures
2. **Specialty-Specific Training:**

- In addition to core training, personnel may require additional training specific to their healthcare discipline and the types of telehealth services they will be providing. This training may cover:
  - Specific clinical protocols for virtual assessments and interventions
  - Use of telehealth technology for specific procedures or specialties
  - Effective communication techniques in a virtual environment

3. **Ongoing Training:**

As telehealth technologies and regulations evolve, the agency will provide ongoing training opportunities for personnel to maintain their competency and knowledge.

Training Delivery and Documentation

- Training programs will be developed and delivered through a variety of methods, including online modules, in-person workshops, or a combination of both.
- All personnel will be required to complete training and demonstrate competence before using telehealth or telemedicine with patients.
- The Human Resources department will maintain documentation of completed training programs and individual completion records.

Responsibilities

- **Department Heads:**

  Identify personnel who require telehealth/telemedicine training based on their roles.

- **Training Department:**
  - Develop and maintain comprehensive training programs for core and specialty-specific telehealth/telemedicine skills.
  - Deliver training sessions or partner with external providers for program delivery.
- **Personnel:**

- Complete all required telehealth/telemedicine training programs.

Ask questions and seek clarification on training content as needed.

EMERGENCY CARE POLICY

This policy outlines the procedures for responding to and managing emergency situations involving patients receiving home health services from **Golden Years Home Health Supported Living LLC**, a home health skilled agency licensed in Washington State. The primary goal is to ensure the safety and well-being of our patients during emergencies.

Emergency Preparedness

- All home health personnel, including nurses, therapists, and aides, will receive ongoing training in emergency preparedness and response. This training will cover:
  - Identifying and responding to various types of emergencies, such as cardiac arrest, stroke, choking, falls, and fires.
  - Basic life support (BLS) skills, including CPR and airway management.
  - Proper use of emergency communication equipment.
  - Evacuation procedures for the patient's home in case of fire or other emergencies.
- The agency will maintain a readily available emergency preparedness kit in each agency vehicle. The kit will contain essential supplies for emergencies, such as:
  - First-aid supplies
  - Oxygen
  - Blood pressure cuff
  - Thermometer
  - Stethoscope
  - Automated External Defibrillator (AED) (if applicable)
- A list of emergency contact information, including local emergency numbers, the patient's physician, and the agency's on-call staff, will be prominently displayed in each agency vehicle and readily accessible to all personnel.

Emergency Response Procedures

- In the event of a patient emergency, the home health personnel on-site will take the following steps:
  - Immediately assess the situation and ensure the safety of themselves and the patient.
  - If necessary, activate the Emergency Medical Services (EMS) by calling 911.
  - Provide basic life support (BLS) care if trained and equipped to do so, following their training guidelines.
  - Remain with the patient until emergency personnel arrive.
  - Document the event in detail, including the nature of the emergency, actions taken, and patient response.
- The agency's on-call staff will be notified immediately of any patient emergency. They will be responsible for:
  - Coordinating with emergency personnel and the patient's physician.
  - Contacting the patient's family or emergency contact person.
  - Providing support and assistance to the patient's family during the emergency.

- Upon a patient's return home after an emergency hospitalization, the agency will work collaboratively with the patient's physician and family to update the plan of care, ensuring appropriate support and education are provided to prevent similar emergencies.

Communication and Reporting

- All patient emergencies will be documented in the patient's medical record and reported to the agency's management team.
- The agency will conduct a thorough review of all patient emergencies to identify any potential trends or areas for improvement in emergency preparedness and response procedures.
- The results of the review will be used to update training materials and emergency response protocols as needed.

RESUSCITATION AND EMERGENCY SERVICES

Our Agency supports your right to self-determination. Advance Directives, such as Living Wills and Do Not Resuscitate (DNR) orders, will be followed when provided, in writing.

If documentation has not been provided, then every person admitted to the Agency shall be presumed to consent to the administration of cardio-pulmonary resuscitation in the event of cardiac or respiratory arrest, unless another decision has been made, by the patient/client or surrogate-decision maker.

Emergency Protocols

Emergency protocols are used only when it is impossible to immediately contact your attending physician and your physician has not specified treatment to be followed.

The 911/EMS number is activated in emergency situations including, but not limited to:

- Cardiac Arrest
- Choking
- Convulsions/Seizures
- Anaphylactic Reactions

In addition, if you do not respond to the doorbell, or knocking, our employee will obtain assistance to enter your home, by superintendent, if you live in an apartment, or a family member/caregiver. If we are unable to obtain assistance to enter your home, then the employee will use their own judgment in calling the police. If the employee is able to look inside your home and identify that you are on the floor, they will immediately call 911.

ACTIONS TO BE TAKEN UPON DEATH OF A PATIENT

Staff members of the Agency will not engage in the pronouncement of death. Agency Registered Nurse (RN) will contact the M.D. for appropriate instructions that may include notifying civil authorities or 911.

1. In the event of an expected death, the Physician, and Funeral Director, should be notified.
2. All disposition arrangements should be made in advance, if possible, for expected deaths:
    a. ADNR/Advance Directive should be established
    b. The DNR/Advance Directive should be registered with the medical examiner.
3. The nurse will remain with the family until the funeral home staff has removed the body.
4. In the event of an unexpected death, the County Coroner, police, physician, and the Agency office are to be contacted. If it has been determined that the death will be a coroner's case, all tubes, IVs, NGs, etc. will remain in place, intact, for further investigation. Do not dispose of medications. The caregiver should remain present until the police arrive.
5. A patient/client's death must be pronounced by a physician, unless local laws allow paramedics, or RNs, to pronounce deaths in the home. If allowed by local law, the nurse will confirm the absences of signs to sustain life. The Medical Examiner will release the body to the funeral home, after which post-mortem care can be accomplished.
6. In the event the Agency is providing care to the patient/client, and the patient/client appears to expire, the RN should call #911, then start CPR and continue until the fire rescue arrives. When support services arrive and assume CPR, the RN should call the patient/client's attending physician and, then, notify the Agency.
7. All actions, interventions, and observations are documented in the clinical record.
8. Any controlled substances remaining are to be destroyed by the family, if possible, prior to the nurse leaving the home. No controlled substances are to be removed from the home. The nurse is to notify the police if the family does not destroy the medication.
9. If the patient has executed an Advance Directive, and has chosen no intervention, respiration and pulse cease in the home.
    a. If a professional staff is present, the staff member will measure the patient’s blood pressure, heart rate, and respiration.
    b. If no staff member is present, the caregiver is to call the emergency number.
    c. If the patient’s family member decides to rescind the Advance Directive at the time of crisis, the Advance Directive cannot be rescinded, if established by the patient.
    d. If the patient has not executed an Advance Directive, and pulse and respiration cease, if a professional staff member is present, he/she will initiate lifesaving, or sustaining, procedures and activate the emergency medical system.
    e. If the emergency medical system is activated:
        i. The EMS personnel, or physician, may decide to transport the patient.
        ii. The EMS personnel may terminate measures, in the home, under physician orders.
10. Educating the patient, or family, concerning the death process, and what to expect, will help lessen family stress.
11. Notify the Director of Nursing or designee, of patient’s death.

12. Notify the primary physician of patient's death.

BACK-UP CARE POLICY

Purpose

This policy outlines the procedures for providing back-up care to patients receiving services from **Golden Years Home Health Supported Living LLC**, a home health skilled agency licensed in Washington State, in accordance with Washington Administrative Code (WAC) 246-335-520(11). Back-up care ensures continuity of care when a scheduled caregiver is unavailable.

- **Back-up care for patient ADLs (Activities of Daily Living):** This includes assistance with tasks such as bathing, dressing, grooming, toileting, and ambulation.
- **Back-up care for patient health services:** This includes assistance with tasks requiring minimum healthcare credentialing, such as medication administration, wound care, or monitoring vital signs.

Non credentialed staff may provide back-up care only when assisting a patient with ADLs or in emergency situations

Staff Qualifications for Back-up Care

- **Back-up care for patient ADLs:** Non-credentialed staff, such as trained personal care assistants, may provide back-up care when assisting a patient with ADLs. These staff members will receive training specific to the needs of the patient and demonstrate competency in performing ADL tasks safely and effectively.
- **Back-up care for patient health services:** Back-up care requiring assistance with patient health services must be provided by staff with the minimum healthcare credentialing necessary for the specific task. This may include licensed nurses, therapists, or certified nursing assistants (CNAs) depending on the service required.

Scheduling Back-up Care

- The agency will make every effort to ensure scheduled caregivers are available to provide services.
- In the event a scheduled caregiver is unavailable due to illness, vacation, or other unforeseen circumstances, the agency will take the following steps:
    - Notify the patient or their designated representative as soon as possible.
    - Assess the patient's needs and determine the appropriate type of back-up care required.
    - Identify qualified back-up staff based on the patient's needs and credentialing requirements.
    - Communicate the back-up care plan to the patient, family, and back-up staff member.

Emergency Situations

- In emergency situations where a qualified back-up staff member is unavailable, the agency may temporarily assign non-credentialed staff to assist with basic patient needs, such as ensuring the patient's safety and comfort, until qualified medical personnel arrive.
- Non-credentialed staff will be instructed to avoid performing any tasks that require healthcare training and to document the situation accurately.

Communication and Documentation

- All back-up care occurrences will be documented in the patient's medical record, including the reason for the need for back-up care, the type of care provided, and the qualifications of the back-up staff member.

- The agency will communicate any back-up care incidents to the patient's physician, particularly if non-credentialed staff were involved in an emergency situation.

ADVANCED DIRECTIVES AND PATIENT ORDERS FOR LIFE-SUSTAINING TREATMENT (POLST)

WAC 246-335-535

Statement of Purpose

A home health agency at the time of admission must provide each patient, designated family member, or legal representative with a written bill of rights affirming each patient's right to:

(14) Be informed about **advanced directives** and **POLST** and the agency's scope of responsibility

Introduction

This policy establishes guidelines for the implementation of a patient's documented wishes regarding life-sustaining treatment as outlined in their Physician Orders for Life-Sustaining Treatment (POLST) form. This policy adheres to the Washington Administrative Code (WAC) Chapter 388-97, specifically WAC 388-97-07001 et seq.

Policy Statement

Our agency is committed to honoring a patient's right to self-determination regarding their medical care, including end-of-life decisions. We will follow the documented wishes outlined in the advanced directives and patient's signed POLST form to ensure their care aligns with their preferences. it is the Agency’s written policy to fully comply through its healthcare providers with the terms of a patient’s Living Will

Procedures

1. **POLST Form Completion and Review:**
   - Agency staff will collaborate with the patient, their healthcare providers, and legal representatives (if applicable) to facilitate the completion of a POLST form.
   - All sections of the POLST form should be completed to the best of the patient's ability.
   - Any section not completed implies full treatment for that section.
   - Once completed and signed by the physician, the POLST form will become part of the patient's medical record.
   - Staff will review the POLST form with the patient and their representatives to ensure understanding.
2. **POLST Form Implementation:**
   - The POLST form will guide all treatment decisions for the patient in the absence of a physician's order.
   - Staff will follow the documented treatment preferences outlined in the POLST form for cardiopulmonary resuscitation (CPR), medical interventions, and additional orders.
   - If a patient's condition changes or their POLST form is deemed outdated, a new form will be completed in collaboration with the physician and patient/representative.
3. **Medical Emergencies and POLST Forms:**
   - In the event of a medical emergency with agency staff present, the POLST form will be readily available to emergency medical personnel (EMP).
   - Staff will clearly communicate the patient's POLST status and treatment preferences to EMP upon arrival.

- A copy of the signed POLST form will be provided to EMP for reference.

4. **Education and Training**
   - Staff will receive ongoing education and training on POLST implementation to ensure proper understanding and application of this policy.

Responsibilities

- **Staff:**
  - Collaborate with patients and healthcare providers on POLST completion.
  - Review POLST forms with patients and representatives.
  - Implement treatment decisions outlined in the POLST form.
  - Communicate POLST information to emergency medical personnel.
- **Patients:**
  - Participate in POLST discussions and completion.
  - Communicate their healthcare preferences.
  - Designate legal representatives if desired.
- **Physician:**
  - Order and complete the POLST form in conjunction with the patient.

NURSE DELEGATION OF TASKS

RCW 18.79.260

Statement of Purpose

This policy outlines the guidelines for registered nurses (RNs) to safely delegate specific nursing tasks to other qualified healthcare personnel. It ensures patient safety by establishing competency requirements, delegation appropriateness criteria, and supervision protocols.

Statement

1. **RN Delegation Authority:** RNs can delegate tasks to qualified individuals if deemed best for the patient's care.
    (a) Delegation is **only permitted for stable and predictable patients** requiring specific nursing tasks **that do not require clinical judgment**
    (b) An approved **home health** service category **only may use their RN** on staff **for patient nurse delegation** needs
    (c) Approved **home health and home care service** categories **may provide nurse delegation** in the following ways:
        - Use an RN from their home health side to contract with and delegate to their home care side
        - Transfer a home care client needing delegation to the agency's home health side
2. **RN Responsibilities During Delegation:**
    - **Competency Assessment:** Ensure the individual possesses the skills to perform the delegated tasks.
    - **Delegation Appropriateness:** Evaluate if delegating the task aligns with the patient's condition and the individual's competency.
    - **Supervision:** Provide ongoing oversight and guidance to the individual performing the delegated task.
    - **Scope of Practice:** Delegate tasks solely within the RN's scope of practice.
3. **Restrictions on Delegation:**
    - Administration of medications (except for specific exceptions).
    - Tasks requiring substantial skill.
    - Piercing or severing tissues.
    - Tasks requiring nursing judgment.
4. **Protection from Employer Reprisal:**
    - RNs cannot be pressured to delegate if they believe it compromises patient safety.
    - They are immune from employer discipline for refusing delegation due to safety concerns.
5. **Delegation in Community-Based/In-Home Care:**
    - Delegation is only allowed for registered/certified nursing assistants or certified home care aides (if they have an NA-C or NA-R license)
    - Simple tasks (e.g., blood pressure monitoring) are exempt from this requirement.
    - Applicable only to patients with stable and predictable conditions.
    - RNs retain discretion to determine delegation appropriateness.
    - Insulin injections, sterile procedures, and central line maintenance are never delegable.

6. **Delegation of Insulin Injections:**
    - RNs can delegate insulin injections only after:
        - Instructing the individual on proper injection procedures and insulin use.
        - Demonstrating proper injection techniques.
        - Supervising and evaluating the individual's performance as per commission rules.
    - Supervision and evaluation frequency are determined by the commission.
7. **RN Accountability:**
    - The RN is accountable for their actions throughout the delegation process.
    - Nurses acting within delegation protocols are immune from liability for actions performed during delegation duties.
8. **Purpose of Delegation Protocols:**
    - To ensure consistent standards of nursing practice.
    - To safeguard the RN's authority for independent delegation decisions.
9. **Additional Delegation:** Delegation can include glucose monitoring and testing to qualified personnel.
10. **Rule-Making Authority:** The Nursing Care Quality Assurance Commission holds the authority to adopt rules for implementing this policy.
11. **Instruction by RNs:**

Only licensed RNs can instruct nurses on technical nursing subjects.

12. **Public Representation:**

Only licensed RNs can identify themselves as registered nurses to the public. An approved **home health** service category **only may use their RN** on staff **for patient nurse delegation** needs

13. **DSHS training:** (14)(d) **Home health aides** must complete **the DSHS nurse delegation class prior** to participating in the delegation process. If the **tasks are ones considered** by the nursing quality assurance commission to be **simple care tasks**, such as blood pressure monitoring, personal care services, diabetic insulin device set up, and verbal verification of insulin dosage for sight-impaired individuals, **the DSHS training is not required**.

WAC 246-335-525 PERSONNEL, CONTRACTOR, AND VOLUNTEER POLICIES

NON-DISCRIMINATION/AFFIRMATIVE ACTION POLICY

WAC 246-335-525; RCW 49.60.030

Statement of Purpose

The applicant or licensee must develop and operationalize personnel, contractor, and volunteer policies and procedures that describe:

(1) Employment criteria regarding discrimination consistent with chapter 49.60 RCW;

(1) The right to be free from discrimination because of race, creed, color, national origin, citizenship or immigration status, sex, honorably discharged veteran or military status, sexual orientation, or the presence of any sensory, mental, or physical disability or the use of a trained dog guide or service animal by a person with a disability is recognized as and declared to be a civil right.

Policy:

1. It is the policy of the Agency to admit and care for all patients without regard to race, creed, color, national origin, families with children, sex, marital status, sexual orientation, age, honorably discharged veteran or military status, or the presence of any sensory, mental, or physical disability or the use of a trained dog guide or service animal by a person with a disability. RCW 49.60.
2. The By-laws of the Agency comply fully with Section 601 of Title VI of the Civil Rights Act of 1964 and with Washington human relations legislation.
3. The Agency agrees to comply with the provision of the Civil Rights Act of 1964 and all requirements imposed pursuant thereto.
4. To that end, no person shall, on grounds of race, sex, age, color, national origin, sexual orientation, religious creed, ability to pay, AIDS or AIDS related conditions or DNR status or disability, be excluded from participation in, be denied benefits of, or otherwise be subjected to discrimination in the provision of any care or service.

Procedure:

1. Employees are assigned to patients based on the employee's qualifications to provide the prescribed care for the patient.
2. The employee's qualification to provide care needed by the patient is a relevant factor in staff assignment.

Decisions on employment of personnel are made on the basis of qualifications of the employee and the needs of the Agency without regard to (a) age, sex, marital status, sexual orientation, race, creed, color, national origin, honorably discharged veteran or military status, or the presence of any sensory, mental, or physical disability or the use of a trained dog guide or service animal by a person with a disability, unless based upon a bona fide occupational qualification: PROVIDED, that the prohibition against discrimination because of such disability shall not apply if the particular disability prevents the proper performance of the particular worker involved: PROVIDED. That this section shall not be construed to require an employer to establish employment goals or quotas based on sexual orientation.

(b) To discharge or bar any person from employment because of age, sex, marital status, sexual orientation, race, creed, color, national origin, honorably discharged veteran or military status, or the presence of any sensory, mental, or physical disability or the use of a trained dog guide or service animal by a person with a disability.

(c) To discriminate against any person in compensation or in other terms or conditions of employment because of age, sex, marital status, sexual orientation, race, creed, color, national origin, honorably discharged veteran or military status, or the presence of any sensory, mental, or physical disability or the use of a trained dog guide or service animal by a person with a disability: PROVIDED, that it shall not be an unfair practice for an employer to segregate washrooms or locker facilities on the basis of sex, or to base other terms and conditions of employment on the sex of employees where the commission by regulation or ruling in a particular instance has found the employment practice to be appropriate for the practical realization of equality of opportunity between the sexes.

(d) To print, or circulate, or cause to be printed or circulated any statement, advertisement, or publication, or to use any form of application for employment, or to make any inquiry in connection with prospective employment, which expresses any limitation, specification, or discrimination as to age, sex, marital status, sexual orientation, race, creed, color, national origin, honorably discharged veteran or military status, or the presence of any sensory, mental, or physical disability or the use of a trained dog guide or service animal by a person with a disability, or any intent to make any such limitation, specification, or discrimination, unless based upon a bona fide occupational qualification: PROVIDED, nothing contained herein shall prohibit advertising in a foreign language. RCW 49.60.180.

REFERENCES FOR PERSONNEL, CONTRACTORS, AND VOLUNTEERS

WAC 246-335-525(3)

Purpose

This policy outlines the procedures for obtaining and providing references for personnel, contractors, and volunteers at Golden Years Home Health Supported Living LLC. It ensures responsible reference practices while protecting the privacy of individuals.

POLICY

- The agency may request references from previous employers, professional associations, or other relevant sources during the application process for all personnel, contractors, and volunteers.
- Written consent will be obtained from the applicant/candidate before contacting any references.
- Reference requests will be limited to information relevant to the position or volunteer role.

Providing References

- The agency may provide references for current or former employees upon written request, with the employee's consent.
- References will be factual and objective, focusing on the individual's job performance, skills, and qualifications relevant to the position being referenced.
- The agency will not disclose confidential information about the employee, such as salary, disciplinary actions, or medical information.

Reference Confidentiality

- The agency will maintain the confidentiality of all reference information received during the application process.
- Reference information will be stored securely in personnel files and only accessed by authorized personnel with a legitimate need to know.

Volunteer References

- For volunteers, the agency may contact references provided by the applicant or request references from previous volunteer organizations, with the volunteer's consent.
- Reference checks for volunteers will be conducted following the same principles as outlined for personnel references.

Record Retention

- Reference information obtained during the application process will be retained for a reasonable period in accordance with record retention policies.

Disclaimer

- The agency cannot guarantee the accuracy or completeness of information provided in references.
- The final decision regarding hiring or volunteer selection will be based on a comprehensive review of all application materials and the candidate's qualifications.

CONTRACTOR POLICY

WAC 246-335-525

Statement of Purpose

The applicant or licensee must develop and operationalize personnel, contractor, and volunteer policies and procedures that describe:

(3) References for personnel, contractors and volunteers;

(4) Contracting process when using a contractor. The contract should include, at minimum, a description of the duties the contractor will perform, and a statement indicating that the contractor, not the employer, is responsible for withholding any necessary taxes. As with personnel and volunteers, contractors are subject to all applicable requirements in this chapter

Policy Statement

Our agency will utilize contractors to supplement our workforce when necessary. We are committed to establishing clear agreements with contractors to ensure a productive working relationship while adhering to all applicable laws and regulations.

Contracting Process

1. **Needs Assessment:**
    - Supervisors will identify the need for a contractor and define the specific services required.
    - The scope of work will be documented, outlining the tasks, deliverables, and expected timeline.
2. **Selection Process:**
    - A transparent selection process will be followed to identify qualified contractors. This may involve requesting proposals, conducting interviews, or referencing previous work.
    - Selection criteria will be based on skills, experience, qualifications, and cost-effectiveness.
3. **Contract Development:**
    - All agreements with contractors will be formalized in a written contract.
    - The contract will, at minimum, include:
        - **Description of Services:** A detailed description of the work the contractor will perform, including deadlines and deliverables.
        - **Compensation:** The agreed-upon payment terms and schedule for the contractor's services.
        - **Independent Contractor Status:** A clear statement confirming the contractor is an independent contractor, responsible for all relevant taxes and benefits associated with their work.
        - **Confidentiality:** A clause requiring the contractor to maintain confidentiality of all agency information.
        - **Insurance:** (Optional) If necessary, the contract may specify required insurance coverage for the contractor.
        - **Termination:** Procedures for contract termination by either party.
            - Contracts will be reviewed and approved by designated personnel within the agency before signing.
4. **Monitoring and Evaluation:**

- Supervisors will monitor the contractor's performance to ensure they meet the outlined expectations of the contract.
- Regular communication and feedback will be maintained throughout the contract period.

Compliance with WAC

- Contractors are subject to all applicable safety requirements as outlined in WAC 296-155, Chapter 155, Safety & Health Standards for Construction Work.
- The agency remains responsible for ensuring a safe work environment for all personnel, including contractors.

Responsibilities

- **Supervisors:**
  - Identify needs for contractors and define required services.
  - Participate in the selection process for qualified contractors.
  - Develop and manage contracts with contractors.
  - Monitor contractor performance and provide feedback.
- **Contractors:**
  - Provide services as outlined in the contract.
  - Maintain all necessary licenses and insurances (if applicable).
  - Comply with all agency safety regulations.
  - Be responsible for withholding and paying all required taxes associated with their earnings.
- **Finance Department:**
  - Review and approve contracts with regards to financial terms.

Recordkeeping

All contract documents and related communication will be maintained for (specify time frame) in accordance with agency record retention policies.

VERIFICATION OF HEALTHCARE PROFESSIONAL CREDENTIALS

WAC 246-335-525

Statement of Purpose

The applicant or licensee must develop and operationalize personnel, contractor, and volunteer policies and procedures that describe:

(5) Credentials of health care professionals that are current and in good standing

Policy Statement

Our agency is committed to providing quality care to our clients. To achieve this, we verify the credentials of all HCPs associated with our organization to ensure they are qualified and authorized to practice their profession within the state of Washington.

Procedures

1. **Verification Process:**
   - **Initial Employment/Contracting:**
     - All HCPs seeking employment or a contract with our agency must provide copies of the following:
       a. Current, valid license to practice their profession issued by the Washington State Department of Health (DOH).
       b. Proof of professional liability insurance.
       c. Verification of current CPR certification.
       d. Any additional state-mandated certifications or licenses relevant to their specific profession.
     - The agency will verify all submitted documents with the issuing entities using online verification tools or by contacting them directly.
   - **Periodic Verification:**
     - HCP credentials will be reviewed and verified periodically, at least [frequency, e.g., annually] to ensure continued validity and good standing.
     - HCPs will be responsible for notifying the agency of any changes in their licensure status or professional liability insurance.
2. **Recordkeeping:**
   - Copies of verified HCP credentials will be maintained in their personnel files.
   - Records will be accessible to authorized agency personnel and regulatory agencies upon request.
3. **Non-Compliance:**
   - In the event an HCP's credentials are found to be invalid, expired, or not in good standing, the agency will take appropriate action, which may include:
     - Restricting the HCP's practice until valid credentials are obtained.
     - Termination of employment or contract.
     - Reporting the non-compliance to the Washington State Department of Health as necessary.

Responsibilities

- **Human Resources Department:**

- Develop and maintain procedures for credential verification.
- Verify and maintain records of HCP credentials.
- Notify supervisors of any discrepancies in HCP credentials.

- **Supervisors:**
  - Ensure all HCPs under their supervision have verified credentials on file.
  - Monitor compliance with credentialing requirements.
- **HCPs:**
  - Provide accurate and up-to-date copies of their credentials for verification.
  - Notify the agency of any changes in their licensure status or professional liability insurance.

BACKGROUND CHECKS FOR PERSONNEL, CONTRACTORS, VOLUNTEERS, AND OTHER INDIVIDUALS

WAC 246-335-525

Statement of Purpose

The applicant or licensee must develop and operationalize personnel, contractor, and volunteer policies and procedures that describe:

(6) Criminal history background checks and disclosure statements for personnel, contractors, volunteers, students, and any other individual associated with the licensee having direct contact with children under sixteen years of age, people with developmental disabilities or vulnerable persons, according to RCW 43.43.830 through 43.43.842 and the following: (a) Criminal history background checks must be processed through the Washington State Patrol

Policy Statement

Our agency is committed to providing a safe and secure environment for all individuals we serve. To achieve this objective, we require criminal history background checks and disclosure statements for all personnel, contractors, volunteers, students, and any other individual associated with the licensee who will have direct contact with children under sixteen years of age, people with developmental disabilities, or vulnerable persons.

Applicability

This policy applies to the following individuals:

- Employees (paid and unpaid)
- Independent contractors
- Volunteers
- Students participating in internships or practicums
- Any other individual with direct contact with children under sixteen, people with developmental disabilities, or vulnerable persons

Background Checks

1. **Process:**
    - All individuals covered by this policy must undergo a criminal history background check processed through the Washington State Patrol (WSP) before being granted unsupervised access to children under sixteen, people with developmental disabilities, or vulnerable persons.
    - The background check will include a fingerprint-based search of national and state criminal history databases.
    - The agency will cover the cost of the background check.
2. **Disclosure Statements:**
    - In addition to the background check, all individuals covered by this policy are required to complete a disclosure statement.
    - The disclosure statement will ask about any past criminal offenses, even if they were not reported during the background check.

- Individuals must disclose any relevant convictions or charges, even if they are pending or have been dismissed.
- Requirement: You must require all individuals mentioned above to complete a disclosure statement approved by the Washington State Department of Health (DOH).
  - This statement typically asks if the applicant has been convicted of any crime against children or other vulnerable individuals.
- **Approval Process:** Obtain the most recent DOH-approved disclosure statement from the department website or contact them for guidance:

https://doh.wa.gov/

3. **Review and Evaluation:**
   - The agency will review the results of the background check and disclosure statement to determine suitability for positions involving direct contact with children under sixteen, people with developmental disabilities, or vulnerable persons.
   - Factors considered during the review may include:
     - The nature and severity of any offenses
     - The time elapsed since the offense
     - Rehabilitation efforts undertaken

Recordkeeping

- Copies of background check reports and disclosure statements will be maintained in confidential personnel files in accordance with applicable laws and regulations.
- Records will be accessible only to authorized agency personnel with a legitimate need to know.

Non-Compliance

- Failure to disclose a criminal offense or any information that could disqualify an individual from working with vulnerable populations may result in disciplinary action, up to and including termination of employment or volunteer status.

Renewal Requirement

All background checks and disclosure statements must be renewed every two years from the date of the previous check.

Responsibilities

- **Human Resources Department:**
  - Implement and maintain background check procedures.
  - Secure and review background check reports.
  - Maintain confidentiality of background check records.
  - Coordinate disclosure statement process.
  - Make final decisions regarding suitability based on background check results and disclosure statements.
- **Supervisors:**

- Inform relevant personnel of this policy.
- Ensure individuals under their supervision comply with background check and disclosure statement requirements.
- Report any concerns regarding potential disqualifying information to Human Resources.

- **Personnel, Contractors, Volunteers, Students, and Others:**
  - Cooperate with the background check process.
  - Complete the disclosure statement truthfully and accurately.

CHARACTER, COMPETENCE, AND SUITABILITY DETERMINATIONS FOLLOWING BACKGROUND CHECKS

WAC 246-335-525

Statement of Purpose

The applicant or licensee must develop and operationalize personnel, contractor, and volunteer policies and procedures that describe:

(7) Character, competence, and suitability determination conducted for personnel, contractors, volunteers, and students whose back-ground check results reveal non disqualifying convictions, pending charges, or negative actions.

Policy Statement

Our agency is committed to ensuring the safety and well-being of the individuals we serve. While background checks are a vital component of this process, a single finding may not automatically disqualify an individual. This policy establishes a framework for evaluating non-disqualifying results and making informed decisions regarding suitability for positions involving direct contact with clients.

Applicability

This policy outlines the process for conducting Character, Competence, and Suitability (CC&S) determinations for personnel, contractors, volunteers, and students in a Washington State home health agency. This applies to situations where a background check reveals non-disqualifying convictions, pending charges, or negative actions.

Purpose

- Ensure the safety and well-being of vulnerable clients receiving care from the home health agency.
- Assess an individual's suitability for working with vulnerable populations despite a non-disqualifying background check finding.
- Comply with Washington Administrative Code (WAC) 388-113-0050.

Process

1. **Non-Disqualifying Finding:** When a background check reveals a non-disqualifying conviction, pending charges, or negative action, initiate a CC&S determination process.
    - Upon receiving background check results, the Human Resources department will conduct a thorough review of any non-disqualifying convictions, pending charges, or negative actions.
    - The review will consider details such as:
        i. Nature and severity of the offense/action
        ii. Date of the offense/action
        iii. Any mitigating factors or evidence of rehabilitation
2. **Information Gathering:** Collect relevant information, including:
    - Nature of the offense/negative action
    - Date of the offense/action
    - Age of the individual at the time of the offense/action

- Completion of any court-ordered sentences or rehabilitation programs
- Documentation from the individual explaining the circumstances and any mitigating factors
- Reference checks (if applicable)

3. **Character, Competence, and Suitability Determination:**
   - A committee comprised of designated personnel (e.g., Human Resources, supervisor, legal counsel) will convene to review the background check findings and assess suitability based on the following factors:
     i. Whether there is a reasonable, good faith belief that the individual would be unable to meet the care needs of the patient.
     ii. The individual's skills, experience, and qualifications for the specific role.
     iii. Any demonstrated history of violence, abuse, neglect, or exploitation.
     iv. References and employment history.
     v. Evidence of rehabilitation or steps taken to address past issues.
   - The committee will determine if a reasonable, good faith belief exists that the individual poses a risk to clients due to their inability to meet their care needs.
4. **Factors for Consideration:** The following factors will be considered during the CC&S determination:
   - **(a) Meeting Patient Needs:** Consider whether there is a reasonable, good faith belief the individual would be unable to meet the specific care needs of the clients they will be assigned to.
   - **(b) Patient Vulnerability:** Evaluate the level of vulnerability of the patients the individual will be working with. Higher vulnerability requires stricter evaluation.
   - **(c) Recent Behavior:** Consider the individual's behavior since the convictions, charges, or negative actions.
   - **(d) Pattern of Behavior:** Evaluate any patterns of offenses or other behaviors that may put the patient at risk.
   - **(e) Time Elapsed:** Consider the number of years that have passed since the offense, negative action, or other issue.
   - **(f) Self-Disclosure:** Consider whether the individual self-disclosed the convictions, pending charges, or negative actions.
   - **(g) Health and Safety Concerns:** Evaluate any other health and safety concerns that may impact client care.
   - **(h) Department Authority:** Acknowledge that although the agency may determine suitability, the Washington State Department of Health (DOH) has the final authority regarding professional credentials.
5. **Decision-Making:** A designated committee or individual will review the information gathered and make a CC&S determination. This determination could be:
   - **Suitable:** The individual is deemed suitable for employment with appropriate supervision or limitations.
   - **Unsuitable:** The individual is not suitable for employment due to concerns about their character, competence, or suitability.
6. **Documentation:** Maintain detailed records of the CC&S determination process, including all collected information and the final decision.

7. **Appeals Process**
    - Individuals may appeal an adverse suitability determination by submitting a written appeal to the designated agency official within (specify time frame).
    - The appeal will be reviewed, and a final decision will be issued.

Additional Considerations

- **Confidentiality:** Maintain the confidentiality of all information collected during the CC&S determination process, complying with HIPAA privacy regulations.
- **Appeal Process:** Establish a clear process for individuals to appeal an unfavorable CC&S determination.
- **Legal Compliance:** Ensure this policy adheres to all applicable state anti-discrimination laws.

Responsibilities

- **Human Resources Department:**
    - Coordinate background check process.
    - Review background check results and identify non-disqualifying findings.
    - Facilitate character, competence, and suitability determination committee meetings.
    - Maintain records of all related documents and decisions.
- **Character, Competence, and Suitability Committee:**
    - Review background check results and other relevant information.
    - Make informed determinations regarding suitability based on established criteria.
    - Document reasoning behind decisions.
- **Supervisors:**
    - Inform relevant personnel of this policy.
    - Participate in the character, competence, and suitability determination process as needed.
- **Personnel, Contractors, Volunteers, and Students:**
    - Cooperate with the background check process.
    - Have the right to appeal an adverse suitability determination.

MANDATORY REPORTING

WAC 246-335-525

Statement of Purpose

The applicant or licensee must develop and operationalize personnel, contractor, and volunteer policies and procedures that describe:

8) (b) Mandatory reporters shall report suspected abandonment, abuse, financial exploitation, or neglect of a person in violation of RCW 74.34.020 or 26.44.030 to the department of social and health services and the proper law enforcement agency. Reports must be submitted immediately when the reporting person has reasonable cause to believe that abandonment, abuse, financial exploitation, or neglect of a vulnerable adult has occurred.

Policy Statement

Our agency is committed to providing quality care and adhering to all applicable regulations. This policy ensures that any identified instances of non-compliance with WAC 246-335 and RCW 70.127 are reported promptly to the DOH for investigation and corrective action.

Who is a Mandatory Reporter?

All agency personnel, including but not limited to:

- Employees (paid and unpaid)
- Supervisors
- Managers
- Volunteers
- Independent contractors providing services

What Must Be Reported?

Mandatory reporting applies to any known instances of non-compliance with WAC 246-335 and RCW 70.127, including:

- Violations of licensing requirements
- Failure to follow established policies and procedures for delivering safe and effective care
- Deficiencies in client care plans
- Improper medication administration
- Lack of qualified personnel
- Any other identified breach of regulations that could potentially jeopardize client safety and well-being

Reporting Process

1. **Identification of Non-Compliance:**
   - If a staff member becomes aware of a potential non-compliance issue, they are obligated to report it.
   - Supervisors and managers are responsible for creating an environment where staff feel comfortable reporting concerns without fear of retaliation.
2. **Reporting Procedures:**

- All reports of non-compliance must be submitted to the agency's designated compliance officer or Human Resources department within fourteen (14) calendar days of acquiring knowledge of the issue.
- Reports should be documented using the DOH-approved mandatory reporter form.
- The report should include details such as:
  - Nature of the suspected non-compliance
  - Date(s) and time(s) of the observed issue(s)
  - Names of individuals involved (if known)
  - Any supporting documentation or evidence

3. **Investigation and Reporting to DOH:**
   - The designated compliance officer or Human Resources department will investigate the reported non-compliance.
   - If the investigation confirms non-compliance, a formal report will be submitted to the DOH within fourteen (14) calendar days using the DOH-approved form.
   - The agency will cooperate with the DOH investigation and implement any corrective action plans as required.
4. **Confidentiality**
   - The agency will maintain the confidentiality of the reporter's identity to the best extent possible, while fulfilling its obligation to report non-compliance.
5. **Retaliation**
   - The agency prohibits retaliation against any employee who makes a good faith report of non-compliance.
6. **Responsibilities**
   - **All Staff:**
     - Report any suspected non-compliance with WAC 246-335 and RCW 70.127 within **fourteen (14) calendar days**.
     - Cooperate with investigations into reported non-compliance.
   - **Supervisors and Managers:**
     - Encourage open communication and a culture of safety reporting.
     - Investigate reported non-compliance concerns.
     - Report confirmed non-compliance to the designated agency official.
   - **Compliance Officer/Human Resources:**
     - Lead the investigation of reported non-compliance.
     - Submit formal reports of confirmed non-compliance to the DOH.
     - Implement corrective action plans as required by the DOH.

NEW HIRE ORIENTATION, SKILLS VERIFICATION, AND SUPERVISED PRACTICE

WAC 246-335-525

Statement of Purpose

The applicant or licensee must develop and operationalize personnel, contractor, and volunteer policies and procedures that describe:

(9) In-person orientation to current agency policies and procedures and verification of skills or training prior to independently providing patient care. Examples of skills verification include written testing, skills observation, and evidence of previous training and experience such as a certified nursing assistant training as detailed in WAC 246-841-400

Policy Statement

Our agency is committed to providing quality care to our patients. To achieve this objective, we implement a comprehensive onboarding process for all new personnel providing direct patient care. This process ensures they are familiar with agency procedures, possess the necessary skills, and demonstrate competency before working independently.

Applicability

This policy applies to all newly hired personnel who will have direct patient care responsibilities, including but not limited to:

- Certified Nursing Assistants (CNAs)
- Home Health Aides
- Personal Care Aides
- Other direct care staff positions

Onboarding Process

1. **Pre-Employment**

During the recruitment and selection process, the agency will verify that applicants possess the required licenses, certifications, and qualifications for the specific position.

2. **New Hire Orientation**

    - Upon hire, all new staff will participate in a comprehensive in-person orientation program.
    - Orientation will cover agency-specific policies and procedures relevant to their role, including but not limited to:
        - Patient care protocols
        - Infection control practices
        - Documentation and recordkeeping requirements
        - HIPAA regulations
        - Client rights and confidentiality
        - Emergency procedures and safety protocols
    - The agency will maintain attendance records and documentation of completed orientation modules.

3. **Skills Verification and Supervised Practice**
    - Prior to providing unsupervised patient care, all new staff must undergo a skills verification process to demonstrate competency in essential tasks.
    - Verification methods may include:
        - **Written Testing:** Assess knowledge of patient care procedures and protocols.
        - **Skills Observation:** Direct observation of staff performing essential skills under the supervision of a qualified trainer or preceptor.
        - **Evaluation of Previous Training and Experience:** Consider documentation such as certifications (e.g., CNA certification as outlined in WAC 246- 841-400), training certificates, or previous work experience evaluations to verify relevant skills and knowledge.
    - The extent of supervised practice will vary depending on the staff member's experience level and the complexity of the care needs.
    - A qualified supervisor will closely monitor performance, provide constructive feedback, and ensure competency before independent practice is authorized.
4. **Documentation**
    - All onboarding activities, including orientation completion, skills verification results, and supervised practice documentation, will be maintained in the employee's personnel file.

Responsibilities

- **Human Resources Department:**
    - Develop and maintain onboarding programs.
    - Coordinate and facilitate new hire orientation sessions.
- **Supervisors:**
    - Oversee the skills verification process for new staff.
    - Provide or assign qualified preceptors for supervised practice.
    - Evaluate staff competency and readiness for independent practice.
    - Document all onboarding activities for new staff under their supervision.
- **New Staff:**
    - Participate actively in all onboarding activities.
    - Demonstrate competency in essential skills through verification processes.
    - Seek clarification and ask questions as needed throughout the onboarding process.

NURSING ASSISTANT REGISTRATION

WAC 246-335-525

Statement of Purpose

The applicant or licensee must develop and operationalize personnel, contractor, and volunteer policies and procedures that describe:

(10) The process for personnel and contractors holding a nursing assistant registration to become credentialed as a nursing assistant certified. The policies and procedures must include, at minimum:

(a) Personnel and contractors holding a nursing assistant registration must become credentialed as a nursing assistant certified within twelve months of the date of hire. The date of hire is specific to each registered nursing assistant, not to the employer, and does not change if the registered nursing assistant changes employers

Policy Statement

Our agency is committed to providing quality care to our patients. We achieve this by ensuring our direct care staff possesses the necessary qualifications and competency. This policy requires all personnel and contractors holding an NA registration to transition to NAC certification within twelve (12) months of their hire date.

Applicability

This policy applies to all personnel and contractors hired for positions requiring direct patient care as a Nursing Assistant, including but not limited to:

- Certified Nursing Assistants (already certified)
- Registered Nursing Assistants

Process

1. **Verification of Registration:**
   - Upon hire, the Human Resources department will verify the validity of the applicant's NA registration.
   - A copy of the registration card or online verification will be maintained in the employee's personnel file.
2. **NAC Certification Requirement:**
   - All personnel and contractors with an NA registration are required to obtain NAC certification within twelve (12) months of their hire date.
   - The twelve-month timeframe applies to the individual's initial hire date and does not reset if the NA changes employers within the state of Washington.
3. **Support for Certification:**
   - The agency will provide resources and support to assist personnel and contractors in their pursuit of NAC certification, such as:
     - Information on testing centers and scheduling procedures
     - Reimbursement programs for certification exam fees (optional, as per agency policy)
     - Study materials or access to online learning resources (optional, as per agency policy)
4. **Verification of Certification:**

Upon successful completion of the NAC certification exam, personnel and contractors must provide documented proof of certification to the Human Resources department within thirty (30) days.

5. **Non-Compliance:**

Failure to achieve NAC certification within twelve (12) months of the hire date may result in limitations on patient care assignments or potential termination of employment. Exceptions may be considered on a case-by-case basis for documented extenuating circumstances.

Responsibilities

- **Human Resources Department:**
  - Verify NA registration upon hire.
  - Inform personnel and contractors of the NAC certification requirement and timeline.
  - Provide resources and support for NAC certification pursuits (optional).
  - Maintain documentation of NAC certification for all personnel and contractors.
  - Address non-compliance issues.
- **Personnel and Contractors:**
  - Maintain their valid NA registration.
  - Successfully complete the NAC certification exam within twelve (12) months of their hire date.
  - Submit proof of NAC certification to the Human Resources department.
  - Personnel and contractors hired by an in-home services licensee prior to March 1, 2018, who held a nursing assistant registration and have maintained their registration and employment with the licensee are not required to become credentialed as a nursing assistant certified.

SAFE FOOD HANDLING FOR PATIENT MEALS

WAC 246-335-525

Statement of Purpose

The applicant or licensee must develop and operationalize personnel, contractor, and volunteer policies and procedures that describe:

(13) Safe food storage, preparation and handling practices consistent with the United States Food and Drug Administration's recommendations for "food safety at home" for personnel, contractors, and volunteers involved in food preparation services on behalf of patients. Personnel, contractors, and volunteers may not provide patients with homemade food items or baked goods that they themselves prepared

Policy Statement

Our agency prioritizes the health and well-being of our patients. This policy dictates safe food handling practices to minimize the risk of foodborne illnesses and ensure patients receive meals prepared and stored in a safe and hygienic manner.

Applicability

This policy applies to all personnel, contractors, and volunteers involved in preparing, handling, or serving food for patients, including:

- Dietary staff
- Kitchen staff
- Volunteers assisting with meal preparation

Safe Food Handling Practices

All personnel, contractors, and volunteers involved in food preparation must adhere to the following safe food handling practices:

- **Personal Hygiene:**
  - Wash hands thoroughly with soap and warm water for at least 20 seconds before and after handling food, changing gloves, and using the restroom.
  - Avoid touching eyes, nose, or mouth while preparing food.
  - Wear clean outer garments (aprons, hairnets) to minimize contamination.
- **Food Preparation:**

- Maintain clean and sanitized work surfaces and utensils before and after food preparation.
- Thaw frozen food properly using a safe method (refrigerator, cold running water).
- Cook food to the proper internal temperature as outlined by the FDA's safe minimum cooking temperatures:

https://www.fsis.usda.gov/food-safety/safe-food-handling-and-preparation/foo d-safety-basics/safe-temperature-chart

- Cool cooked food rapidly and store it promptly in the refrigerator or freezer.
- Reheat leftover food to an internal temperature of 165°F (74°C) before serving.
- Label and date all prepared food for proper storage and inventory control.

- Follow proper food rotation practices (first-in, first-out) to ensure fresh ingredients are used first.
- Never prepare or serve food that has passed its expiration date.

- **Food Storage:**
  - Store raw meat, poultry, and seafood below cooked foods to prevent cross-contamination.
  - Keep all food items covered and separated in the refrigerator and freezer.
  - Maintain proper refrigerator and freezer temperatures (refrigerator: 40°F or below, freezer: 0°F or below).

Homemade Food Prohibition

For reasons of patient safety and potential allergen concerns, personnel, contractors, and volunteers are strictly prohibited from providing patients with homemade food items or baked goods that they themselves prepared.

Training and Resources

- All personnel, contractors, and volunteers involved in food preparation will receive training on safe food handling practices based on the FDA's "food safety at home" recommendations.
- Training materials and resources will be readily available to staff, including posters, handouts, and online modules.
- Ongoing education and reminders about safe food handling practices will be provided periodically.

Enforcement

Failure to comply with this policy may result in disciplinary action, up to and including termination of employment or volunteer service.

Responsibilities

- **Department Heads:**

  Ensure personnel under their supervision are aware of and comply with this policy.

- **Supervisory Staff:**
  - Implement and monitor safe food handling practices in their respective areas.
  - Provide training and support to staff on safe food handling procedures.
- **Personnel, Contractors, and Volunteers:**
  - Complete all required training on safe food handling practices.
  - Adhere to all safe food handling procedures outlined in this policy.
  - Ask questions if unsure about proper food handling practices.

FOOD SAFETY AT HOME — PATIENT & CAREGIVER GUIDE

Adapted from U.S. Food & Drug Administration (FDA) and USDA Food Safety and Inspection Service guidance

Each year millions of people get sick from foodborne illness, which can cause flu-like symptoms and, for some people, serious health problems. Older adults, pregnant women, young children and people with weakened immune systems (for example from diabetes, cancer treatment, kidney disease or HIV/AIDS) are at higher risk. Personnel, contractors and volunteers who prepare meals for patients follow the four steps below, and teach patients and families to do the same.

1 Clean

Always wash your food, hands, counters and cooking tools.

- Wash hands in warm soapy water for at least 20 seconds before and after touching food.
- Wash cutting boards, dishes, utensils and counter tops with hot soapy water after each use.
- Rinse fruits and vegetables under running water. Do **not** wash raw meat, poultry, fish or eggs — splashing spreads germs.
- Clean the lids of canned goods before opening.

2 Separate (keep apart)

Keep raw foods to themselves. Germs can spread from one food to another.

- Keep raw meat, poultry, seafood and eggs away from other foods in the cart, bags and refrigerator.
- Use a separate cutting board for raw meat, poultry and seafood.
- Never re-use marinades from raw foods unless brought to a boil first.
- Use a clean plate for cooked food — never the plate that held raw food.

3 Cook

Foods need to get hot and stay hot. Heat kills germs. Use a food thermometer — you cannot tell by looking.

Food	Safe minimum internal temperature
Beef, pork, veal, lamb — steaks, chops, roasts	145 °F, then rest 3 minutes
Fish and shellfish	145 °F
Ground beef, pork, lamb	160 °F
Egg dishes	160 °F
Poultry (whole, parts, ground) and stuffing	165 °F
Leftovers and casseroles	165 °F

4 Chill

Put food in the refrigerator right away.

- Keep the refrigerator at **40 °F or below** and the freezer at **0 °F or below**.
- **2-hour rule:** refrigerate or freeze food within 2 hours of cooking or buying — within **1 hour** if it is above 90 °F outside.
- Never thaw food on the counter. Thaw in the refrigerator, under cold running water, or in the microwave (cook right away).
- Marinate foods in the refrigerator.

Food-safe shopping and storage

Do	Don't
Keep produce separate from raw meat, poultry, seafood and eggs in shopping carts and bags.	Choose meat, poultry or seafood in torn or leaking packages.
Place meat, poultry and seafood in plastic bags at checkout.	Buy eggs with cracked shells or in damaged cartons.
Keep produce and ready-to-eat foods in a separate area of the refrigerator from raw meat, poultry, seafood and eggs.	Remove eggs from the carton to store them in the refrigerator door — keep them in the carton on a shelf.
Refrigerate perishable food within 2 hours (1 hour above 90 °F).	Leave perishable groceries in a hot car.

Food-safe meal preparation

Do	Don't
Wash hands with soap and water for at least 20 seconds before preparing food.	Wash raw meat, poultry, seafood or eggs.
Rinse whole fruits and vegetables under running water and dry with a clean cloth, paper towel or salad spinner.	Use soap or detergent on fruits and vegetables.
Wash utensils that touched raw meat, poultry, eggs or flour before using them with food that will be eaten raw.	Let raw meat, poultry, seafood, eggs or flour touch food that will be eaten raw.
Use a food thermometer and cook to the temperatures above.	Leave food out for more than 2 hours (1 hour above 90 °F).

Think you have a food illness?

- Call your doctor and get medical care right away; tell the agency.
- Save the food package, can or carton.
- Call the USDA Meat & Poultry Hotline at 1-888-674-6854 for meat, poultry or eggs, or FDA at 1-888-723-3366 for all other foods.
- Call your local health department if you think you got sick from food you ate at a restaurant or other food seller.

SEGURIDAD DE LOS ALIMENTOS EN CASA

Adaptado de la guía de la Administración de Alimentos y Medicamentos (FDA) de EE. UU.

Siga estos cuatro pasos al comprar, guardar, preparar y servir los alimentos para prevenir las enfermedades transmitidas por los alimentos.

Paso	Qué hacer
1. Limpiar	Lávese las manos con agua tibia y jabón por al menos 20 segundos antes y después de tocar alimentos. Lave las tablas de cortar, platos, utensilios y mostradores con agua caliente y jabón. Enjuague las frutas y verduras bajo el chorro del grifo. No lave la carne, las aves, el pescado ni los huevos crudos.
2. Separar	Mantenga la carne, las aves, los mariscos y los huevos crudos separados de otros alimentos en el carrito de compras, las bolsas y el refrigerador. Use una tabla de cortar distinta para los alimentos crudos.
3. Cocinar	Use un termómetro para alimentos: res, cerdo, ternera y cordero (cortes enteros) 145 °F y dejar reposar 3 minutos; pescado 145 °F; carne molida 160 °F; platos con huevo 160 °F; aves y relleno 165 °F; sobras y guisos 165 °F.
4. Enfriar	Mantenga el refrigerador a 40 °F o menos y el congelador a 0 °F o menos. Refrigere los alimentos dentro de 2 horas (1 hora si la temperatura es mayor de 90 °F). Nunca descongele los alimentos sobre el mostrador; hágalo en el refrigerador, en agua fría o en el microondas.

Sí	No
Mantenga los productos agrícolas separados de la carne, las aves, el pescado, los mariscos y los huevos en el carrito y en las bolsas.	Compre carne, aves o mariscos en paquetes rotos o que gotean.
Ponga la carne, las aves, el pescado y los mariscos en bolsas plásticas en la caja.	Compre huevos con la cáscara rota o en cartones dañados.
Lave los utensilios que tocaron alimentos crudos antes de usarlos con alimentos que se comen crudos.	Deje la comida afuera por más de 2 horas, o 1 hora a temperatura mayor de 90 °F.

CURRENT CARDIOPULMONARY RESUSCITATION (CPR) TRAINING

WAC 246-335-525

Statement of Purpose

The applicant or licensee must develop and operationalize personnel, contractor, and volunteer policies and procedures that describe:

(14) Current cardiopulmonary resuscitation (CPR) training consistent with agency policies and procedures for direct care personnel and contractors. Internet-based classroom training is permissible but demonstration of skills must be hands on and observed by a certified trainer

Policy Statement

Our agency prioritizes patient safety and well-being. We achieve this by ensuring all direct care personnel and contractors are trained and proficient in performing CPR according to current American Heart Association (AHA) guidelines and agency protocols.

Applicability

This policy applies to all direct care personnel and contractors who interact with patients and may be called upon to respond to a cardiac emergency, including:

- Nurses
- Nursing Assistants
- Therapists
- Emergency Medical Technicians (EMTs)
- Other personnel who provide direct patient care

Training Requirements

- **CPR Certification:** All direct care personnel and contractors must maintain a current CPR certification from the American Heart Association (AHA) or an equivalent organization approved by the Washington State Department of Health (DOH).
- **CPR Course Content:** CPR training must cover the following, as outlined by the AHA guidelines:
  - Adult CPR with a bag-mask device
  - Automated External Defibrillator (AED) use
  - Choking relief maneuvers for adults and infants
  - Two-rescuer CPR techniques (optional, depending on course type)
- **Hands-on Skills Demonstration:** While internet-based classroom training modules are permissible for knowledge acquisition, CPR certification requires a skills demonstration component. This demonstration must be performed in person and observed by an AHA-certified CPR instructor to ensure proficiency in CPR techniques.
- **Training Frequency:** CPR certification must be renewed at least every two years as per AHA guidelines.

Documentation and Monitoring

- All personnel and contractors must submit documentation of their current CPR certification to the Human Resources department upon hire and within 30 days of their certification renewal.

- The Human Resources department will maintain a record of all CPR certifications for personnel and contractors.
- Department heads or supervisors will be responsible for monitoring CPR certification expiration dates for personnel under their supervision.

Responsibilities

- **Human Resources Department:**
  - Maintain a record-keeping system for CPR certifications.
  - Provide reminders to personnel and contractors approaching CPR certification expiration dates.
- **Department Heads/Supervisors:**
  - Ensure all direct care personnel and contractors under their supervision maintain current CPR certification.
- **Personnel and Contractors:**
  - Obtain and maintain current CPR certification from an AHA-approved training program.
  - Submit documentation of CPR certification to the Human Resources department.
  - Attend CPR recertification training as required.

INFECTION CONTROL PRACTICES

WAC 246-335-525

Statement of Purpose

The applicant or licensee must develop and operationalize personnel, contractor, and volunteer policies and procedures that describe:

(15) Infection control practices, communicable disease testing, and vaccinations. Policies and procedures must include, at minimum: (a) Standard precautions such as hand hygiene, respiratory hygiene and cough etiquette, and personal protective equipment.

Policy Statement

This policy establishes the guidelines and procedures for infection control practices, communicable disease testing, and vaccinations within the agency. Adherence to these policies is essential to safeguard the health and well-being of clients, personnel, volunteers, and contractors.

Policy Details

1. **Standard Precautions**

Standard precautions are the foundation of infection control and must be applied to all patient care, regardless of suspected diagnosis. They aim to minimize the risk of transmission of bloodborne pathogens, other infectious agents, and multidrug- resistant organisms (MDROs).

Key elements of standard precautions include

- Hand Hygiene
  - Wash hands with soap and water for at least 20 seconds whenever visibly soiled or before and after any patient contact, including donning and doffing personal protective equipment (PPE).
  - Use alcohol-based hand sanitizer when soap and water are not readily available.
- Personal Protective Equipment (PPE)
  - Use appropriate PPE (gloves, gowns, eye protection, masks) based on the risk assessment of the procedure and potential for exposure to body fluids.
  - Don and doff PPE in the correct sequence to avoid contamination.
- Respiratory Hygiene and Cough Etiquette
  - Patients with respiratory infections should wear masks and practice proper cough etiquette (coughing into a tissue or elbow).
  - Staff should also practice proper cough etiquette and avoid close contact with patients exhibiting respiratory symptoms.
- Safe Handling of Sharps
  - Use sharps containers for disposal of needles, syringes, and other sharp instruments to prevent needlestick injuries.
  - Never recap needles.
- Safe Handling of Soiled Linen
  - Handle soiled linen with minimal manipulation to prevent dissemination of pathogens.

- Laundry workers should wear appropriate PPE when handling soiled linen.

- Environmental Cleaning and Disinfection
  - Routinely clean and disinfect environmental surfaces (e.g., countertops, medical equipment) with appropriate disinfectants based on the type of organism.
  - Pay particular attention to high-touch surfaces.

2. **Communicable Disease Testing**
   - Healthcare agencies should follow guidelines from the Washington State Department of Health (DOH) and Centers for Disease Control and Prevention (CDC) regarding recommended communicable disease testing for staff.
   - This may include testing for tuberculosis (TB), hepatitis B, and varicella (chickenpox) depending on job duties and potential patient exposure.
3. **Vaccinations**
   - The agency should strongly encourage staff to stay up-to-date on recommended vaccinations according to the CDC's Advisory Committee on Immunization Practices (ACIP) guidelines.
   - This may include vaccinations for influenza, measles, mumps, rubella, hepatitis B, varicella, and tetanus, diphtheria, and pertussis (Tdap).
   - Some agencies may mandate certain vaccinations for staff based on patient population and risk assessment.
4. **Staff Education and Training**
   - The agency must provide ongoing education and training for all staff on infection control practices, including standard precautions, transmission of infectious diseases, proper use of PPE, and hand hygiene techniques.
   - Training should be conducted at hire, annually, and whenever there are changes in procedures or outbreaks.
5. **Reporting and Investigation**
   2. Healthcare personnel with suspected or confirmed communicable diseases must report them to the agency and follow DOH guidelines for isolation or restriction from patient care activities.
   3. The agency must have a process for investigating potential outbreaks of infectious diseases and implementing appropriate control measures.
4. **Availability of Personal Protective Equipment (PPE):**
   a. Personal protective equipment necessary for implementing client plans of care shall be readily available to personnel and contractors.
   b. The agency shall ensure sufficient stock of PPE and other equipment required for infection control practices. This includes:
      - Hand sanitizer dispensers
      - Soap and disposable paper towels at handwashing sinks
      - Gloves in various sizes
      - Gowns
      - Masks
      - Eye protection

- Shoe covers
- Biohazard waste containers

5. **Tuberculosis (TB) Infection Control Program:**
   a. Conducting TB risk assessments for all new employees upon hire using the tuberculosis risk assessment form provided by the department.
   b. Based on risk assessment results, determine the agency's responsibility to conduct TB testing of new employees as per department recommendations.
   c. Conducting annual assessments of new TB risk factors for all employees using the department-provided tuberculosis risk assessment form.
   d. Ensuring workers receive TB-related training and education during new employee orientation, consistent with departmental educational materials.
6. **Response to Communicable Disease Symptoms:**
   a. Personnel, volunteers, contractors, and clients exhibiting or reporting symptoms of a communicable disease in an infectious stage must be managed in accordance with chapters 246-100 and 246-101 WAC.
   b. Appropriate actions shall be taken to prevent the spread of communicable diseases within the agency.
7. **Blood-Borne Pathogens Exposure:**
   a. The agency will conduct an initial risk assessment of the work environment to determine occupational exposure potential for bloodborne pathogens like Hepatitis B and HIV, in accordance with WAC 296-823 WAC.
   b. Develop policies and procedures based on the risk assessment results and provide necessary training and education to employees, volunteers, and contractors.
   c. Hepatitis B Vaccination:
      - If the risk assessment identifies a reasonably anticipated risk of occupational exposure for personnel, volunteers, or contractors, the agency will offer the Hepatitis B vaccine series free of charge.
      - Workers have the right to decline the Hepatitis B vaccine series, but must sign a declination form acknowledging the risks associated with such a decision.
8. **Annual Review and Update**
   a. Agencies must document an annual review of state health authority recommendations related to infection control practices, communicable disease testing, and vaccinations.
   b. Policies, procedures, and training shall be updated as necessary based on the review findings.
9. **Implementation**

This policy shall be communicated to all personnel, volunteers, and contractors upon adoption. Training sessions on infection control practices, communicable disease testing, and vaccinations shall be conducted to ensure understanding and compliance.

10. **Enforcement**

Failure to comply with this policy may result in disciplinary action as per agency protocols.

11. **Review Cycle**

- The agency will conduct an annual review of applicable state health authority recommendations related to infection control practices, communicable disease testing, and vaccinations.
- Based on the review findings, training and policies and procedures will be updated as necessary to maintain compliance and best practices.

Hand Hygiene Policy

Purpose

To establish uniform procedures for effective hand hygiene practices among all Golden Years Home Health Supported Living LLC personnel, contractors, and volunteers. Proper hand hygiene minimizes the risk of transmitting infections to clients, staff, and visitors during the delivery of in-home health services.

Policy

All employees, contractors, and volunteers of Golden Years Home Health Supported Living LLC must comply with current **Centers for Disease Control and Prevention (CDC)** and **Washington State Department of Health** hand hygiene guidelines. Hand hygiene must be performed before and after any direct client contact and at other key moments outlined in this policy.

Failure to adhere to this policy may result in disciplinary action and retraining requirements.

Procedure

A. Indications for Handwashing

Employees shall wash hands with soap and running water in the following circumstances:

1. When hands are visibly soiled or contaminated with blood, body fluids, or other material.
2. Before eating and after using the restroom.
3. Before and after direct client contact.
4. Before inserting urinary catheters, IV lines, or any invasive devices.
5. After contact with body fluids, mucous membranes, or non-intact skin.
6. After removing gloves.
7. When moving from a contaminated body site to a clean body site during client care.
8. After contact with environmental surfaces or equipment near the client.

B. Indications for Alcohol-Based Hand Rub

When hands are not visibly soiled, an alcohol-based hand rub (containing 60–95% ethanol or isopropanol) may be used in place of soap and water:

1. Before and after direct client contact.
2. Before handling medication or clean supplies.
3. After removing gloves.
4. After contact with intact skin, such as taking a pulse or blood pressure.
5. After touching equipment or other objects in the client's environment.

C. Handwashing Technique

1. Wet hands under clean, running water.
2. Apply antimicrobial or non-antimicrobial soap.
3. Rub hands vigorously for **at least 15 seconds**, covering all surfaces and between fingers.
4. Rinse thoroughly with hands angled downward.

5. Dry with a disposable towel.
6. Use the towel to turn off the faucet.

D. **Alcohol-Based Hand Rub Technique**

1. Apply sufficient product to palm of one hand.
2. Rub hands together, covering all surfaces and between fingers.
3. Continue rubbing until completely dry (approximately 20 seconds).

Glove Use and Hand Hygiene

1. Gloves must be worn when contact with blood, body fluids, mucous membranes, non-intact skin, or contaminated surfaces is anticipated.
2. Change gloves between procedures on the same client or when moving from contaminated to clean areas.
3. Remove gloves immediately after use and perform hand hygiene.
4. Never reuse disposable gloves.

Nail and Jewelry Policy

- Artificial nails or nail extensions are **not permitted** for any staff providing direct client care.
- Natural nails must be kept clean and no longer than **¼ inch** beyond the fingertip.
- Hand and wrist jewelry should be kept minimal to facilitate thorough cleaning.

Education and Compliance

- All personnel shall receive orientation and annual training on proper hand hygiene technique and infection control.
- Supervisors shall monitor compliance through direct observation and corrective coaching as part of the agency's Quality Improvement Program.
- Documentation of hand hygiene training and compliance audits shall be maintained in personnel records in accordance with **WAC 246-335-525(15)**.

References

- CDC Guidelines for Hand Hygiene in Health-Care Settings, 2002.

http://www.cdc.gov/mmwr/preview/mmwrhtml/rr5116a1.htm

- Washington Administrative Code **WAC 246-335-525(15)**: Infection Control Practices and Communicable Disease Testing.

HOME MEDICAL SUPPLIES AND EQUIPMENT

WAC 246-335-540

Statement of Purpose

Except as provided in subsections (5) and (6) of this section, the licensee must:

(3) (f) Home **medical equipment and supplies** used by the patient;

Responsibilities

- **Management:**
  - Develop and implement this policy and associated procedures.
  - Allocate resources for the proper maintenance and replacement of medical supplies and equipment.
- **Supervisors:**
  - Ensure staff receive training on this policy and proper use of medical supplies and equipment.
  - Monitor compliance with this policy and procedures.
- **Staff:**
  - Follow this policy and procedures for maintaining, cleaning, using, and storing medical supplies and equipment.
  - Report any malfunctioning, damaged, or expired equipment to their supervisor.

Medical Supplies and Equipment Management

1. **Inventory and Maintenance:**
   - A comprehensive inventory of all medical supplies and equipment will be maintained.
   - A schedule for routine cleaning, inspection, repair, and calibration of equipment will be established based on manufacturer's recommendations and industry standards.
   - Records will be maintained documenting the date, type of maintenance performed, and the name of the individual who performed the maintenance.
2. **Safe Handling and Storage:**
   - Medical supplies and equipment will be stored in a clean, dry, and secure location, following manufacturer's recommendations for temperature, humidity, and light exposure.
   - Expired or damaged supplies and equipment will be promptly removed from use and disposed of according to appropriate regulations.
3. **Patient Education and Consent:**
   - For patients requiring durable medical equipment (DME) or other medical supplies at home, the agency will:
     - Inform the patient, designated family member, or legal representative about the cost and payment options for the equipment, repairs, and replacements.
     - Obtain documented approval from the patient or authorized representative before acquiring the equipment.
     - Provide clear and comprehensive instructions on the proper use and maintenance of the equipment in a format understandable to the patient or family member. This may include written instructions, verbal instructions, or demonstrations.

        - Document the patient or family member's understanding of the instructions provided.
4. **Equipment Replacement:**
    - Supplies and equipment essential for patient health or safety will be replaced as needed. This includes:
        - Proactive replacement based on established schedules and expiration dates.
        - Reactive replacement of malfunctioning or damaged equipment.
        - Replacing equipment recalled by the manufacturer to ensure patient safety.

Documentation

- All activities related to medical supplies and equipment management will be documented, including:
    - Inventory records
    - Maintenance logs
    - Patient education and consent forms
    - Equipment replacement records

Training

- Staff will receive ongoing training on this policy, proper use of medical supplies and equipment, and safe handling procedures.

Contractor Selection and Oversight

- During the contractor selection process, the agency will evaluate the vendor's policies and procedures for managing medical supplies and equipment.
- The agency will only contract with vendors who demonstrate practices consistent with WAC 246-841-001(1), as outlined in this policy.
- **Contractual Requirements:**
    - The contract with the vendor will explicitly reference this policy and require the vendor to comply with all its provisions.
    - The contract will outline the agency's right to audit the vendor's practices related to medical supplies and equipment management.

Monitoring and Enforcement

- The agency will conduct periodic audits or reviews to ensure the contracted service provider adheres to the agreed-upon policies and procedures for medical supplies and equipment management.
- This may involve reviewing the vendor's documentation for maintenance logs, inventory records, and staff training related to medical supplies and equipment use.
- If any discrepancies or non-compliance issues are identified, the agency will take corrective action, which may include:
    - Issuing a corrective action plan for the vendor to address deficiencies.
    - Withholding payment until corrective actions are implemented.
    - Termination of the contract in cases of severe or persistent non-compliance.

Responsibilities

- **Contracting Department:**

- Include clear requirements for medical supplies and equipment management practices in vendor contracts.
- Evaluate vendor responses during the selection process regarding their compliance with WAC 246-841-001(1).

- **Quality Improvement Department:**
  - Develop and conduct audits or reviews to assess contractor compliance with this policy.
  - Report audit findings and recommend corrective actions as needed.
- **Management:**
  - Ensure the agency upholds its responsibility for patient safety by enforcing this policy with contracted service providers.

If the Agency contracts for medical supplies or equipment services

1. **Contractor Selection**
   - Prior to contracting with a medical supply and equipment service provider, a thorough evaluation process will be conducted. This will assess the contractor's:
     - Licenses and certifications in Washington State.
     - Experience and reputation in providing medical supplies and equipment.
     - Policies and procedures related to infection control, patient safety, and quality assurance.
     - References from other healthcare agencies.
   - The agency will only contract with vendors who demonstrate adherence to high standards for quality, safety, and infection control practices.
2. **Contractual Requirements**
   - All contracts with medical supply and equipment service providers will include clear language outlining the following:
     - The specific medical supplies and equipment to be provided.
     - Delivery schedules and procedures.
     - Maintenance and repair protocols for equipment.
     - Infection control practices for cleaning, disinfecting, and storing equipment.
     - Procedures for handling product recalls and defects.
     - A communication plan for addressing any concerns or issues regarding supplies or equipment.
     - Confidentiality requirements for protecting patient information.
3. **Contractor Monitoring**
   - The agency will establish a system for monitoring the performance of our contracted medical supply and equipment service providers. This may include:
     - Regular reviews of contractor policies and procedures.
     - Site visits to observe the contractor's operations.
     - Reviewing customer satisfaction surveys or feedback reports.
     - Monitoring for recalls or safety alerts related to medical supplies or equipment provided by the contractor.
4. **Corrective Action**

- If the agency identifies any concerns regarding a contractor's compliance with infection control standards, patient safety protocols, or contractual obligations, a corrective action plan will be implemented.
- This plan may involve requiring the contractor to revise their procedures, conduct additional training for their staff, or potentially terminate the contract if serious issues persist.

5. **Documentation**
    - The agency will maintain complete records of all contracts with medical supply and equipment service providers, including:
        - Evaluation documents used in selecting the contractor.
        - The signed contract outlining terms and expectations.
        - Documentation of any monitoring activities or communication with the contractor.
        - Records of any corrective action plans implemented.
6. **Compliance**
    - This policy is designed to ensure our agency adheres to subsection (1) of this section and all applicable Washington State regulations related to infection control, patient safety, and contracting for medical supplies and equipment services.

By implementing this policy, our home health agency can foster strong working relationships with reliable and compliant medical supply and equipment service providers. This ultimately benefits patient care by ensuring access to safe and effective medical supplies and equipment.

TUBERCULOSIS INFECTION CONTROL PROGRAM

Tuberculosis (TB) is a serious infectious disease that can affect the lungs. This program outlines the procedures for preventing the transmission of TB within Golden Years Home Health Supported Living LLC, a home health skilled agency, to protect our employees, patients, and their families, in accordance with Washington Administrative Code (WAC) 246-335-525(15)(c). **Risk Assessment**

- **New Employees:** All new employees will undergo a TB risk assessment upon hire, following the requirements of WAC 246-335-525(15)(c). This assessment will be conducted using the form provided by the Department of Health (DOH). It will consider factors such as:
  - Previous TB diagnosis or exposure
  - Immigration history from high-burden TB countries
  - Work experience in healthcare settings
  - Presence of risk factors for latent TB infection (LTBI)
- **Testing Based on Risk Assessment:** Based on the DOH-provided risk assessment results, the agency will determine the need for TB testing of new employees:
  - Employees identified as high-risk will undergo testing for LTBI using a Mantoux tuberculin skin test (TST) or interferon-gamma release assay (IGRA) blood test, following the DOH's tuberculosis risk assessment form testing recommendations.
  - Employees with a positive TB test will be referred to a healthcare provider for further evaluation and treatment.
  - Employees with negative test results will be documented and monitored according to their risk category.
- **Annual Risk Assessments:** All employees will undergo annual assessments for new TB risk factors using the DOH-provided form. This ensures timely identification of individuals who may require further evaluation or testing.

Training and Education

- In compliance with WAC 246-335-525(15)(c)(iii), all new employees will receive TB-related training and education **at the time of hire or during new employee orientation**. This training will be consistent with the DOH's tuberculosis program's online posted educational materials and will cover:
  - Understanding TB transmission and symptoms
  - Importance of TB risk assessments and testing
  - Proper use of personal protective equipment (PPE) when indicated
  - Infection control practices for preventing the spread of TB
  - Procedures for reporting suspected TB cases to the appropriate authorities

Actions When Infectious Disease Symptoms Are Present

- Adhering to WAC 246-100 and 246-101 WAC, the agency will have established procedures for responding to situations where personnel, volunteers, contractors, or clients exhibit or report symptoms of a communicable disease in an infectious stage. These procedures will include:
  - Prompt identification and assessment of symptoms by a qualified healthcare professional.

- Implementation of appropriate infection control measures to prevent the spread of the disease, following guidance from chapters 246-100 and 246-101 WAC.
- Exclusion of individuals with suspected or confirmed contagious diseases from direct patient care activities until they are no longer infectious, as determined by a healthcare professional.
- Reporting of confirmed cases of communicable diseases to the local health department, as required by law.
- Communication with affected individuals and their families regarding the situation and necessary precautions.

Additional Considerations

- The agency will maintain a record of all TB risk assessments, testing results, and employee education completion.
- N95 respirators or other appropriate PPE will be readily available for use in situations where exposure to a suspected TB case is possible.
- The agency will develop a communication plan to inform staff about suspected or confirmed TB cases while protecting patient confidentiality.
- The agency will collaborate with the DOH for guidance, resources, and reporting requirements related to TB infection control.

Program Review

This TB infection control program will be reviewed and updated annually to reflect any changes in DOH recommendations or agency practices.

ANNUAL PERFORMANCE EVALUATION

WAC 246-335-525

This policy outlines the annual performance evaluation process for all personnel and volunteers providing direct patient care, as well as contractors delivering such services, in the agency.

1. **Personnel and Volunteer Evaluations**

1.1. Purpose

- To assess the performance of staff and volunteers in providing quality patient care.
- To identify strengths and areas for improvement.
- To promote professional development and ensure staff and volunteers possess the necessary skills and knowledge to meet patient needs.

1.2. Frequency: Annual evaluations will be conducted for all personnel and volunteers directly involved in patient care.

1.3. Evaluation Components

- **Self-Assessment:** Staff and volunteers will complete a self-assessment form reflecting on their performance over the past year.
- **Supervisor Observation:** Supervisors will conduct on-site observations of staff and volunteers providing patient care. This will assess skills specific to the care needs of patients and adherence to agency policies and procedures.
- **Performance Review Meeting:** Supervisors will hold a meeting with each staff member or volunteer to discuss the self-assessment, observation findings, and overall performance.
- **Documentation:** Supervisors will document the evaluation process, including key findings, feedback provided, and any goals or development plans established.

1.4. Evaluation Criteria: Evaluations will consider factors such as:

- Clinical skills and knowledge relevant to assigned duties
- Quality of care provided to patients
- Communication skills with patients, families, and colleagues
- Teamwork and collaboration
- Professionalism and work ethic
    - Adherence to agency policies and procedures
    - Ability to meet job expectations
    - Initiative and problem-solving skills

2. **Contractor Evaluations**

2.1. Purpose

- To assess the quality of services provided by contractors delivering direct patient care.
- To ensure contractors meet the agency's standards for patient care and service delivery.
- To identify areas for improvement in the contractor's performance.

2.2. Frequency: Annual evaluations will be conducted for all contractors providing direct patient care.

2.3. Evaluation Methods

- **Review of Contractor Performance Data:** Review data collected throughout the year, such as patient satisfaction surveys, incident reports, and adherence to contractual agreements.
- **On-Site Visits:** Agency representatives may conduct on-site visits to observe contractor staff performance and service delivery.
- **Meetings with Contractor Representatives:** Regular meetings will be held to discuss performance, address concerns, and ensure alignment with agency standards.

2.4. Evaluation Criteria: Contractor evaluations will consider factors such as:

- Qualifications and competence of contractor staff
- Quality of care provided to patients
- Adherence to contractual agreements and service level expectations
- Communication and collaboration with agency staff
- Timeliness and responsiveness to patient needs
- Ability to meet established performance metrics

3. **Outcomes**
   - Based on the evaluation results, action plans may be developed to address identified areas for improvement for both staff/volunteers and contractors.
   - These plans may include providing additional training, assigning mentors, or revising job expectations.
   - For contractors, corrective action plans may be implemented or contractual agreements may be re-negotiated.
4. **Recordkeeping**

The agency will maintain a record of all performance evaluations for staff, volunteers, and contractors.

5. **Legal Compliance**

This policy will comply with all applicable state regulations regarding employee performance evaluations.

By implementing this comprehensive performance evaluation policy, your healthcare agency can ensure the continued quality of care provided to patients, promote professional development for staff and volunteers, and maintain strong working relationships with contractors.

WAC 246-335-530 PERSONNEL, CONTRACTOR, AND VOLUNTEER RECORDS

PERSONNEL POLICIES AND RECORDS

WAC 246-335-530

Statement of Purpose

The licensee must maintain records on all personnel and volunteers and have access to records on all contractors

Personnel Policies

1. There shall be established, upon approval from the Administrator, written policies concerning qualifications, responsibilities, and conditions of employment for each discipline.
2. These policies include a system for recruitment, orientation and continuing training and are reviewed and adopted annually by the Administrator.
3. Each employee shall acknowledge, in writing, receipt/review of said policies which shall include, but not be limited to: hours of work, employment qualifications including licensure, registration or certification when applicable, leave policies, evaluation of employee performance, job descriptions, Agency benefits, and other related personnel policies.

Personnel Records Management

1. Personnel records shall be maintained for all personnel and contractors.
2. They shall be kept in the Agency's office in a locked file or in a locked room and safeguarded from unauthorized use.
3. Employees may examine their own personnel file in the presence of the Administrator or designee.
4. Personnel records shall be retained for three (3) years after the employee has left the Agency.

Personnel Record Content

Personnel records shall contain at a minimum the following:

1. Current practice certification, credential or licensure, as applicable.
2. Documentation of references.
3. Contracts if using contractors.
4. Evidence of Orientation to current agency policies and procedures.
5. Verification of personnel and contractor’s skills or training specific to meeting the care needs of patients.
6. Documentation that personnel and contractors holding a nursing assistant registration became credentialed as a nursing assistant certified under chapter 246-841 WAC within twelve (12) months of the date of hire or met the exemption criteria in WAC 246-335-525 (10) (c).
7. Evidence of initial and subsequent criminal history background checks and disclosure statement(s) according to RCW 43.43.830 through 43.43.842.

8. Training on current and revised agency policies and procedures, including patient care issues.
9. Current CPR training for direct care personnel and contractors.
10. Documentation for personnel and contractors who prepare food for the patient independent of the patient's assistance, including:
    a. A current food worker card per chapter 246-215 WAC or
    b. Training equivalent to United States Food and Drug Administration for personnel and contractors involved in food preparation services on behalf of patients.
11. Communicable disease testing and vaccination according to current state health authority recommendations.
12. Documentation of performance evaluations of personnel providing direct patient care and evaluations of services provided by contractors providing direct patient care as required in WAC 246-335-525 (16) and (17).

Access

1. The Administrator and Director of Clinical Services shall have access to the personnel, volunteers and contractors files.
2. Management Staff may be provided personnel information on a need to know basis as deemed appropriate by the Administrator.
3. The privacy of the employee will be of paramount concern.
4. It shall be the Agency's policy that all employees be able to review their personnel file.
5. To do so, the employee must make an appointment with the administrator or designee. The review of the personnel file will be done under the supervision of the Administrator or designee.
6. Under no circumstance should the employee remove documentation from the file. Any concerns need to be brought to the attention of the Administrator or designee.
7. It shall be the Agency's policy to reply to written requests for information on former employees, providing the Agency has written authorization from the employee to do so.

Employee Separation

1. In the event that an employee resigns or is terminated, the Agency shall prepare an Employee Separation record and maintain it along with other records for the period stipulated above.
2. The Agency shall also attempt to obtain an Exit interview from the departing employee.
3. If the employee refuses to cooperate on this matter, it should be noted on the Employee Separation Record.

PATIENT BILL OF RIGHTS

WAC 246-335-535

Patient Rights and Responsibilities

Statement of Purpose

It is anticipated that observance of these rights and responsibilities will contribute to more effective care and greater satisfaction for the patient as well as the staff. The rights will be respected by all personnel and integrated into all Home Health Care programs. A copy of these rights will be given to patients and their families or designated representative. If the patient, designated family member or legal representative is unable to read the Bill of Rights and Responsibilities, it will be read to them. If the patient or his/her representative does not speak English, a copy of these rights will be provided in a language that is understood. The patient or his/her designated representative has the right to exercise these rights. In the case of a patient adjudged incompetent, the rights of the patient are exercised by the person appointed by law to act on the patient's behalf. In the case of a patient who has not been adjudged incompetent. Any legal representative may exercise the patient's rights to the extent permitted by law.

The Patient has the right

1. To receive effective treatment and quality services from Golden Years Home Health Supported Living LLC Home Health Care for services identified in the plan of care.
2. To be cared for by appropriately trained or credentialed personnel or contractors with coordination of services.
3. To a statement advising of the right to ongoing participation in the development of the plan of care.
4. To a statement advising of the right to access to the Department of Health listing of licensed home health agencies and to select any licensee to provide care, subject to the individual’s reimbursement mechanism or other relevant contractual obligations.
5. To a listing of the total services offered by Golden Years Home Health Supported Living LLC Home Health Care and those being provided to the patient.
6. To refuse specific treatments or services.
7. To the name of the individual within Golden Years Home Health Supported Living LLC Home Health Care responsible for supervising the patient’s care and the manner in which that individual can be contacted.
8. To be treated with courtesy, respect, and privacy.
9. To be free from verbal, mental, sexual, and physical abuse, neglect, exploitation, and discrimination.
10. To have property treated with respect.
11. To privacy and confidentiality of personal information and health care related records.
12. To be informed of what Golden Years Home Health Supported Living LLC Home Health Care charges for services, to what extent payment may be expected from health insurance, public programs, or other sources, and what charges the patient may be responsible for paying.
13. To a fully itemized billing statement upon request, including the date of each service and the charge. Note: if Golden Years Home Health Supported Living LLC Home Health Care is providing services through a managed care plan, we are not required to provide an itemized billing statement.

14. To be informed about advanced directives and POLST and Golden Years Home Health Supported Living LLC Home Health Care scope of responsibility.
15. To be informed of Golden Years Home Health Supported Living LLC Home Health Care policies and procedures regarding the circumstances that may cause the agency to discharge a patient.
16. To be informed of Golden Years Home Health Supported Living LLC Home Health Care policies and procedures for providing back-up care when services cannot be provided as scheduled.
17. A description of Golden Years Home Health Supported Living LLC Home Health Care process for patients and families to submit complaints to the home health agency about the services and care they are receiving and to have those complaints addressed without retaliation.
18. To be advised of the toll-free home health hotline for the State of Washington and the purpose of the hotline to receive complaints or ask questions about the agency or credentialed health care professionals. The State of Washington Home Health Hotline is 800-633-6828.

    You may also register complaints in writing to:

    Washington State Dept. of Health Facility Services Licensing PO Box 47852 Olympia, WA 98504-7852
19. To be informed of the DSHS end harm hotline number to report suspected abuse of children or vulnerable adults: 1-866-363-4276.

The Patient has the responsibility

1. To provide, to the best of his/her knowledge, accurate and complete information about:
    a. Past and present medical histories.
    b. Unexpected changes in his/her condition.
    c. Whether he/she understands a course of action selected.
2. To follow the treatment recommended by the particular handling of the case.
3. For his/her actions if he/she refuses treatment or does not follow the Authorizing Practitioner's orders.
4. For accruing that the financial obligations of his/her health care are fulfilled as promptly as possible.
5. To respect the rights of all staff providing service.
6. To notify the agency promptly in advance of an appointment or visit you must cancel.
7. To become independent in care to the extent possible, utilizing self, family and other sources.
8. To pay for care or services not covered by 3rd party payers.
9. To comply with the rules and regulations established by the agency and any changes subsequent to the rules.

Signature of Patient Date

Nurse/Therapist Signature Date

PATIENT NAME (Last, First)	MEDICAL RECORD #

Declaración de Propósitos

Se prevé que la observancia de estos derechos y responsabilidades contribuirá a la atención más eficaz y una mayor satisfacción para el paciente, así como el personal. Los derechos serán respetados por todo el personal y se integran en todos los programas de atención médica domiciliaria. Una copia de estos derechos se le dará

a los pacientes y sus familias o representante designado. Si el paciente o sus / su representante designado es incapaz de leer la Declaración de Derechos y Responsabilidades, se les leerá a ellos. Si el paciente o su representante / no habla Inglés, se proporcionará una copia de estos derechos en un idioma que se entienda. El paciente o sus / su representante designado tiene el derecho de ejercer estos derechos. En el caso de un paciente declarado incompetente, los derechos del paciente son ejercidas por la persona designada por la ley para actuar en nombre del paciente. En el caso de un paciente que no ha sido juzgado incompetente, cualquier representante legal podrá ejercer los derechos del paciente en la medida permitida por la ley.

El Paciente tiene derecho a

1. Ser plenamente informados por escrito de todos los derechos y responsabilidades antes de proporcionar atención pre-planificada y entender que estos derechos se pueden ejercer en cualquier momento.
2. Recibir la atención adecuada sin discriminación, de acuerdo con las órdenes del médico.
3. Elegir un proveedor de cuidado de la salud, incluyendo la elección de un médico de cabecera.
4. Solicitar servicios de la Agencia de Atención Domiciliaria de su elección y solicitar la información completa de su agencia antes de concederse la atención en relación con los servicios prestados, las alternativas disponibles, la licencia y los requisitos de acreditación, de propiedad y control organización.
5. Ser plenamente informados de antemano acerca de la atención / servicio a prestar, incluyendo las disciplinas que proporcionan cuidado y la frecuencia de las visitas, así como cualquier modificación del plan de atención.
6. Ser informado de que la familia o el tutor del paciente podrán ejercitar los derechos del paciente cuando el paciente ha sido juzgado incompetente.
7. Participar en la elaboración y revisión periódica del plan de atención / servicio.
8. El acceso a sus registros médicos y de la confidencialidad y privacidad de toda la información contenida en la historia clínica del paciente y de la información de salud protegida de acuerdo con HIPAA, y las leyes estatales y los requisitos de acreditación.
9. La información necesaria para rechazar el tratamiento dentro de los límites de la ley y ser plenamente informados de las consecuencias.
10. El tratamiento con la máxima dignidad y respeto por todos los representantes de las agencias, independientemente del estilo de vida elegido por el paciente, las costumbres culturales, estado civil, creencias políticas, religiosas, éticas, tener o no haber ejecutado una directiva anticipada y fuente de pago independientemente de la raza, credo, color, sexo, edad o discapacidad.
11. Que su propiedad y su persona sea tratada con el respeto, la consideración y el reconocimiento de la dignidad del paciente y la individualidad.
12. Recibir servicios de acceso de manera consistente, con continuidad, y en el momento oportuno de la agencia de su solicitud de servicio.
13. Recibir información sobre el cuidado de los servicios cubiertos bajo el beneficio de Home Health, en su caso.
14. Un plan individualizado de atención y enseñanza desarrollado por todo el equipo de salud, incluyendo el paciente y / o su familia.
15. Recibir información sobre el alcance de los servicios que la HHA proporcionará y de las limitaciones específicas sobre esos servicios.
16. Ser informado de los resultados anticipados de servicio / atención y de los obstáculos en el logro de resultados.

17. Ser informado de los derechos de los pacientes en cuanto a la recopilación y presentación de información de OASIS. La información no será compartida excepto para fines legítimos permitidos por la Ley de Privacidad.
18. Tener en cuenta en las políticas y procedimientos de la agencia en cuanto a la divulgación de las historias clínicas.
19. Ser informado en un plazo razonable de terminación anticipada del servicio o de los planes para la transferencia a otra instalación de cuidado de la salud / proveedor y el motivo de la terminación / transferencia de servicios.
20. Ser informado verbalmente y por escrito y antes de que se inicie la atención de las políticas de facturación de la organización y los procedimientos de pago y la medida en que:
    a. El pago que se puede esperar de cualquier programa financiado o ayudado conocido por la organización.
    b. El pago que se puede esperar de cualquier otro pagador de terceros.
    c. Cargos que el individuo puede tener que pagar.
21. Ser capaz de identificar a los miembros del personal que visitan a través de la identificación apropiada.
22. Ser informado oralmente y por escrito de cualquier cambio en la información de pago tan pronto como sea posible, pero no más tarde de 30 días a partir de la fecha en que la organización se da cuenta del cambio
23. Tener acceso a los servicios profesionales necesarios las 24 horas del día, 7 días a la semana.
24. Ser referido a otra agencia si él / ella no está satisfecho con la agencia o la agencia no puede satisfacer las necesidades del paciente.
25. Ser informado de las ventajas financieras cuando se refiere a una organización.
26. La educación, instrucción y lista de requisitos para la continuidad de la atención cuando se terminan los servicios de la agencia.
27. Estar libre de maltrato, negligencia o abuso verbal, mental, sexual y física, incluyendo lesiones de origen desconocido, y la apropiación indebida de la propiedad del paciente.
28. Saber que la agencia tiene un seguro de responsabilidad suficiente para las necesidades de la agencia.
29. Tener en cuenta que la agencia cumpla con la Subparte 1 de 42 CFR 489 y recibir una copia de las políticas de la organización y procedimientos escritos con respecto a las directivas anticipadas, incluyendo una descripción del derecho de un individuo bajo la ley estatal aplicable a formular directivas anticipadas sin temor a represalias
    - no una directiva anticipada es preparado y saber que la agencia seguirá peticiones del paciente en relación con la directiva anticipada en la prestación de cuidados.
30. Recibir información directivas anticipadas antes o en el momento de la primera visita a la casa, siempre y cuando se suministra la información de que se proporciona la atención y saber que el número (1-800-562-6078) Línea Directa del estado puede ser utilizado para presentar quejas con respecto a la aplicación del requisito de instrucción anticipada.
31. Expresar quejas sobre el tratamiento o cuidado que es (o se abstiene de ser), o por la falta de respeto de la propiedad o recomiendan cambios en la política, personal, o servicio / atención sin restricción, interferencia, coerción, discriminación o represalia y saber que las quejas se resolverán y el paciente notificado de la resolución dentro de los 30 días.

32. Tener en cuenta de la hotline salud en el hogar a la línea gratuita para el Estado y el propósito de la línea telefónica para recibir quejas o hacer preguntas sobre la organización. La línea gratuita del estado es (1-800-562-6078). El número es operado de 08 a.m.-5 p.m. Usted también puede enviar quejas via email al

HSQAComplaintIntake@doh.wa.gov y en línea en el sitio oficial del Departamento

de Salud para la búsqueda de credenciales de proveedores:

https://fortress.wa.gov/doh/providercredentialsearch/ComplaintIntakeForm.aspx

33. No ser negada la igualdad de oportunidades porque ellos o sus familias porque son de otro país, porque tienen un nombre o un acento relacionado con un grupo de origen nacional, porque participan en ciertas costumbres asociadas con un grupo de origen nacional, o porque están casados a, o asociado con, las personas de un determinado grupo nacional.
34. Ser informado del número de la línea gratuita para reportar abuso infantil en el estado. La línea gratuita es: 1-800-932-0313. Para reportar abuso de ancianos por favor ponerse en contacto con su Oficina de Servicios de Protección para Adultos de su Condado 1-877-734-6277 / (TTY) 1-833-866-5595.

El Paciente tiene la responsabilidad de

1. Suministrar según su conocimiento, información completa y precisa sobre:
   a. Su historia clínica pasada y presente.
   b. Cambios inesperados en su condición.
   c. Si él/ella entiende los procedimientos a seguir.
2. Seguir el tratamiento recomendado por el profesional que lo atiende.
3. Decidir si rechaza el tratamiento o no sigue las órdenes del médico.
4. Asegurar el pago de sus gastos médicos lo más pronto posible.
5. Respetar los derechos del personal que provee el servicio.
6. Notificar a las agencias con anticipación de la cancelación de una visita.
7. Independizarse en cuanto a su cuidado en la forma que le sea posible, bien por sí mismo a través de un familiar u otro recurso.
8. Pagar por los servicios que no estén cubiertos por una tercera parte.
9. Acatar las reglas y regulaciones establecidas por la agencia y/o cualquier modificación de las mismas.

Firma del Paciente Fecha

Firma del representante de la agencia Fecha

NOMBRE DEL PACIENTE (Apellido, Nombre)	EXPEDIENTE #

Firma del Paciente Fecha

Firma del representante de la agencia Fecha

WAC 246-335-540 HOME HEALTH PLAN OF CARE

PLAN OF CARE

WAC 246-335-540

Policy

This document outlines the process for developing and implementing a written home health plan of care (POC) for patients receiving services from our Washington State home health agency. It emphasizes collaboration with the patient, their designated family member/legal representative, and the authorizing practitioner.

Process

1. **Initial Assessment:**
    a. A registered nurse (RN) or other qualified healthcare professional will conduct a comprehensive assessment of the patient's needs upon referral.
    b. This assessment will involve:
        i. Reviewing the patient's medical history and current condition.
        ii. Assessing the patient's functional abilities and limitations in activities of daily living (ADLs) like bathing, dressing, eating, toileting, and mobility.
        iii. Identifying the patient's psychosocial needs and social support system.
        iv. Considering the patient's home environment and safety hazards.
2. **Gathering Input:**
    a. The RN will involve the patient, their designated family member/legal representative (if applicable), and the authorizing practitioner in developing the POC.
    b. This can be achieved through:
        i. Patient interviews
        ii. Family/representative meetings
        iii. Communication with the authorizing practitioner (physician) through phone calls, chart reviews, or progress notes.
3. **Developing the POC:**
    a. The RN will draft the POC based on the assessment findings and gathered input.
    b. The POC will address the following elements:
        i. Patient Information: Name, date of birth, contact information.
        ii. Diagnosis: Primary medical condition(s) prompting home health services.
        iii. Goals: Specific, measurable, achievable, relevant, and time-bound (SMART) goals for improvement in ADLs, wound healing, pain management, etc.
        iv. Interventions: The specific home health services to be provided (e.g., skilled nursing care, physical therapy, occupational therapy, medication management, personal care assistance).
        v. Frequency and Duration: How often and for how long each service will be provided.

vi. Teaching and Training: Specific skills or self-care activities the patient or caregiver will be trained on.

vii. Plan of Care Review and Revision Schedule: Frequency for reviewing and updating the POC based on patient progress.

viii. Disciplinary Standing Orders: Standing orders from the authorizing practitioner for routine medications and procedures.

ix. Discharge Planning: Strategies for transitioning the patient back to the community or to a higher level of care if needed.

x. Palliative care if applicable

4. **Communication and Collaboration:**
   a. The draft POC will be shared with the patient, family/representative, and authorizing practitioner for review and agreement.
   b. Any questions or concerns will be addressed collaboratively.
   c. Once finalized, all parties will receive a copy of the signed POC.
5. **Implementing the POC:**
   a. The home health agency staff will implement the POC according to the outlined interventions, frequency, and duration under the supervision of the administrator.
   b. Documentation will be maintained in the patient's medical record for all services provided.
   c. Communication channels will be established for the patient and family/representative to report any changes in condition or concerns.
6. **Review and Revision:**
   a. The POC will be reviewed at regular intervals (as defined in the plan) or more frequently if necessary based on patient progress.
   b. The review will involve the patient, family/representative, and authorizing practitioner.
   c. The POC will be revised as needed to reflect changes in the patient's condition, treatment plan, or goals.
7. Each patient will have a written home health plan of care developed and implemented with input from the patient, designated family member, or legal representative and authorizing practitioner.
8. "Authorizing practitioner" means the individual practitioners licensed in Washington state, or another state according to the exemption criteria established in chapters 18.57, 18.71, and 18.79 RCW, and authorized to approve a home health plan of care:
   a. A physician licensed under chapter 18.57 or 18.71 RCW;
   b. A podiatric physician and surgeon licensed under chapter 18.22 RCW; or
   c. An advanced registered nurse practitioner (ARNP), as authorized under chapter 18.79 RCW.
9. Each service to be provided will have a plan of care developed based on the specific assessment for that discipline.
10. The patient care plan shall be updated as often as the patient’s condition indicates but at least every two (2) months for patients receiving acute care services. The plan will be signed by the authorizing practitioner within sixty (60) days of the initial date of service or date of review and update.

11. The patient care plan for patients receiving maintenance services will be updated as often as the patient condition indicates but at least every six (6) months. The plan does not have to be signed by an authorizing practitioner unless required by the payer.
12. The plan shall be maintained as a permanent part of the patient's record.
13. The plan of care will include at a minimum the following items:
    a. Current diagnosis and information on health status
    b. Individualized patient goals and outcome measures
    c. Types and frequency of services to be provided
    d. Home medical equipment and supplies used by the patient
    e. Orders for treatments and their frequency to be provided and monitored by the agency
    f. Special nutritional needs and food allergies
    g. Orders for medications to be administered and monitored by the agency including name, dose, route and frequency
    h. Medication allergies
    i. Patient's physical, cognitive and functional limitations
    j. Discharge and referral plan
    k. Patient and family education needs pertinent to the care being provided by the agency
    l. Indication that the patient has a signed advanced directive or POLST, if applicable
    m. Resuscitation status according to advance directives, or POLST, if applicable
    n. The level of medication assistance to be provided

Steps to be taken in developing the care plan include

1. Collection of baseline data including all pertinent diagnosis, including mental status, types of services, identification of any services furnished by other providers and how those services are coordinated, equipment required, frequency and duration of visits, prognosis, rehabilitation potential, functional limitations, activities permitted, instructions for timely discharge or referral and any other appropriate items.
2. Identification of the patient's continuing need for home care and meet the patient's medical, nursing, rehabilitative, social and discharge planning needs including health needs/problems with which a patient/family needs assistance.
3. The Plan of Care reflects the participation of the patient to the extent possible. The Agency communicates the plan of care to the patient/caregiver in a comprehensive way that the patient/caregiver understands.
4. Determination of the nature of the home care service(s) needed. To determine this, the following must be considered:
    a. If applicable, what are the medical orders?
    b. What are the specific needs of the patient?
    c. What measures are required for comfort, safety, prevention of complications?
    d. Based upon the goals to be achieved, what actions must be taken to achieve them?
    e. Do family members have unmet health needs?
    f. Are other services indicated from either the Agency or other community resources?
    g. Can the Agency safely and adequately meet the identified needs?

5. Determination of the role to be played by various persons, e.g., RN, LPN, Aide, patient/family, is to be considered in implementing the care plan.
6. Determination of the goals to be achieved.
7. Staff responsible for each element of service.
8. Anticipated outcome of service approach with an estimated time frame for completion.
9. Establishment of a schedule for the provision of services which lists the description and frequency of each health care service, the required treatment and procedure, as well as the medication and diet regimen to be provided.
10. Include on the plan of care, the authorized practitioner's parameters for vital signs, and fasting blood sugar. If the authorized practitioner does not specify the parameters to be reported the following parameters will be used by the Agency:

Temperature ↑101

B/P ↑170/90 or ↓90/60

Pulse ↑120 or ↓60

Respirations ↓10 or ↑22

Fasting blood sugar ↓60 or ↑250

Oxygen sat <90%

11. Potential for discharge from service.

Development and Assessment

This policy outlines the process for developing and implementing written home health plans of care (POC) for patients receiving services from our home health agency. It ensures compliance with Washington Administrative Code (WAC) regulations, specifically WAC 246-335-540, and emphasizes the importance of a comprehensive patient assessment.

1. **Personnel and Training**
   a. Only appropriately trained or credentialed agency personnel will develop POCs. This includes:
      - Registered nurses (RNs)
      - Licensed practical nurses (LPNs) with appropriate training and supervision
   b. Staff developing POCs will receive ongoing training on:
      - Patient assessment techniques
      - POC development best practices
      - Regulatory requirements
2. **Patient Assessment**
   a. A comprehensive patient assessment will be conducted upon referral to identify individual needs and inform the development of the POC.
   b. Assessment elements may include:
      - Reviewing medical history and current medical condition

- Assessing functional abilities and limitations in activities of daily living (ADLs)
- Evaluating psychosocial needs and social support system
- Assessing the home environment for safety hazards

3. **Plan of Care Development**
   a. Based on the patient assessment, the RN or LPN will draft a POC that is:
      - **Patient-centered:** Addresses the patient's specific needs, goals, and preferences.
      - **Individualized:** Tailored to the unique circumstances of each patient.
      - **Goal-oriented:** Focused on achieving measurable outcomes.
   b. The POC will be developed in collaboration with the patient, their designated family member or legal representative (if applicable), and the authorizing practitioner.
   c. Input may be gathered through:
      - Interviews with the patient and family/representative
      - Reviewing medical records
      - Communication with the authorizing practitioner (physician)
4. **Plan of Care Content**
   a. The POC will include the following elements as outlined in WAC 246-335-540 (Section 3):
      - Current Diagnoses and Health Status: Primary medical condition(s) and relevant medical history.
      - Individualized Goals and Outcomes: Specific, Measurable, Achievable, Relevant, and Time-bound (SMART) goals for improvement.
      - Types and Frequency of Services: Home health services to be provided (e.g., skilled nursing, therapy, personal care) and their frequency.
   b. Additional elements may include:
      - Palliative Care Plan (if applicable): Details of interventions to improve comfort and quality of life.
      - Telehealth/Telemedicine Utilization (if applicable): How telehealth/telemedicine will be used in care delivery.
      - Home Medical Equipment and Supplies: List of necessary equipment and supplies.
      - Treatment Orders: Standing orders from the authorizing practitioner for routine medications and procedures.
      - Special Nutritional Needs: Any specific dietary requirements or food allergies.
      - Medication Orders: Authorized medications with name, dose, route, and frequency.
      - Medication Allergies: Documented medication allergies.
      - Functional Limitations: Patient's physical, cognitive, and functional limitations.
      - Discharge and Referral Plan: Strategies for transitioning the patient back to the community or a higher level of care.
      - Patient/Family Education Needs: Educational needs related to the patient's care.
      - Advanced Directives (if applicable): Indication of a signed advanced directive or POLST (Physician Orders for Life-Sustaining Treatment) and resuscitation status.
      - Level of Medication Assistance: Level of medication assistance to be provided by agency staff (e.g., reminders, administration).

5. **Communication and Approval**
    c. The draft POC will be shared with the patient, family/representative, and authorizing practitioner for review and approval.
    d. All concerns or questions will be addressed collaboratively.
    e. Once finalized, all parties will receive a signed copy of the POC.
6. **Conclusion**

By implementing this policy, our home health agency ensures that all POCs are developed by qualified personnel based on a comprehensive patient assessment, adhering to regulatory requirements and promoting patient-centered care.

Required Plan of Care Content

All home health POCs developed by our agency will include the following elements as mandated by WAC regulations

1. **Patient Information**
    a. Patient name and date of birth
    b. Current address and phone number
2. **Clinical Information**
    a. Current Diagnoses: Primary medical condition(s) prompting home health services.
    b. Health Status Summary: A concise overview of the patient's current health status and relevant medical history.
3. **Goals and Outcomes**
    a. Individualized Goals: Specific, Measurable, Achievable, Relevant, and Time-bound (SMART) goals for improvement in areas like ADLs, wound healing, pain management, etc., tailored to the patient's needs.
    b. Outcome Measures: Measurable indicators used to track progress towards achieving the established goals.
4. **Services and Frequency**
    a. Types of Services: Specific home health services to be provided (e.g., skilled nursing care, physical therapy, occupational therapy, personal care assistance).
    b. Frequency of Services: How often each service will be provided.
5. **Palliative Care:** Details of palliative care interventions focused on improving comfort and quality of life for patients with a serious illness (If Applicable).
6. **Telehealth/Telemedicine:** How telehealth or telemedicine will be utilized to deliver care (e.g., virtual consultations with healthcare professionals). (If Applicable)
7. **Home Medical Equipment and Supplies:** List all home medical equipment and supplies the patient will be using.
8. **Treatment Orders:** Orders from the authorizing practitioner for routine medications and procedures.
9. **Nutritional Needs**
    a. Special Diets: Any specific dietary requirements the patient has.
    b. Food Allergies: Documented food allergies.

10. **Medication Orders**
    a. Authorized Medications: A list of medications the patient is authorized to receive, including:
        - Medication name
        - Dosage
        - Route of administration (e.g., oral, injection)
        - Frequency
11. **Medication Allergies:** A list of any documented medication allergies the patient has.
12. **Functional Limitations**
    a. Physical Limitations: A description of the patient's physical limitations that may impact their ability to perform ADLs.
    b. Cognitive Limitations: A description of any cognitive limitations that may affect the patient's care.
    c. Functional Limitations: A description of the patient's functional limitations in ADLs (e.g., bathing, dressing, eating).
13. **Discharge and Referral:** Strategies for transitioning the patient back to the community or to a higher level of care if needed.
14. **Patient and Family Education:** Identification of any educational needs the patient and family/representative have related to the patient's care (e.g., medication management, wound care).
15. **Advanced Directives**
    a. If Applicable: Indication of whether the patient has a signed advanced directive or POLST (Physician Orders for Life-Sustaining Treatment).
    b. Resuscitation Status: Documentation of the patient's resuscitation status according to advance directives or POLST, if applicable.
16. **Medication Assistance:** The level of medication assistance to be provided by agency staff (e.g., reminders, administration).

Plan of Care Review Schedule

POCs will be reviewed and updated by appropriate agency personnel according to the following schedule:

a. Acute Care Services: Every two months.
b. Maintenance Services: Every six months.
c. Professional Medical Equipment Assessment or Home Health Aide Only Services: Every twelve months.

1. **Review Process:**
    a. Reviews will involve the patient, their designated family member or legal representative (if applicable), and the authorizing practitioner (physician).
    b. The review will consider:
        - Changes in the patient's condition.
        - Progress towards established goals.
        - Need for modifications to services or interventions.
        - Updated information from healthcare providers.

2. **Documentation and Signatures**
    a. Following the review, the POC will be updated to reflect any changes.
    b. The updated POC will be signed or authenticated and dated by:
        - Appropriate agency personnel (e.g., RN, LPN) who performed the review.
        - The authorizing practitioner.
3. **Return of Updated Plan of Care**
    a. The signed or authenticated updated POC will be returned to the agency within sixty days of:
        - The initial date of service (for new patients).
        - The date of review and update (for existing patients).
4. **Communication with Authorizing Practitioner**
    a. Agency staff will inform the authorizing practitioner regarding any significant changes in the patient's condition that may necessitate an update to the POC.
    b. Approval from the authorizing practitioner will be obtained for any additions or modifications made to the POC during the review process.
5. **Verbal Orders**
    a. Any verbal orders from the authorizing practitioner for modifications to the POC will be:
        - Immediately documented in writing.
        - Signed or authenticated and dated by the agency staff member authorized to receive the order within their scope of practice.
        - Forwarded to the authorizing practitioner for signature or authentication.
        - Returned to the agency within sixty days of receiving the verbal order.

Modified Plan of Care for Home Health Aide Services

This policy applies to situations where a patient receives services solely from home health aides. These services may include assistance with:

a. Activities of daily living (ADLs) like bathing, dressing, eating, toileting, and mobility.
b. Light housekeeping.
c. Meal preparation.
d. Companionship.

6. **Modified Plan of Care Content**

For patients receiving only home health aide services, the POC will be modified to include the following elements as outlined in WAC regulations:

a. Types and Frequency of Services
    - A clear description of the specific home health aide services to be provided (e.g., assistance with bathing, medication reminders).
    - The frequency of each service (e.g., daily, twice a week).
b. Home Medical Equipment and Supplies
    - A list of any home medical equipment or supplies the patient uses that the home health aide may need to assist with.

c. Special Nutritional Needs and Food Allergies
   - Any specific dietary requirements the patient has.
   - Documented food allergies.

d. Physical, Cognitive, and Functional Limitations
   - A description of the patient's physical limitations that may impact their ability to perform ADLs.
   - A description of any cognitive limitations that may affect the patient's care.
   - A description of the patient's functional limitations in ADLs.

e. Level of Medication Assistance
   - The level of medication assistance to be provided by the home health aide (e.g., reminders, supervision).

7. **Authorizing Practitioner Signature:** Unlike full POCs, modified POCs for home health aide services do not require an authorizing practitioner signature.
8. **Collaboration with Healthcare Team**
   - Although a formal signature is not required, collaboration with the patient's primary care physician or other healthcare providers involved in the patient's care remains important. This may involve:
     - Sharing the modified POC for review.
     - Obtaining recommendations for safe and effective home health aide services.

Documentation for One-Time Visits

Purpose

This policy outlines the required documentation for situations where our Washington State home health agency provides a one-time visit to a patient. It ensures compliance with Washington Administrative Code (WAC) regulations, specifically WAC 246-335-540(6).

Applicability

This policy applies to situations where a patient requires a single visit from a home health agency professional (e.g., RN, LPN) for a specific service, such as:

- Initial assessment for potential home health needs.
- Medication administration or teaching.
- Wound care education.

Documentation Requirements

In lieu of a full plan of care, the following written documentation will be maintained for one-time visits:

1. **Patient Information**
   - Patient's full name.
   - Patient's age.
   - Patient's current address.
   - Patient's phone number.
2. **Bill of Rights Confirmation**

- Documentation confirming that the patient received a written copy of their rights and responsibilities under WAC 246-335-535 (Home Health Patient's Bill of Rights).

3. **Patient Consent**
   - Written informed consent from the patient for the specific services to be provided during the one-time visit.
4. **Authorizing Practitioner Orders**
   - A copy of any written orders from the authorizing practitioner (physician) for the one-time visit (e.g., specific medications to be administered, wound care instructions).
5. **Documentation of Services Provided**
   - A detailed record of the services provided during the one-time visit, including:
     - Date and time of the visit.

- Home health professional who provided the service.
- Specific interventions or procedures performed.
- Patient assessment findings (if applicable).
- Patient education provided.
- Notes on the patient's condition and response to services.

Retention

All documentation for one-time visits will be maintained in the patient's medical record according to agency record retention policies.

Evaluation of the Care Plan

The patient’s health needs, plans and goals are considered by the coordinating professional on an ongoing basis.

The steps used are:

1. A regular review of the health needs/problems, plan and goals with the patient, family, authorized practitioner and others involved in the care of the patient.
2. A review of current patient status vs. services rendered.

If progress has been made:	If progress has not been made:
Should the care plan be modified?	What factors are contributing to lack of progress?
Should service be decreased or terminated?	Should service be increased or discontinued?

1. A determination of whether the previously determined goals should be changed:
   a. When applicable, regularly contact the attending authorized practitioner to discuss the patient's status, care plan and to obtain medical orders and/or recommendations.
   b. Evaluate the appropriateness, cleanliness, and safety of equipment prescribed.
2. Instituting a periodic evaluation is accomplished by:
   a. Inform the authorizing practitioner regarding changes in the patient’s condition that indicate a need to update the plan.

b. Obtaining approval from the authorizing practitioner for additions and modifications.
c. Discussion during a staff-supervisor conference.
d. Scheduling/attending case conferences.
e. Informal record review.
f. Caseload analysis.

Patient Involvement in Care Planning and Decision-Making.

1. In order to optimize patient/family participation in care planning and decisions affecting patient care, including planning for transfer, referral or discharge, and executing advance directives.
2. Agency patients (and families, as appropriate) will be assured the right to make decisions regarding their care and to be involved in the planning process.
   a. The Agency staff will be attentive to specific needs of the patient that might impact care planning.
   b. Whenever the plan of care is revised or updated (including changes in treatments or level of care), the Agency staff will confer with each other and with the patient and/or caregiver to ensure continuous attention to the involvement of the patient in the planning process.
   c. The clinical record will contain evidence of Agency staff communication with the patient/caregiver, and/or periodic case conference documentation will reflect patient/caregiver issues during planning (ex. initial assessment, daily visit note).
d. Patients /families will be adequately informed regarding procedures, including organization personnel.

Compliance with Washington Regulations

This process aligns with Washington Administrative Code (WAC) regulations, specifically WAC 246-335-540, which outlines the requirements for developing and implementing a home health plan of care.

Technology Considerations

Our agency may utilize electronic health record systems (EHRs) to facilitate POC creation, review, and sharing with all parties involved.

WAC 246-335-545 SUPERVISION OF HOME HEALTH SERVICES

DIRECTOR OF CLINICAL SERVICES - TRAINING PLAN

WAC 246-335-545

1. **Purpose**

This policy outlines the annual training requirements for the Director of Clinical Services (DOCS) and their designated alternate.) The director of clinical services must designate in **writing a similarly qualified alternate** to act in the director's absence. It ensures compliance with Washington Administrative Code (WAC) regulations requiring a minimum of ten hours of training annually.

2. **Training Objectives** This training program aims to:
    a. Enhance the knowledge and skills of the DOCS and alternate in their supervisory roles.
    b. Ensure ongoing competence in delivering and overseeing home health services.
    c. Maintain awareness of best practices in clinical supervision and disaster preparedness.
3. **Training Requirements**
    a. The DOCS and their designated alternate must complete a minimum of ten (10) hours of training annually.
    b. Training may cover a combination of topics related to clinical supervision duties and the delivery of home health services.
    c. Examples of appropriate training formats include:
        i. **Agency-Sponsored In-Services:** The agency may offer internal training sessions on relevant topics led by qualified professionals.
        ii. **Community Venues:** Workshops or presentations offered by local healthcare organizations or professional associations can be valuable learning opportunities.
        iii. **Community Classes:** Educational courses at colleges, universities, or training centers can provide in-depth knowledge on specific subjects.
        iv. **Conferences & Seminars:** Attending regional or national conferences focused on home health care and leadership can offer exposure to diverse perspectives.
        v. **Continuing Education (CE):** Training related to the DOCS's specific healthcare professional credential, if applicable, can contribute to supervisory knowledge.
        vi. **Supervisory Responsibilities in Emergencies:** Training on effective leadership and response protocols during natural disasters, man-made incidents, or public health emergencies is crucial.
4. **Training Approval and Documentation**
    a. The agency will maintain a list of pre-approved training programs that meet the requirements of this policy.
    b. Staff can also propose additional training opportunities for approval based on their relevance to supervisory duties.

   c. The DOCS and alternate are responsible for maintaining documentation of all completed training, including:
      i. Course titles and descriptions
      ii. Dates and locations of training
      iii. Names of instructors or presenters
      iv. Number of training hours completed

5. **Recordkeeping** The agency will maintain a central recordkeeping system for all training documents related to the DOCS and alternate. This documentation must be readily available for review upon request by the Washington State Department of Health (DOH).
6. **Monitoring and Evaluation**
   a. The agency will periodically review the effectiveness of this training program.
   b. This may involve soliciting feedback from the DOCS and the alternate on the value of training topics and formats.
   c. Based on the evaluation, the agency may revise the approved training list or adjust the training program as needed.
7. **Availability of Clinical Leadership During Patient Care Hours**
   a. **Importance:** This policy ensures the DOCS or a designated alternate is readily available during all hours when patient care is being provided. This allows for timely access to clinical expertise and support for staff and patients.
   b. **Definition of Availability:**
      i. "Readily available" means the DOCS or designee can be reached by phone or other reliable communication methods within a reasonable timeframe.
      ii. "Patient care hours" refers to the entire time frame the agency schedules and delivers services.
   c. **DOCS Responsibilities:**
      i. The DOCS must be available during patient care hours unless on approved leave or unavailable.
      ii. When unavailable, the DOCS must designate a qualified alternate with experience in home health and clinical supervision.
   d. **Alternate Designation:** Documentation will be maintained specifying the alternate's qualifications and contact information. This will be readily accessible to staff and provided to patients upon request.
   e. **Communication Plan:** A clear communication plan will ensure staff know:
      i. DOCS's typical availability schedule
      ii. Designated alternate's identity and contact information
      iii. Preferred contact method for urgent needs
      iv. This plan will be disseminated through staff orientation, meetings, and readily available resources.
   f. **After-Hours Emergencies:**
      i. A call-out protocol will be established for after-hours emergencies.
      ii. This will outline the process for reaching on-call staff for urgent patient care needs outside regular business hours.

g. **On-Call Staff Responsibilities:** On-call staff will be responsible for responding to urgent patient care needs outside regular business hours. This may involve providing guidance, authorizing care plan changes, or dispatching personnel.

h. **Documentation:**

i. Records will be maintained of:

- DOCS's availability schedule
- Designated alternate's information
- On-call staff rotation

ii. This documentation will be readily available for DOH review.

8. **Responsibilities of the DOCS or Designee**

a. **Patient Care Policies and Procedures**

i. The DOCS or designee is responsible for:

- Coordinating the development and revision of written patient care policies and procedures for each service provided by the agency.
- These policies and procedures will ensure consistent, evidence-based practices are followed in delivering quality care.

ii. Staff and volunteers will be trained on these policies and procedures to ensure understanding and compliance.

b. **Supervision of Patient Care**

i. The DOCS has the overall responsibility for supervising all patient care provided by agency personnel and volunteers.

ii. This supervision ensures the quality and safety of care delivered.

iii. The DOCS may delegate supervision tasks to a registered nurse (RN) or other appropriately credentialed professional with demonstrated supervisory experience.

iv. Delegation will be based on the complexity of patient care needs and the qualifications of the delegated staff member.

c. **Contractor Evaluation**

i. The DOCS or designee will oversee the evaluation of services provided by contractors who deliver care to our patients.

ii. This evaluation will assess the contractor's:

- Compliance with contractual agreements
- Qualifications and competence of staff
- Quality of care provided to patients

iii. Regular evaluations ensure contractors meet our standards for patient care and service delivery.

d. **Coordination of Multi-Agency Care**

i. When multiple licensed agencies are involved in a patient's care, the DOCS or designee will facilitate communication and coordinate services.

ii. This ensures a unified approach to patient care and avoids duplication of services.

e. **Plan of Care Compliance**

i. The DOCS or designee will ensure all patient care adheres to the established plan of care.

ii. The plan of care is a collaborative document outlining the patient's goals, interventions, and desired outcomes.

f. **Monitoring Patient Condition**
   i. The policy requires all direct care personnel, contractors, and volunteers to:
      - Observe and recognize changes in the patient's condition and needs.
      - Promptly report any changes to the DOCS or designee.
   ii. This allows for timely adjustments to the plan of care if necessary.

g. **Emergency Procedures**
   i. The policy mandates that all direct care personnel, contractors, and volunteers:
      - Be familiar with the agency's emergency procedures.
      - Be able to initiate these procedures according to established protocols in the event of an emergency.
   ii. Staff training will include emergency procedures to ensure preparedness for various situations.

h. **Communication and Documentation**
   i. The DOCS or designee will establish clear communication channels to facilitate information flow between staff, volunteers, contractors, and other healthcare providers involved in patient care.
   ii. Detailed documentation of all care provided, patient assessments, and changes in condition will be maintained in patient records.

9. **Legal Framework** This policy adheres to the following regulations:
   a. WAC regulations requiring documentation of supervision
   b. Chapter 18.79 RCW, which defines the scope of practice for registered nurses (RNs) and licensed practical nurses (LPNs)

10. **Types of Supervision Documented** This policy addresses the documentation of supervision for the following home health services:
    a. RN/LPN Services: Supervision of RN or LPN services by a qualified healthcare professional.
    b. Acute Care Services: Monthly supervisory visits for patients receiving acute care services.
    c. Maintenance Care or Home Health Aide Services Only: Biannual supervisory visits for patients receiving maintenance care or home health aide services only.

11. **RN/LPN Services Supervision**
    a. All RN and LPN services provided by our agency will be documented with evidence of supervision by a qualified healthcare professional in accordance with Chapter 18.79 RCW.
    b. This documentation will include:
       - The name of the RN or LPN providing the service.
       - The date and time of the service.
       - A summary of the service provided.
       - The name and title of the supervising healthcare professional.

12. **Acute Care Services Supervision**
    a. For patients receiving acute care services, supervision of home health aide services will occur at least once a month through on-site visits.

b. These visits may be conducted with or without the home health aide present.
c. The purpose of these visits is to:
    - Evaluate the home health aide's compliance with the plan of care.
    - Assess patient satisfaction with care.
d. Supervisory visits must be conducted by a licensed nurse or therapist in accordance with their respective practice acts.
e. Documentation of these visits will include:
    - The date of the visit.
    - Whether the home health aide was present.
    - The name and title of the supervising nurse or therapist.
    - Findings regarding compliance with the plan of care.
    - Patient satisfaction assessment.

13. **Maintenance Care or Home Health Aide Services Only**

a. For patients receiving maintenance care or home health aide services only, supervision of home health aide services will occur at least every six months through on-site visits.
b. Similar to acute care, these visits may be conducted with or without the home health aide present.
c. The goals of these visits are identical to those for acute care supervision.
d. Supervisory visits must be conducted by a licensed nurse or therapist following their practice acts.
e. Documentation of these visits will mirror the format used for acute care supervisory visits.

14. **Documentation Retention** All documentation related to supervision of home health services will be maintained in accordance with our agency's record retention policy. This ensures compliance with regulatory requirements and facilitates audits.
15. **Plan of Care Review**
    a. All home health aides are required to:
        - Review the plan of care for each patient before providing any services.
        - This review should occur whenever there is a change in the plan of care.
    b. The plan of care is a collaborative document outlining the patient's goals, interventions, and desired outcomes.
    c. By reviewing the plan of care, home health aides can ensure they are providing services that meet the patient's specific needs.
16. **Additional Instructions**
    a. In addition to the plan of care, home health aides must also review any additional written instructions for patient care.
    b. These instructions may be provided by the patient's physician, nurse, or other healthcare professionals involved in the patient's care.
    c. Reviewing these instructions ensures all aspects of the patient's care plan are followed accurately.
17. **Medication Assistance**
    a. Home health aides may assist patients with medications according to our agency policy and WAC regulations (Chapter 388-96 WAC).

b. Our agency policy on medication assistance will detail:
   - The specific tasks home health aides can perform related to medications (e.g., reminding patients to take medication, opening medication containers).
   - Training requirements for home health aides in medication administration.
   - Procedures for ensuring medication safety and accuracy.

c. Home health aides will not perform tasks that fall outside the scope of their training and our agency policy.

18. **Communication and Documentation**

a. Home health aides are responsible for communicating any concerns they have about a patient's condition or needs to the supervising nurse or therapist.

b. This communication can be verbal or documented in the patient's medical record.

c. Proper documentation ensures all healthcare professionals involved in the patient's care are aware of any changes or concerns.

19. **Training**

a. Our agency will provide ongoing training to home health aides on:
   - Plan of care review and interpretation.
   - Following written instructions for patient care.
   - Safe medication assistance techniques (if applicable based on agency policy).
   - Recognizing and reporting changes in patient condition.

b. This training will ensure home health aides have the knowledge and skills necessary to fulfill their responsibilities effectively.

WAC 246-335-550 PATIENT RECORDS

Golden Years Home Health Supported Living LLC, a home health skilled agency licensed in Washington State, is committed to maintaining accurate, complete, and confidential patient records as required by Washington Administrative Code (WAC) 246-335-550. This policy outlines the procedures for managing patient records to ensure accessibility, security, and compliance with regulations. Record Content: Patient records will include the following information, as required by WAC 246-335-550(3):

- Patient demographics (name, age, address, phone number) (3)(a)
- Consent for services (3)(b)
- Payment source and patient responsibility (3)(c)
- Initial assessment (3)(d)
- Plan of care (3)(e)
- Documentation of services provided (3)(f)
- Observations and changes in patient condition (3)(g)
- Physician orders and medication responses (3)(h) (for home health services, excluding home health aide only)
- Home health aide service supervision (3)(i) (for home health services, excluding home health aide only)
- Other required documentation as outlined in WAC 246-335

CLINICAL RECORD MAINTENANCE

WAC 246-335-515

Statement of Purpose

Policy

1. The Agency shall maintain a current clinical record for each patient which contains the Authorizing Practitioner's plan of care (if required), clinical and progress notes, and other pertinent information.
2. Entries to patient record documentation are made only by authorized staff and in accordance with organizational policy and procedure.
3. Patient records will be:
    a. Accessible in the agency's office for review by appropriate direct care personnel, contractors and the department
    b. Written legibly in permanent ink or retrievable by electronic means
    c. On standardized forms or electronic templates
    d. In a legally acceptable manner
    e. Kept confidential
    f. Kept in chronological order in its entirety
    g. Fastened together to avoid loss or record contents for paper documents
    h. Kept current with all documents filed according to timeframes
4. Records are safeguarded from loss or damage and are retained per State guidelines.
5. Except if the patient is a single visit, the record will include:
    a. Patient's name, age, current address and phone number
    b. Patient's consent for services, care and treatments
    c. Payment source and patient responsibility for payment
    d. Initial assessment when providing home health services
    e. Plan of Care according to WAC 246-335-540, depending on services provided
    f. Signed or electronically authenticated and dated notes documenting and describing services provided during each patient contact
    g. Observations and changes in the patient's condition or needs
    h. Authorized practitioner orders as applicable
    i. Documentation and response to medications and treatments ordered
    j. Supervision of home health aide services in accordance with WAC 246-335-545
    k. Any other documentation as appropriate
6. If the agency is making a single visit to the patient the following documentation will be in the patient record:
    a. Patient name, age, current address, and phone number
    b. Confirmation that the patient was provided with a written Bill of Rights
    c. Patient consent for services to be provided

   d. Authorizing practitioner orders
   e. Documentation of services provided
7. At least quarterly, a committee reviews a sample of active and closed cases to assure that established policies are followed in providing services.
8. Policies addressing access, storage removal and retention of patient records and information are consistent with HIPAA standards.
9. **The clinical record is the exclusive property of the Agency and all policies regarding confidentiality, release of information, and maintenance and retention of information shall apply.**
10. The patient has access to his or her own record.
11. Upon request, in accordance with agency policy, will provide patient information or a summary of care when the patient is transferred or discharged to another agency or facility.
12. The contents of the clinical record are discussed in detail in section of the Patient Care policies.

Record Review: Clinical Records are systematically reviewed on a quarterly basis to assess appropriateness and adequacy of care and services provided as well as to ensure compliance with policies regarding care delivery and professional standards of practice.

1. A random sample of clinical records are reviewed on the following basis:
   a. The selection includes 10% of annual unduplicated admissions with a maximum of 120 samples per year.
   b. Record selection includes representation of all disciplines providing care, diagnostic categories, lengths of stay and discharged records.
2. Record reviews for clinical services include a sample of 5% of the current caseload with a minimum of either five records or the total number of records, if less than five.
3. Reviews are based on specific criteria and include;
   a. Services rendered
   b. Following established agency policy
   c. Plan of Care
   d. Need for continued care
4. Reviews are conducted by the Clinical Record Review Committee which consists of representatives of all disciplines being provided to patients. Minutes of each meeting are completed. The results of the review are discussed as part of the quarterly meetings of the Quality Improvement Committee and reported to the Administrator with problem areas targeted for improvement.

Record Retention

1. It is the Agency Policy to retain patient records for:
   a. **Adults**: three (3) years following the date of termination of services
   b. **Minors**: three (3) years after attaining the age of eighteen (18), or five (5) years following discharge, whichever is longer
   c. **Patient death**: three (3) years following the last date or termination of services if the patient was on services with death occurred

2. Records will be stored in a secured, safeguarded storage facility to maintain the integrity of the record and to prevent unauthorized use. All medical records, including physician orders, nursing notes, assessments, discharge summaries, and medication administration records, must be retained for a minimum of six (6) years from the date of the last service provided to the patient.
3. All records removed to another location for storage will be transported by a licensed and bonded company and stored in a waterproof protected facility.
4. If the Agency ceases to operate or in the event of a disaster, records will be securely stored, and the Administrator will notify the Department in accordance with 70.02 RCW.
5. Copies of clinical records and/or information therein utilized by staff during the time of services will be destroyed in a manner so as not to violate confidentiality.
6. The Washington department will be informed in writing whenever a patient's clinical records are defaced or destroyed before termination of the required retention period.
7. Financial Records: Financial records related to patient care, such as billing statements and insurance claims, must be retained for a minimum of four (4) years from the date of the last service.
8. Other Records: Any other patient records not categorized as medical or financial records must be retained for a minimum of three (3) years from the date of the last service.

Confidentiality/Agency Use of Records/Authority to Make Entries

1. All clinical records and the contents thereof are confidential and will be protected from loss and unauthorized use.
2. Patient clinical records will be maintained in a secure location and current electronic patient records in an appropriate secure manner as to maintain the integrity of the patient data through routine backups on or off site. The Agency (with pertinent information maintained in the patient's residence as indicated) will be available to administrative, service delivery and clerical staff who require the use of records in the performance of Agency services or their job requirements. Such staff may use the records and make entries pertinent to the performance of their job.
3. Billing records will be maintained in the Agency and will be available to administrative, financial and clerical staff that require the use of records in the performance of Agency services or their job requirements. Such staff may use the records and make entries pertinent to the performance of their job. All entries must be legible, clear, complete, and appropriately authenticated and dated. Authentication must include signatures or a secured computer entry.
4. Records will be made available to properly authorized state representatives for the purpose of Agency audits, certification and licensure reviews.
5. In order to promote uniformity concerning confidentiality, security and integrity of the Agency's home health care information for all home health care staff, only authorized home care staff will have access to the home health care record as follows:

    a. Access to patient information files (medical records and billing) will be limited to Agency staff involved in the care/service of the patient and may include:

        i. Administrative staff.
        ii. Clinical Supervisors.
        iii. Clinical staff (direct and through contract) including RNs/LPNs, HHAs, PTs, OTs, STs, MSWs.

iv. Office staff having a need to know to perform their home care functions and processes including Intake RNs, Coordinators, Schedulers and Billing staff.

v. When the Agency Administrator and Director of Clinical Services are unavailable the Office Manager has the authority to admit surveyors and other authorized individuals and to allow access to records.

b. Clinical staff should only have access to information on those patients to which they are actively providing care and service, such as primary nurses, on-call, relief, etc.

c. Office staff should only have access to information on those patients they are actively providing service to, such as working on bills, following up on complaints, etc.

d. Access to records for individuals from outside the organization, such as surveyors, reviewers, consultants, etc. will be approved by the Administrator and/or Director of Clinical Services on a per occurrence basis providing they have appropriate identification.

e. Any Agency committee may review the clinical record at meetings as requested by the Administrator or Director of Clinical Services.

f. Any release of information contained in the clinical record other than that required by law must be authorized in writing by the patient.

g. Review of the clinical record for the purposes of performance improvement activities and clinical record review will be conducted according to Agency policy.

6. When the patient is transferred to another health care facility, a transfer form, or discharge summary or pertinent information is sent to the attending Authorizing Practitioner and facility.
7. Records and information pertaining to persons with HIV/AIDS shall be handled in accordance with state laws and the Agency policies.
8. Non-confidential information or "non-privileged" information can be released under appropriate circumstances without requiring the patient's written authorization. The reason for the "need to know" should always be considered. Certain identification data obtained on admission is considered "non-privileged" which means that this data may be given without violating the patient's right to privacy or the patient/Authorizing Practitioner privilege.
9. This data includes:
    a. Name of patient
    b. Verification of hospitalization
    c. Address of residence given on admission
    d. Name of attending Authorizing Practitioner
    e. Sex, age, and occupation of the patient
    f. Names of relatives/friends given on admission
    g. Date of admission and discharge

One-time visit

1. Patient Information
2. Reason for Visit
3. A brief summary of the patient's relevant medical history and current condition as it pertains to the one-time visit

4. For one-time visits, the plan of care may be brief but should outline any follow-up instructions or recommendations for the patient
5. All documentation must be completed by a qualified healthcare professional who participated in the one-time visit (e.g., registered nurse, therapist). The documentation should include the healthcare professional's name, title, and signature.
6. Even though this policy addresses one-time visits, it's important to maintain a communication link with the patient's primary care physician or other healthcare providers involved in the patient's care. This may involve sending a brief report summarizing the one-time visit and any recommendations.

Record Transport and Off-Site Storage

1. The Agency may be required to transport closed records to an off-site location due to lack of space, damage to the office, etc. It is imperative that all records be protected from damage and unauthorized use.
2. Off-site storage is provided at a secure facility following HIPAA rules of confidentiality and security.
3. All boxes containing medical records should be numbered and appropriately labeled and kept in a locked room or secure area.
4. The Administrator should supervise all aspects of the move to ensure that the movers are aware of exactly what needs to be transported and proper secure handling of sensitive records at all times during transit.
5. To ensure adequate security and to protect records against weather, light, pollution and other dangers, vehicles must be covered, locked, attended at all times, and not used for transporting other materials, such as chemicals, that may cause risk to the records.

ACCESS TO CLINICAL RECORDS

Statement of Purpose

The licensee must:

(1) Maintain a current record for each patient consistent with chapter 70.02 RCW

Patient Access to Records

1. Patients wishing to review the contents of their clinical record must submit a written request to the Administrator.
2. The request must identify the specific portions of the record to be reviewed. Under no circumstances will the record be removed from the Agency. The Administrator or designee shall be present at all times during the review.
3. The Agency may charge the patient for copying and/or searching the records involved, but access to the patient's medical records will not be denied due to the patient's inability to pay. Charges for coping will be in accordance with 70.02.010 RCW.
4. If, after reviewing his/her record, the patient/legal guardian feels there is a mistake, notation of such should be dated and documented on a separate note and attached to the patient's record.
5. No alteration of the original record by crossing out and initialing should be made.
6. If the patient wishes to amend the records, documentation by the patient will be accepted and included in the record.

Release of Information

1. Upon admission, the patient or his legal representative shall sign an authorization for the Agency to request and to release medical information.
2. The authorization may indicate certain information which cannot be released and may also specify persons or entities to whom information may not be released, if the patient so desires.
3. The patient or legal guardian may consent to release of his/her records. The Agency must respond to such request however, the following guidelines will apply:
    a. The consent must be in writing on an Agency approved release of information form.
    b. The patient's consent must be voluntary and informed.
    c. The written consent must be signed and dated by the patient or legal representative and must be current.
4. Any patient consent for release of medical records to another must set forth "with specificity" the Agency directed to make the disclosure, the name of the patient, the extent or nature of the information to be disclosed, statement of the purpose for which the information is being released, a specific expiration date for the consent and a statement that the consent is subject to revocation at any time (or that the consent expires at a given time and when). A copy of the consent must be kept with the patient's records.
5. When a decision to release a record has been made, the patient's record will be reviewed by the Administrator prior to its release.
6. The person authorized to receive the information may be charged a reasonable fee for its provision.

7. Persons allowed access to the patient's medical record shall not make secondary or additional disclosures to others.

Authorizing Practitioner Access to the Record

1. Care is provided under the direction of the Authorizing Practitioner; therefore, it is the Agency's policy to allow the attending Authorizing Practitioner access to the clinical record.
2. If the patient refuses to release necessary information to his/her Authorizing Practitioner, the Agency will not accept the patient for care.
3. When the Authorizing Practitioner other than the attending Authorizing Practitioner requests access to the record, a release of information will be obtained from the patient.

Attorney Access to the Record

1. Requests to release information to attorneys must be made in writing to the Administrator.
2. A signed, dated release of information must be obtained from the patient. A charge may be made for any copies of the record given to the attorney.
3. The Administrator may contact legal counsel for a legal opinion and advice regarding these requests.

Third Party Payer Access to the Record

The Agency will exercise caution in the release of information to patient's insurers. The following guidelines will apply:

1. Completed and signed patient consents for the release of records will be presented by the requesting party.
2. Any disclosure of patient records will be noted on the patient's permanent record along with a notation of what information or records were disclosed and the date of the disclosure.
3. A copy of the written request for the information release will then be attached to the patient's permanent record.

Other Parties Access to the Record

1. Hospitals/clinics for further patient care - A signed authorization will be required prior to transfer of information.
2. Worker's Compensation - Information may be released without the patient's authorization.
3. Government Agencies - Release of information to government agencies shall generally require a signed authorization from the patient.
4. Department of Health - No authorization from the patient will be required.
5. FBI and all law enforcement officials - A patient's authorization is required before release.
6. Information authorized by a Health Professional's Licensing Act - No patient authorization is required.
7. Transporting EMS providers, for the purpose of determining the patient's diagnosis and outcome of hospital admission - No authorization from the patient will be required.
8. A member of the clergy carrying out their duties - No authorization from the patient will be required.
9. A qualified organ or tissue procurement organization - No authorization from the patient will be required.

10. Prospective health care providers for the purpose of securing the services of that health care provider as part of the patient's continuum of care as determined by the patient's Authorizing Practitioner - No authorization in addition to the service agreement from the patient will be required.
11. American Red Cross, a regional poison control center, or a utilization review agent - No authorization from the patient will be required.

Record of Information Released

The Agency shall keep an account of all medical records released. Such accounting shall be kept in the individual clinical record and noted elsewhere as indicated.

Deceased Patients

When a patient expires, information can be furnished:

1. To the executor of the will upon presentation of letters testamentary.
2. To the spouse or to the closest surviving relative if the patient was unmarried.
3. To a legally appointed guardian.

Subpoena of Records

Subpoenas for records will be immediately brought to the attention of the Administrator. The Agency may release information without a prior authorization if the Administrator or supervising nurse is served with a valid subpoena. Photostatic copies will be submitted unless otherwise ordered. Only upon receipt of a valid subpoena shall medical records be taken from the Agency. The Administrator shall verify the legality of the subpoena issued. If an original record is submitted to the court, an Agency representative must accompany the record to court.

The following are the requirements for a valid subpoena:

1. The subpoena must be served upon the person to whom it is addressed.
2. The subpoena must be issued by a court within the state in which the Agency is operating. A subpoena from courts outside the state should be reviewed by the Agency's legal counsel.
3. Date, time and place of appearance should be stated on the subpoena.
4. It should have a docket number, although this is not always available when the subpoena is prepared.
5. A witness fee is usually sent with the subpoena. Legal counsel should determine if the subpoena can be ignored if the witness fee is not included and there is no arrangement to pay this fee.

If any question exists about the validity of the subpoena, the Administrator should consult legal counsel. In cases of litigation involving the Agency, the Administrator should notify counsel and other appropriate staff. Courts and administrative agencies have authority to require that designated personnel of the Agency:

1. Be present at trials, hearing and depositions.
2. Give testimony concerning records.
3. Bring records and allow review and reproduction of records.
4. Under no circumstances will a medical record be left in the custody of an attorney or court.

WAC 246-335-555 QUALITY IMPROVEMENT PROGRAM

This policy outlines the implementation of a quality improvement program (QIP) throughout all our approved home health service areas. This program ensures we continuously monitor, evaluate, and improve the quality of care and services we provide, adhering to WAC 246-335-555.

1. **QIP Structure**

Our QIP will consist of the following core elements:

- **Quality Improvement Committee:** A designated committee comprised of qualified healthcare professionals, staff representatives, and, when appropriate, consumer representatives, will oversee the QIP.
- **Performance Measures:** We will identify key performance indicators (KPIs) aligned with national and state quality standards for home health care. These KPIs may include:
  - Client satisfaction with care
  - Readmission rates to hospitals
  - Rates of hospital-acquired infections
  - Medication adherence rates
  - Pain management effectiveness
  - Client functional outcomes
- **Data Collection and Analysis:** We will establish a system for collecting data on our identified KPIs. Data collection methods may involve client surveys, chart reviews, incident reports, and staff feedback. The QIP committee will regularly analyze this data to identify trends and areas for improvement.
- **Quality Improvement Activities:** Based on data analysis, the QIP committee will recommend and implement quality improvement activities. These activities may involve:
  - Updating policies and procedures.
  - Implementing new training programs for staff.
  - Piloting new care models or interventions.
  - Revising plans of care to address identified needs.
- **Communication and Reporting:** The QIP committee will communicate program findings and improvement activities to all staff and leadership. Regular reports on the QIP will be submitted to the appropriate regulatory agencies.

2. **Application Across Service Areas**

The QIP will be applied consistently across all our approved service areas:

- **Service Area Integration:** KPIs and data collection methods will be adapted to address the specific needs and services provided in each area.
- **Communication and Collaboration:** Staff from all service areas will be encouraged to participate in the QIP process by sharing best practices and feedback.
- **Benchmarking:** We will benchmark our performance against national and state quality standards, as well as compare data across service areas to identify areas for improvement.

3. **Continuous Improvement**

The QIP is an ongoing process committed to continuous improvement in the quality of care provided by our agency. The program will be reviewed and updated regularly to reflect changes in regulations, best practices, and client needs.

Our Quality Improvement Program, incorporates three key components:

1. Complaint Process
2. Problem Identification and Correction
3. Patient Satisfaction Assessment

1. **Complaint Process**
   - **Receipt:** Complaints regarding our services can be submitted verbally or in writing through various channels, according to the agency policies.
   - **Documentation:** All complaints will be documented promptly, including the date, nature of the complaint, contact information of the complainant (if provided), and any relevant details.
   - **Investigation:** The administrator and director of clinical services will investigate the complaint thoroughly and objectively. This may involve interviewing the complainant, staff involved, and reviewing relevant records.
   - **Disposition:** The complainant will be kept informed of the investigation process and receive a timely response outlining the outcome. This response may include:
     - Explanation of the investigation findings
     - Information on any corrective actions taken
     - Resolution options, if applicable
2. **Problem Identification and Correction**
   - **Identifying Problems:** We will actively seek out opportunities to identify problems with our services. This includes:
     - Reviewing complaint data
     - Analyzing internal incident reports
     - Conducting regular staff meetings and performance evaluations
     - Encouraging feedback from patients, families, personnel, contractors, and volunteers
   - **Monitoring and Evaluation: The role of the supervisor and administrator** will be to make sure the identified problems will be monitored and evaluated to determine their root cause and potential impact.
   - **Corrective Actions:** Based on the evaluation, appropriate corrective actions will be implemented to address the problem and prevent recurrence. This may involve:
     - Updating policies and procedures
     - Providing additional staff training
     - Enhancing communication channels
     - Revising equipment or supplies
3. **Patient Satisfaction Assessment**
   - **Method:** The administrator will conduct regular assessments to measure patient satisfaction with our overall services. This may involve:
     - Patient satisfaction surveys

    - Phone interviews
    - Focus groups
    - Review of online reviews (if applicable)
- **Frequency:** Patient satisfaction assessments will be conducted at least annually.
- **Data Analysis:** Survey data and other feedback will be analyzed to identify areas of strength and weakness.
- **Action Plan:** Based on the analysis, an action plan will be developed to address identified areas for improvement. This may involve implementing new initiatives or expanding existing programs.

Program Management

- A Quality Improvement Committee will be established to oversee the program's effectiveness and ensure continuous improvement.
- The program procedures will be reviewed and updated annually or as needed.
- All staff will be trained on the Quality Improvement Program procedures.

WAC 246-335-560 HOME MEDICAL SUPPLIES AND EQUIPMENT

HOME MEDICAL SUPPLIES AND EQUIPMENT

WAC 246-335-540

Statement of Purpose

Except as provided in subsections (5) and (6) of this section, the licensee must:

(3) (f) Home **medical equipment and supplies** used by the patient;

Responsibilities

- **Management:**
  - Develop and implement this policy and associated procedures.
  - Allocate resources for the proper maintenance and replacement of medical supplies and equipment.
- **Supervisors:**
  - Ensure staff receive training on this policy and proper use of medical supplies and equipment.
  - Monitor compliance with this policy and procedures.
- **Staff:**
  - Follow this policy and procedures for maintaining, cleaning, using, and storing medical supplies and equipment.
  - Report any malfunctioning, damaged, or expired equipment to their supervisor.

Medical Supplies and Equipment Management

5. **Inventory and Maintenance:**
   - A comprehensive inventory of all medical supplies and equipment will be maintained.
   - A schedule for routine cleaning, inspection, repair, and calibration of equipment will be established based on manufacturer's recommendations and industry standards.
   - Records will be maintained documenting the date, type of maintenance performed, and the name of the individual who performed the maintenance.
6. **Safe Handling and Storage:**
   - Medical supplies and equipment will be stored in a clean, dry, and secure location, following manufacturer's recommendations for temperature, humidity, and light exposure.
   - Expired or damaged supplies and equipment will be promptly removed from use and disposed of according to appropriate regulations.
7. **Patient Education and Consent:**
   - For patients requiring durable medical equipment (DME) or other medical supplies at home, the agency will:
     - Inform the patient, designated family member, or legal representative about the cost and payment options for the equipment, repairs, and replacements.

- Obtain documented approval from the patient or authorized representative before acquiring the equipment.
- Provide clear and comprehensive instructions on the proper use and maintenance of the equipment in a format understandable to the patient or family member. This may include written instructions, verbal instructions, or demonstrations.
- Document the patient or family member's understanding of the instructions provided.

8. **Equipment Replacement:**
    - Supplies and equipment essential for patient health or safety will be replaced as needed. This includes:
        - Proactive replacement based on established schedules and expiration dates.
        - Reactive replacement of malfunctioning or damaged equipment.
        - Replacing equipment recalled by the manufacturer to ensure patient safety.

Documentation

- All activities related to medical supplies and equipment management will be documented, including:
    - Inventory records
    - Maintenance logs
    - Patient education and consent forms
    - Equipment replacement records
    - the patient, designated family member, or legal representative's approval

Training

- Staff will receive ongoing training on this policy, proper use of medical supplies and equipment, and safe handling procedures.

Contractor Selection and Oversight

- During the contractor selection process, the agency will evaluate the vendor's policies and procedures for managing medical supplies and equipment.
- The agency will only contract with vendors who demonstrate practices consistent with WAC 246-841-001(1), as outlined in this policy.
- **Contractual Requirements:**
    - The contract with the vendor will explicitly reference this policy and require the vendor to comply with all its provisions.
    - The contract will outline the agency's right to audit the vendor's practices related to medical supplies and equipment management.

Monitoring and Enforcement

- The agency will conduct periodic audits or reviews to ensure the contracted service provider adheres to the agreed-upon policies and procedures for medical supplies and equipment management.
- This may involve reviewing the vendor's documentation for maintenance logs, inventory records, and staff training related to medical supplies and equipment use.
- If any discrepancies or non-compliance issues are identified, the agency will take corrective action, which may include:

- Issuing a corrective action plan for the vendor to address deficiencies.
- Withholding payment until corrective actions are implemented.
- Termination of the contract in cases of severe or persistent non-compliance.

Responsibilities

- **Contracting Department:**
  - Include clear requirements for medical supplies and equipment management practices in vendor contracts.
  - Evaluate vendor responses during the selection process regarding their compliance with WAC 246-841-001(1).
- **Quality Improvement Department:**
  - Develop and conduct audits or reviews to assess contractor compliance with this policy.
  - Report audit findings and recommend corrective actions as needed.
- **Management:**
  - Ensure the agency upholds its responsibility for patient safety by enforcing this policy with contracted service providers.

If the Agency contracts for medical supplies or equipment services

7. **Contractor Selection**
   - Prior to contracting with a medical supply and equipment service provider, a thorough evaluation process will be conducted. This will assess the contractor's:
     - Licenses and certifications in Washington State.
     - Experience and reputation in providing medical supplies and equipment.
     - Policies and procedures related to infection control, patient safety, and quality assurance.
     - References from other healthcare agencies.
   - The agency will only contract with vendors who demonstrate adherence to high standards for quality, safety, and infection control practices.
8. **Contractual Requirements**
   - All contracts with medical supply and equipment service providers will include clear language outlining the following:
     - The specific medical supplies and equipment to be provided.
     - Delivery schedules and procedures.
     - Maintenance and repair protocols for equipment.
     - Infection control practices for cleaning, disinfecting, and storing equipment.
     - Procedures for handling product recalls and defects.
     - A communication plan for addressing any concerns or issues regarding supplies or equipment.
     - Confidentiality requirements for protecting patient information.
9. **Contractor Monitoring**
   - The agency will establish a system for monitoring the performance of our contracted medical supply and equipment service providers. This may include:

- Regular reviews of contractor policies and procedures.
- Site visits to observe the contractor's operations.
- Reviewing customer satisfaction surveys or feedback reports.
- Monitoring for recalls or safety alerts related to medical supplies or equipment provided by the contractor.

10. **Corrective Action**
    - If the agency identifies any concerns regarding a contractor's compliance with infection control standards, patient safety protocols, or contractual obligations, a corrective action plan will be implemented.
    - This plan may involve requiring the contractor to revise their procedures, conduct additional training for their staff, or potentially terminate the contract if serious issues persist.
11. **Documentation**
    - The agency will maintain complete records of all contracts with medical supply and equipment service providers, including:
        - Evaluation documents used in selecting the contractor.
        - The signed contract outlining terms and expectations.
        - Documentation of any monitoring activities or communication with the contractor.
        - Records of any corrective action plans implemented.
12. **Compliance**
    - This policy is designed to ensure our agency adheres to subsection (1) of this section and all applicable Washington State regulations related to infection control, patient safety, and contracting for medical supplies and equipment services.

By implementing this policy, our home health agency can foster strong working relationships with reliable and compliant medical supply and equipment service providers. This ultimately benefits patient care by ensuring access to safe and effective medical supplies and equipment.